Psychiatry - Previous Year Questions

Marrow Q-Bank Collection • NEET PG / INI-CET / AIIMS (2017-2025)

Total Questions: 126
High-Yield Study Edition

Q1. Psychiatry NEET 2022 ID: MF5263

A 16-year-old girl has intense cravings for food. She eats large amounts of food, which is followed by self-induced vomiting. What is the probable diagnosis?

A Anorexia nervosa

B ✓ Bulimia nervosa

C Atypical depression

D Binge eating disorder

Correct Answer: Bulimia nervosa

The probable diagnosis in the given scenario of a 16-year-old girl with **intense food cravings** followed by **self-induced vomiting** is **bulimia nervosa.**

Bulimia nervosa is an eating disorder, often seen in young women. It is characterized by **binge-eating episodes**, followed by inappropriate, compensatory measures to stop weight gain, like **self-induced vomiting**, laxative abuse, diuretics, enema, and emetic abuse. These binge-eating episodes occur at least once a week and are present for at least 3 months. The patient fears gaining weight and has a relentless drive to be thin.

The **weight** is **normal** in most patients with bulimia nervosa. However, few patients may either be overweight or underweight. There is a risk of dehydration and electrolyte imbalance.

Treatment modalities are cognitive behavioral therapy, psychotherapy, and antidepressants such as fluoxetine.

Other options:

Option A: **Anorexia nervosa** is an eating disorder that is often seen between the ages of 10 and 30. Typical features are self-induced **starvation**, voluntary and **unhealthy reduction in weight**, intense drive for thinness, and fear of becoming obese. These symptoms are usually present for at least a period of 3 months. Associated features can include depression, anxiety, amenorrhea in females, hypothermia, and bradycardia.

Option C: **Atypical depression** is a type of depression characterized by **mood reactivity,** which is brightening of the mood in response to positive events; **significant weight gain** or increase in appetite; **hypersomnia; leaden paralysis;** and/or interpersonal rejection sensitivity, resulting in significant social or occupational impairment.

Option D: **Binge-eating disorder** is the most common eating disorder. It is characterized by **rapid intake of abnormally large quantities of food** over a short period of time, even when not hungry. There are feelings of guilt about the eating episodes, but these patients **do not compensate** in any way following the episodes of binge.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 363, 509, 511, 512, 514, 516-520
Online Resource, https://www.mayoclinic.org/diseases-conditions/atypical-depression/diagnosis-treatment/drc-20369754

Q2. Psychiatry NEET 2022 ID: MF5261

A woman, who is 4 days postpartum, presented with tearfulness, mood swings, and occasional insomnia. What is the likely diagnosis?

A Postpartum depression

B ✓ Postpartum blues

C Postpartum psychosis

D Postpartum anxiety

Correct Answer: Postpartum blues

Postpartum blues is the most likely diagnosis in a postpartum woman with **tearfulness**, **mood swings**, and **insomnia** from **day 4.**

Postpartum blues, also known as baby blues, is a transient mood disorder characterized by sadness, tearfulness, confusion, and dysphoria. There are no suicidal thoughts or thoughts of harming the baby. These symptoms typically develop **3-5 days after delivery.** They are thought to be due to hormonal changes, the stress of giving birth, and the responsibilities of motherhood. They **resolve spontaneously within 10-14 days.**

Management involves **reassurance, education,** and **support.** The patient must be followed up and assessed if symptoms persist for >2 weeks to evaluate postpartum depression.

Other options:

Option A: When the symptoms of depressed mood, anxiety, insomnia, and weight changes **start within 4 weeks** as per **DSM 5** or **6 weeks** as per **ICD 11,** and the symptoms **persist for ≥ 2 weeks**, the diagnosis will be **postpartum depression.** The patient may have **occasional thoughts** of **harming** the **baby** and **suicidal thoughts**. It can persist for months to years when untreated.

Option C: **Postpartum psychosis** is a psychiatric **emergency** that usually occurs within **4 weeks** after delivery (DSM 5) or **6 weeks** after delivery (ICD 11). The predominant symptoms are **hallucinations**, **delusions**, **suicidal thoughts**, and thoughts of **harming the baby**. Psychotic symptoms such as insomnia, restlessness, labile mood, and agitation are seen. Lithium**,** antipsychotics, and an antidepressant is the treatment of choice.

Option D: **Postpartum anxiety** is not defined by DSM-5. However, some platforms might describe postpartum anxiety as excessive or severe worry after childbirth.

Note: Zuranolone is the first-ever oral drug to be approved specifically for the treatment of postpartum depression. It is a neuroactive steroid that enhances the action of GABA in the brain and regulates mood.

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References: DSM-5 (R) Guidebook: The Essential Companion to the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Page no: 68, 112
Online Resource, https://faculty.ksu.edu.sa/sites/default/files/icd-_11_2018.pdf

Q3. Psychiatry NEET 2018 ID: MA7151

A 40-year-old lady met with a car accident 2 months ago, following which she complains of screaming and waking up in the night after having nightmares of the same incident over and over again. Which of the following conditions is she suffering from?

A Acute stress reaction

B Adjustment disorder

C Mania

D ✓ Post traumatic stress disorder

Correct Answer: Post traumatic stress disorder

The given clinical scenario is suggestive of **post-traumatic stress disorder**. PTSD is a psychiatric disorder occurring after a **life-threatening or potentially fatal event**, (e.g., war, house fire, serious accident, rape, robbery) affecting the patient or the patient's close friend or relative.

Symptoms can be divided into four types:

- **Re-experiencing** (e.g., intrusive memories of the event [**flashbacks**] and **nightmares**)
- **Hyperarousal** (e.g., anxiety, increased startle response, impaired sleep, hypervigilance)
- **Negative cognition** (e.g., negative beliefs, emotional numbing)
- **Avoidance** (e.g., avoiding stimuli associated with the traumatic event)

Psychotherapies found to be effective for PTSD include exposure therapy, trauma-focused cognitive-behavioral therapy, eye movement desensitization, and reprocessing.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page no 429, 172-174

Q4. Psychiatry NEET 2018 ID: MC0277

Which of the following features is more in favour of delirium?

A Occurs gradually over a period of time

B ✓ Fluctuating course

C Preserved consciousness

D Commonly associated with auditory hallucinations

Correct Answer: Fluctuating course

Fluctuating course is suggestive of **delirium.**

Characteristic presentation of delirium:

- **Acute onset**
- **Brief duration** - usually days to weeks
- Decline in the level of consciousness.
- **Fluctuating course** - unpredictable fluctuations in severity and other clinical manifestations
- **Sundowning** - worsening of symptoms at night
- **Rapid improvement** - when the underlying cause is treated
- Other characteristics include asterixis, **carphologia** (picking movements on cover sheets and clothes)
- More commonly associated with **visual hallucinations**

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no: 699

Q5. Psychiatry NEET 2018 ID: MC4279

What is defined as the emotional release and discharge after consciously reliving a painful experience that has been repressed?

A Catharsis

B ✓ Abreaction

C Venting out

D Guided relaxation

Correct Answer: Abreaction

Abreaction is a psychoanalytical term for **reliving an experience** to release it emotionally. It is a specific type of catharsis done by **becoming conscious** of repressed traumatic events.

Catharsis is a general term for the process of releasing strong or repressed emotions. It can be done through various mediums, like expressing emotions through art. When the unconscious repressed memories are consciously recalled and relived, this technique specifically is called abreaction.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 154

Q6. Psychiatry NEET 2018 ID: MC4195

What is 'mental retardation' now called according to the American Psychiatric Association in its revision of DSM-5?

A Feeble mindedness

B Mental subnormality

C ✓ Intellectual disability

D Intellectually challenged

Correct Answer: Intellectual disability

Mental retardation is now known as **Intellectual Disability**, according to the revised **DSM-5,** by the American Psychiatrist Association.

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References: Online Resource, https://journals.lww.com/co-psychiatry/Citation/2013/05000/New_terminology_for_mental_retardation_in_DSM_5.6.aspx

Q7. Psychiatry NEET 2018 ID: MC3760

The confusion assessment test is used in:

A Dementia

B Depression

C ✓ Delirium

D Schizophrenia

Correct Answer: Delirium

The confusion assessment test is used in the detection of **delirium.**

Delirium is reported to occur in a wide range of mechanically ventilated ICU patients and can be detected by the Confusion Assessment Method (CAM)-ICU or the Intensive Care Delirium Screening Checklist.

These tools are used to ask patients to answer simple questions and perform simple tasks and can be used readily at the bedside.

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References: Harrison's Principles of Internal Medicine 20th Edition, Page no. 2029

Q8. Psychiatry NEET 2018 ID: MC4196

Which of these disorders was earlier known as minimal brain dysfunction?

A Dyslexia

B ✓ Attention deficit/hyperactivity disorder

C Mental subnormality

D Oligophrenia

Correct Answer: Attention deficit/hyperactivity disorder

Attention deficit/hyperactivity disorder (ADHD) was known as **minimal brain dysfunction** earlier.

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References: Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3000907/
Kaplan and Sadock's Comprehensive Textbook of Psychiatry - Ebook - 10th Edition, Page no.8230

Q9. Psychiatry NEET 2018 ID: MC4278

Seman's squeeze technique is used for which of the following conditions?

A Retrograde ejaculation

B Erectile dysfunction

C ✓ Premature ejaculation

D Painful ejaculation

Correct Answer: Premature ejaculation

Seman's squeeze technique is used for **premature ejaculation**, to prolong the time period until ejaculation.

Seman's technique (stop-start technique): It is a variation of the squeeze technique where a man approaches the point close to ejaculation and then stops before ejaculation and relaxes before starting stimulation again. This method prolongs the time until ejaculation by relying on behavior modification.

Other treatments for premature ejaculation:

- **Squeeze technique (Master and Johnson's technique)-** as a man approaches a climax, he squeezes the tip of the penis just below the head of the penis. The pressure is held there until the sensation of impending orgasm diminishes. The process can then be started over again so that over time a man prolongs the time period until he reaches ejaculation.
- **Sensate focusing-** refocussing on the non-coital aspects of sexual activity.
- **Selective serotonin reuptake inhibitors (SSRIs)-** delays ejaculation. e.g.- dapoxetine, paroxetine.

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References: Kaplan and Sadock's Comprehensive Textbook of Psychiatry - Ebook - 10th Edition, Page no 5051

Q10. Psychiatry NEET 2018 ID: MC2942

Addison's disease can present with symptoms similar to which of the following disorders?

- **A** ✓ Depression
- **B** Generalised anxiety disorder
- **C** Bipolar disorder
- **D** Panic disorder

Correct Answer: Depression

Addison's disease presents most similarly to **depression.** Apathy, social withdrawal, impaired sleep, decreased concentration, and prominent fatigue is frequently seen which is usually mistaken for depression.

Following infection, trauma, or surgery, a patient with previously unrecognized Addison's disease may suffer from an Addisonian crisis. Such patients present with either shock or hypoglycemia. This is a medical emergency requiring urgent medical referral.

Endocrinopathy	Differential psychiatric diagnosis
Addison's disease	Depression
Hyperthyroidism	Mood disorder, panic disorder, generalized anxiety disorder
Hypothyroidism	**Depression,** hypomania, delusions, schizophrenia (**myxedema madness**).
Hypoparathyroidism	**Delirium** is the most common.
Hyperparathyroidism	Depression, anxiety, **paranoid delusions**. Tetrad of "painful bones, renal stones, abdominal groans, and psychiatric overtones".
Hypoglycemia	Anxiety, depersonalization, derealisation, aggressive behavior, amnesia.

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References: Emergencies in Psychiatry, Page no 56
Kaplan and Sadock's Comprehensive Textbook of Psychiatry - Ebook - 10th Edition, Page nos 1790-1803

Q11. Psychiatry NEET 2018 ID: MC4472

Which of the following is not an OCD related disorder?

A Hair pulling disorder

B ✓ Temper tantrums

C Hoarding disorder

D Skin picking

Correct Answer: Temper tantrums

Temper tantrums are not OCD-related.

Obsessive-compulsive and related disorders are:

- Obsessive-Compulsive Disorder
- Body Dysmorphic Disorder
- **Hoarding Disorder**
- **Hair-Pulling Disorder** (Trichotillomania)
- Excoriation (**Skin-Picking**) Disorder

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References: Kaplan and Sadock's Comprehensive Textbook of Psychiatry - Ebook - 10th Edition, page no 2760
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, page no 294
Online Resource, https://icd.who.int/browse11/l-m/en#/http%3a%2f%2fid.who.int%2ficd%2fentity%2f1321276661
DSM-5 (R) Guidebook: The Essential Companion to the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, page no 147

Q12. Psychiatry NEET 2018 ID: MD7324

Which of the following is not true about Alzheimer's disease?

A Intracellular neurofibrillary tangles are seen

B Neuritic plaques made of beta amyloid are found

C ✓ Nucleus of Meynert is not affected

D Short term memory is affected

Correct Answer: Nucleus of Meynert is not affected

Nucleus of Meynert is affected in Alzheimer's disease.

Biochemically, there is a reduction of acetylcholine and **degeneration** of **cholinergic neurons** in the **nucleus basalis of Meynert** that project throughout the cortex. There is also noradrenergic and serotonergic depletion due to degeneration of brain stem nuclei such as the locus ceruleus and the dorsal raphe.

Microscopically, there are neuritic plaques containing amyloid-β, neurofibrillary tangles, and amyloid-β accumulation in the blood vessel walls of the cortex and leptomeninges.

The FDA recently approved **Flortaucipir F18** for **intravenous injection**, the **first radioactive diagnostic agent** for adult patients with cognitive impairment who are being evaluated for Alzheimer's disease. It is indicated for **PET imaging of the brain** to estimate the density and distribution of aggregated **tau** neurofibrillary tangles (NFTs), a primary marker of Alzheimer's disease.

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References: Harrison's Principles of Internal Medicine 20th Edition, Page no 3108-11

Q13. Psychiatry AIIMS 2018 ID: MC5211

A patient was prescribed escitalopram. Which of the following adverse effect is the least likely?

A Nausea

B Vivid dreams

C Anorgasmia

D ✓ Sialorrhea

Correct Answer: Sialorrhea

Sialorrhea is **not a side effect** of escitalopram.

Escitalopram is a selective serotonin reuptake inhibitor (SSRI). **SSRIs** are associated with **dry mouth, and not sialorrhea.**

System	Side effects of SSRIs
Sexual	**Anorgasmia**, Decreased libido
Gastrointestinal (5-HT3 receptor)	**Nausea**, Diarrhea, Anorexia, Vomiting, Flatulence, and Dyspepsia
Nervous system	Extremely **vivid dreams** or nightmares, Anxiety, Insomnia/sedation, Seizures, Emotional blunting
Anticholinergic	Dry mouth, Constipation, Sedation
Cardiovascular	Drug induced long QT syndrome
Hematological	Easy bruising
Endocrine	Mammoplasia, Galactorrhea

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 1019

Q14. Psychiatry AIIMS 2018 ID: MC3474

Abrupt stoppage of which of the following drugs causes agitation, anxiety and insomnia?

A Valporate

B Olanzapine

C Imipramine

D ✓ Venlafaxine

Correct Answer: Venlafaxine

Abrupt stoppage of **venlafaxine** causes agitation, anxiety, and insomnia.

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References: Stahl's Essential Psychopharmacology, Page no 343

Q15. Psychiatry AIIMS 2018 ID: MC5209

A young male who is shy and prefers social isolation appears to be emotionally cold and resides alone. There is no history of hallucinations or delusions. The most likely diagnosis is _____

A Paranoid personality disorder

B ✓ Schizoid personality disorder

C Antisocial personality disorder

D Emotionally unstable personality disorder

Correct Answer: Schizoid personality disorder

The diagnosis, in this case, is **schizoid personality disorder**.

Schizoid personality disorder is a **cluster A personality disorder**. The patient neither desires nor enjoys close relationships (shy and prefers social isolation, resides alone as in the question) and shows **emotional coldness and detachment.**

In contrast, subjects with emotionally unstable (**borderline**) personality disorder **cannot tolerate being alone**, and they search for companionship.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page nos 746-750

Q16. Psychiatry AIIMS 2018 ID: MC5212

A 14 year old girl presents to the emergency room with fracture forearm and says she has tripped and fell. On examination you notice multiple cigarette burns on her forearm. What will you do next?

A ✓ Conduct a thorough examination

B Seek a social worker's help

C Inform higher authorities immediately

D Inform your colleagues

Correct Answer: Conduct a thorough examination

A child who has been physically abused should undergo a **thorough physical examination** at first.

This should be **followed by a psychiatric interview.**

When the child is brought to the emergency room, a detailed and spontaneous account of the injury should be obtained promptly from parents or other caregivers **before secondary details and rationalizations** cloud the information provided.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 1318

Q17. Psychiatry AIIMS 2018 ID: MC5210

Among the following, what is the most common symptom of adult obsessive-compulsive disorder?

A Aggressive symptoms

B Need for symmetry

C ✓ Pathological doubt

D Sexual symptoms

Correct Answer: Pathological doubt

Among the above options, pathological doubt is the common symptom of adult OCD. **Pathological doubt** is the **second most common** pattern of adult obsessive-compulsive disorder (OCD), only **after obsession of contamination**.

Pathological doubt is an obsession with doubt, followed by a compulsion to check. The obsession often implies some danger of violence e.g., forgetting to turn off the stove or not locking a door. The checking may involve multiple trips back into the house to check the stove. These patients have obsessional self-doubt and always feel guilty about having forgotten or committed something.

The frequency of presenting patterns of OCD :

Obsession of contamination > Pathological doubt > Intrusive thoughts> Symmetry

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 423

Q18. Psychiatry AIIMS 2018 ID: MC5208

Which of the following is not a feature of atypical depression?

A Increased sleep

B Reactive mood response to positive stimulus

C Heaviness of the body

D ✓ Responds better to TCAs than MAO-I and SSRI

Correct Answer: Responds better to TCAs than MAO-I and SSRI

Patients with atypical features do **respond effectively to treatment with monoamine oxidase inhibitors (MAOIs) and selective serotonin reuptake inhibitors (SSRIs)**.

Mood reactivity, increased sleep, weight gain, and leaden paralysis (heaviness of body) are **features of atypical depression.**

Mood reactivity is the capacity to be cheered up when presented with positive events.

Leaden paralysis is the heavy, leaden feelings in arms or legs.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Pages: 382, 572, 390

Q19. Psychiatry AIIMS 2018 ID: MC7069

Under the Mental Health Care Act 2017, a patient has the right to choose his or her caretaker and future course of action in treatment. What is this provision known as?

A Treatment directive

B Mental will

C Future directive

D ✓ Advance directive

Correct Answer: Advance directive

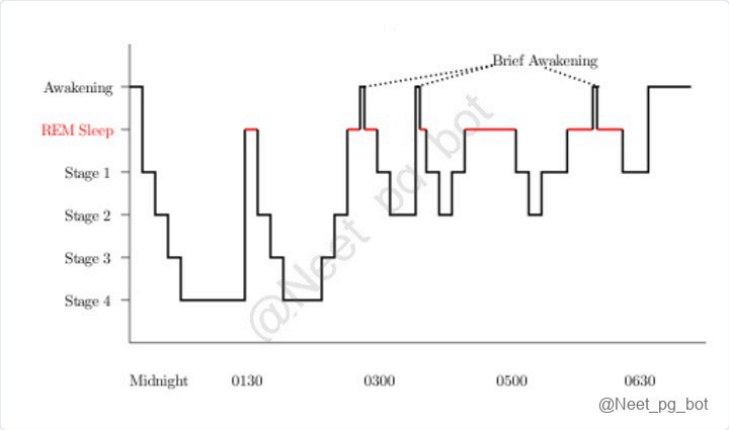
The provision by which a patient has the right to choose his caretaker and future course of action in treatment because of **illness or incapacity** is known as an **advance directive**. It is also known as a **living will,** personal directive, or medical directive.

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References: Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5932926/
Online Resource, https://indiacode.nic.in/handle/123456789/2249?view_type=search&sam_handle=123456789/1362

Q20. Psychiatry AIIMS 2018 ID: MC6986

What does the given image represent?



A ✓ Hypnogram

B Electroencephalogram

C Polysomnogram

D Epworth sleep assessment

Correct Answer: Hypnogram

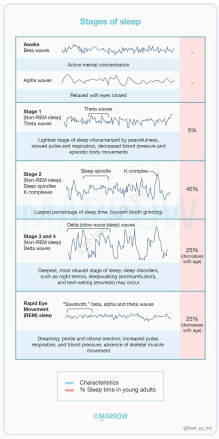
The above image represents a **hypnogram**. It is a graph that **depicts the stages of sleep as a function of time**. It provides a visual representation of sleep structure and is useful in the diagnosis of various sleep disorders.

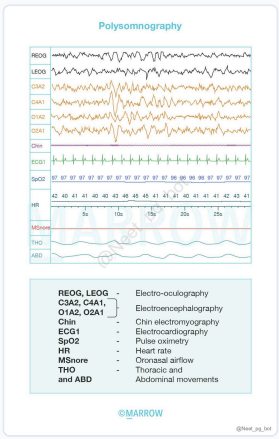
An **electroencephalogram** is the depiction of the **electrical activity of the surface** of the brain. Different waveforms (alpha, beta, theta, delta) are produced depending on the functional status of the brain.

Polysomnography is a multi parametric test that is considered the **gold standard** in the **diagnosis of sleep disorders**. It is a continuous recording of biophysiological changes that occur during sleep. It includes,

- Continuous EEG recording, from occipital and parietal leads
- Electro-oculography- to record eye movements
- Electromyography- for muscle potential and its activities
- ECG
- Multiple sleep latency tests- repeated measures of time taken to sleep
- Nasal-oral airflow, respiratory effort, oxyhemoglobin saturation, and leg movements during sleep are measured.

Epworth sleep assessment scale is a questionnaire used to **quantify daytime sleepiness.** It determines how frequently a patient is likely to doze off in 8 commonly encountered situations. It is a subjective assessment and hence less reliable.





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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, pg no 562
Online Resource, Characterizing Sleep Structure Using the Hypnogram - NCBI - NIHhttps://www.ncbi.nlm.nih.gov/pmc/articles/PMC2542492/
Online Resource, What is the role of the Epworth Sleepiness Scale (ESS) in ... - Medscapehttps://www.medscape.com/answers/295807-53519/53519-print

Q21. Psychiatry AIIMS 2018 ID: MC7070

A 40-year-old woman has a history of urinating in public without having any remorse for it. She also has difficulty in getting motivated and has mild memory deficits. Which of the following lobes may be affected?

A Parietal

B Occipital

C ✓ Frontal

D Temporal

Correct Answer: Frontal

The **frontal lobe** is affected in this patient. The woman has difficulty in getting motivated, memory deficits, and urinary incontinence. Frontal lobe injury is commonly due to trauma, infarcts, tumors, lobotomy, multiple sclerosis, or Pick's disease. The following features may be seen in a patient with frontal lobe injury:

- Impaired executive functions such as motivation, attention, and sequencing of actions
- Aphasia, apraxia, constructional apraxia, neglect, and memory deficits
- Urinary incontinence and fecal incontinence is commonly seen
- Changes in personality
- Apathetic indifference and impulsive behavior.

Note: Frontal lobe pathology may become apparent only under unstructured, stressful, real-life situations and are difficult to detect using Intelligence scales, as IQ mostly requires parietal lobe activation.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 15

Q22. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD4353

You are attending physiology viva. The examiner asks you who recorded the first EEG. You say "I don't know ". The examiner feels bad for you and gives you a clue that "the name sounds like something you get commonly in McDonalds". You remember the name and answer correctly. What type of memory has been implicated here?"

A Semantic memory

B Implicit memory

C Associative learning

D ✓ Priming

Correct Answer: Priming

In the given scenario, **priming** is the **most appropriate answer.**

The examiner is asking for a fact, and then prompting the answer, by giving a clue which sounds (phonological) like the answer. This is an example of priming.

Prime is any cue, which precedes the recall of the target word. It maybe similar to hear (sun-bun-gun) or may hold similarity to the target word (Sun- moon - star).

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, P 116
Online Resource, https://link.springer.com/content/pdf/10.3758/BF03213414.pdf
Journal, Tulving E, Schacter DL. Priming and human memory systems. Science. 1990 Jan 19;247(4940):301-6. doi: 10.1126/science.2296719. PMID: 2296719.
http://alicekim.ca/TulvingSchacter92.pdf
Ganong's Review Of Medical physiology, 26th Edition, Page no 280-281

Q23. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD4352

A known alcoholic patient is brought to the emergency in a disoriented state. He complains of seeing snakes and insects crawling on his skin. Which of the following should be given to this patient?

A Haloperidol + thiamine

B Lorazepam only

C ✓ Lorazepam + thiamine

D Haloperidol only

Correct Answer: Lorazepam + thiamine

The given clinical scenario points towards **delirium tremens likely due to acute alcohol withdrawal**. **Lorazepam + thiamine** should be given to this patient.

Delirium tremens occurs after 2-4 days of abstinence in a heavy or chronic drinker. It is characterized by autonomic hyperactivity, tremors, delusions, and hallucinations. It usually resolves spontaneously in 7 days but may cause the death of the patient due to cardiovascular collapse or infections or self-injury.

Delirium tremens is treated using benzodiazepines starting with a higher dosage which is gradually tapered as withdrawal symptoms disappear. In cases of chronic alcohol use, vitamin deficiency (especially thiamine) may be co-existing and hence bolus doses of thiamine are given to prevent/treat Wernicke encephalopathy.

Haloperidol is generally not the first-line drug in case of alcoholic withdrawal. It might be used as an add-on in agitated patients. It decreases the seizure threshold and its use would be appropriate in a case of co-existing psychotic disorder like schizophrenia.

Drug Therapy For Alcohol Intoxication and Withdrawal

Clinical Problem	Drug	Route	Dosage
Tremulousness and mild to moderate agitation	Chlordiazepoxide	Oral	25-100 mg every 4-6 hrs
	Diazepam	Oral	5-20 mg every 4-6 hrs
Hallucinosis	Lorazepam	Oral	2-10 mg every 4-6 hrs
Extreme agitation	Chlordiazepoxide	Intravenous	0.5 mg/kg at 12 mg/min
Withdrawal seizures	Diazepam	Intravenous	0.15 mg/kg at 2.5 mg/min
Delirium tremens	Lorazepam	Intravenous	0.1 mg/kg at 2.0 mg/min

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition, Page nos 38 and 40
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 631, 1410
Online Resource, https://www.uptodate.com/contents/management-of-moderate-and-severe-alcohol-withdrawal-syndromes

Q24. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD4343

A drug that selectively reduces the reuptake of serotonin would likely cause which of the following side effects?

A Constipation

B ✓ Sexual dysfunction

C Sialorrhea

D Visual disturbance

Correct Answer: Sexual dysfunction

Selective serotonin reuptake inhibitors (SSRIs) are associated with **sexual dysfunction.**

System	Side effects of SSRIs
Sexual	**Anorgasmia**, Decreased libido
Gastrointestinal (5-HT3 receptor)	Nausea, **diarrhea**, anorexia, vomiting, flatulence, and dyspepsia
Nervous system	Extremely **vivid dreams** or nightmares, Anxiety, Insomnia/sedation, seizures, emotional blunting
Anticholinergic	**Dry mouth**, constipation, sedation
Cardiovascular	Drug-induced long QT syndrome
Hematological	Easy bruising
Endocrine	Mammoplasia, galactorrhea

Note - Sexual dysfunction is the most prominent side effect of SSRIs and **weak** anticholinergic effects are only shown by **paroxetine**, and the GI effects of SSRIs predominantly include **diarrhoea.**

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 1019

Q25. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD4337

A patient undergoing ECT for his severe depression was given thiopentone and succinylcholine. The purpose of these medications was to?

A To bring calming effect

B To produce mood elevation

C ✓ To induce anesthesia and relax muscles

D To prevent memory loss after the ECT

Correct Answer: To induce anesthesia and relax muscles

Thiopentone and succinylcholine are given for **anesthesia induction and muscle relaxation** respectively.

Thiopentone sodium is a barbiturate and acts by opening chloride channels, whilst succinylcholine is a depolarising muscle relaxant.

ECT can be of 2 types:

- Direct - without anesthesia and muscle relaxant
- Modified - with anesthesia and muscle relaxant

Currently modified ECT is preferred and is given with adequate spacing i.e., no more than 3 ECT sessions per week.

After adequate preparation, a current of 200-1600mA (110V for 0.1-1 seconds) is given, resulting in an adequate seizure. Electroconvulsive therapy treatments are associated with retrograde amnesia for a period of several minutes before the treatment and anterograde amnesia after the treatment.

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition, Page nos 201 and 202
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, P 1069

Q26. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MC5209

A young male who is shy and prefers social isolation appears to be emotionally cold and resides alone. There is no history of hallucinations or delusions. The most likely diagnosis is _____

A Paranoid personality disorder

B ✓ Schizoid personality disorder

C Antisocial personality disorder

D Emotionally unstable personality disorder

Correct Answer: Schizoid personality disorder

The diagnosis, in this case, is **schizoid personality disorder**.

Schizoid personality disorder is a **cluster A personality disorder**. The patient neither desires nor enjoys close relationships (shy and prefers social isolation, resides alone as in the question) and shows **emotional coldness and detachment.**

In contrast, subjects with emotionally unstable (**borderline**) personality disorder **cannot tolerate being alone**, and they search for companionship.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page nos 746-750

Q27. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6702

A 40-year-old male patient comes to the psychiatry OPD with complaints of repetitive thoughts that, he always feels his hands are dirty, though they are not. This gives him discomfort and hence he has to wash them again and again. This is a disorder of thought _____.

A Flow

B Form

C ✓ Possession

D Content

Correct Answer: Possession

The clinical history is suggestive of obsessive-compulsive disorder, which is a disorder of **thought possession**.

The diseases under thought possession include:

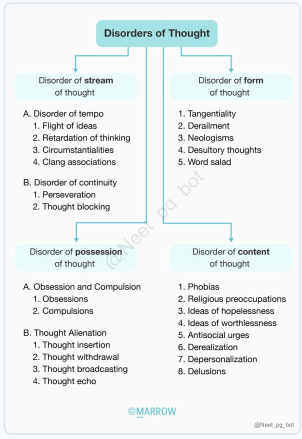
1. Obsession and Compulsion
2. Thought Alienation

In obsessions and compulsions, although the sense of thought possession in the patient is not impaired, he/she experiences the **obsessive thoughts** to **appear against their will**. Hence, despite the patient perceiving a loss of control over the **voluntary act** (**compulsion**), they still retain personal possession (ownership) of the act.

Obsessive-compulsive disorder is defined as the presence of obsessions and/or compulsions.

- **Obsessions** are recurrent and persistent **ideas,** thoughts, urges, or images that are experienced as intrusive and unwanted and cause anxiety or distress to the patients.
- **Compulsions** are the repetitive and intentional behaviors or **acts** performed in response to obsessions or according to certain rules that must be applied rigidly (e.g., repetitive hand washing, ritualistic checking).

In the given case, the obsessional thoughts are the repetitive, unwanted thoughts that his hands are dirty, while the compulsion is to wash his hands to neutralize the obsession.



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References: Fish's Clinical Psychopathology: Signs and Symptoms in Psychiatry, Page no 46, 87
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC2919798/
Online Resource, https://oaji.net/articles/2014/1170-1412139787.pdf

Q28. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6703

Which of the following is not a type of subcortical dementia?

- A ✓ Pick's disease
- B Parkinson's disease
- C Huntington's disease
- D Wilson's disease

Correct Answer: Pick's disease

Pick's disease (a type of frontotemporal dementia) is a type of **cortical dementia.**

	Cortical dementia	Subcortical dementia
Site of lesion	Cortex	Thalamus, basal ganglia, cerebellum, brain stem
Higher cortical symptoms	Aphasia Apraxia Agnosia Prosopagnosia Acalculia	No
Motor symptoms	No	Dysarthria Tremors Ataxia Rigidity
Memory	Recall and recognition are impaired	Recall is impaired
Examples	• Alzheimer's disease • Fronto-temporal dementia • Creutzfeldt Jakob disease	• Parkinson's disease • Progressive supranuclear palsy • Huntington's disease • Wilson's disease • Idiopathic basal ganglia calcification • Normal-pressure hydrocephalus • Vascular dementia

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 1346

Q29. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6704

Which of the following is not a good prognostic factor in schizophrenia?

A Positive symptoms

B Late age of onset

C ✓ Insidious onset of symptoms

D Associated with depression

Correct Answer: Insidious onset of symptoms

Insidious onset of symptoms is a **poor prognostic factor** of schizophrenia.

Prognostic factors of Schizophrenia:

Good Prognostic factors	Poor prognostic factors
Acute onset	Insidious onset
Late-onset	Early-onset
Presence of a stressor	Absence of a stressor
Short duration < 6 months	Duration >2 years
Presence of depression	No depression
Positive symptoms	Negative symptoms
Family history of mood disorder	Family history of schizophrenia
Married, female sex	Single, male sex
Catatonic schizophrenia	Hebephrenic schizophrenia
Type 1 schizophrenia	Type 2 schizophrenia
	No remissions in 3 years
	History of perinatal trauma
	History of assaultiveness

T.J.Crow classified schizophrenia into two separate types :

Type 1	Type 2
Positive symptoms present	Negative symptoms present
More responsive to treatment	Less responsive to treatment
No demonstrable neuropathology	Demonstrable neuropathology (CT/MRI)
Good prognosis	Poor prognosis

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 318

Q30. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6705

A 7-year-old child presented with complaints of bed-wetting almost twice a week for the past 1 year. A detailed investigation was done to rule out the organic cause. What should be the treatment plan for this child?

A Pharmacotherapy with imipramine

B Psychodynamic psychotherapy

C Bladder training, with reward for delaying micturition during daytime

D ✓ Bell and pad based classical conditioning

Correct Answer: Bell and pad based classical conditioning

The clinical features are suggestive of nocturnal enuresis. **Bell and pad based classical conditioning** is the most effective treatment.

Nocturnal enuresis refers to the occurrence of involuntary urination at night **after 5 years of age**. It may be a normal phenomenon in children up to 5 years of age.

Treatment for nocturnal enuresis may include:

Behavioural therapy:

- Classic conditioning with the **bell** (or buzzer) and **pad** (alarm) apparatus is the **most effective** treatment for enuresis.
- Dryness is achieved in 50% of the cases.
- **Bladder training** with encouragement or reward for delaying micturition for increasing times during waking hours is useful but considered **inferior** to bell and pad.

Pharmacotherapy:

- Medications should be considered **if behavioural therapy has failed,** and if enuresis is causing impairment in social, family, and school function.
- Desmopressin (an antidiuretic) and Reboxetine (norepinephrine reuptake inhibitor) are commonly used drugs. Reboxetine is considered as a **safer alternative to imipramine.**

Psychotherapy:

- It is indicated in cases with chronic enuresis, to deal with any coexisting psychiatric problems and emotional and family difficulties.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 1216

Q31. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6706

A male patient who lost his job recently became irritable and had a sad mood for most of the time. Thinking about the future worried him and he felt irritation towards his family members. He occasionally went out to watch movies with his friends and was able to enjoy but on returning home he felt similar symptoms. What is the probable diagnosis?

A Generalised anxiety disorder

B ✓ Adjustment disorder

C Mixed anxiety with depression

D Moderate depression

Correct Answer: Adjustment disorder

The clinical features are suggestive of an **adjustment disorder.**

Adjustment disorder is an emotional response to a stressful event. **Stressors** may be **single** like loss of job or divorce; **multiple** like the death of a parent together with loss of a job; or **recurrent** like seasonal business difficulties.

Diagnosing criteria for adjustment disorder:

- Development of emotional or behavioural response **within 3 months** of the onset of the stressor.
- The symptoms or behaviour are **clinically significant,** where, any one of the following are met:
  - The response is **out of proportion** to the severity or intensity of the stressor.
  - Significant **impairment** in **social, occupational,** or other areas of functioning.
- The stress-related disturbance does not meet the criteria for another mental disorder or exacerbation of pre-existing mental disorder.
- Mostly depressive but can also have anxiety symptoms.
- The symptoms **disappear** within **6 months** if the **stressor** is **removed.**

In the given case, it has been mentioned that the patient lost his job recently. Although a time period is not mentioned in the question, the term recently can be understood as well within the 3 months duration. The patient's behaviour such as sad and irritated affected his social functioning. As he was finding joy in watching movies, depression can be ruled out.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 446-448

Q32. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6707

A 10-year-old child is suffering from selective mutism. He is most probably suffering from?

A Childhood depression

B Childhood psychosis

C ✓ Childhood anxiety disorder

D Hyperkinetic disorder

Correct Answer: Childhood anxiety disorder

The child is most likely suffering from a **childhood anxiety disorder.**

Selective mutism refers to the **lack of speaking** (being mute) in **one or more specific social situations**, commonly in school, but **speaking well in other situations**, commonly at home. The child may be either **completely silent** at school or maybe just **whispering.** Rarely, a child may be silent at home but communicating normally at school.

The disorder may develop in children between **4 to 8 years** of age and is associated with a **social anxiety disorder.** They may have symptoms of separation anxiety disorder, school refusal, and delayed language acquisition.

The first-line treatment includes **cognitive behavioural therapy.**

Option A and B: selective mutism is not commonly associated with childhood depression or psychosis

Option D: Hyperkinetic disorder is associated with **overfamiliarity.**

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Pages: 201-203

Q33. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6708

A psychotic patient developed voluntary purposeless movements and was once observed to be standing still for hours with waxy flexibility and negativism. What is the appropriate medical management?

A Haloperidol

B ✓ Lorazepam

C Clonidine

D Propanolol

Correct Answer: Lorazepam

The clinical features are suggestive of a **catatonic disorder**. Benzodiazepines such as **lorazepam** are effective in the treatment of catatonia.

Catatonia is a syndrome characterized by striking behavioral abnormalities such as motoric immobility or excitement, negativism, or echolalia (mimicry of speech), or echopraxia (mimicry of movement).

The clinical picture may include **three or more of the following features:**

1. **Stupor:** Absence of psychomotor activity and not actively relating to the environment.
2. **Catalepsy:** Passive induction of a posture held against gravity.
3. **Waxy flexibility:** Slight, even resistance to positioning by examiner.
4. **Negativism:** Opposition or no response to instructions or external stimuli.
5. **Posturing:** Spontaneous and active maintenance of a posture against gravity.
6. **Mannerism:** Odd, circumstantial caricature of normal actions.
7. **Stereotypy:** Repetitive, abnormally frequent, non-goal-directed movements
8. **Automatic obedience:** Commands are followed automatically, irrespective of their nature.
9. **Grimacing**
10. **Ambitendency:** Due to ambivalence, opposing actions are carried out.
11. **Echopraxia:** Mimicking another's movement.
12. **Echolalia:** Mimicking another's speech.
13. **Mutism:** Very little or no verbal response to instructions or external stimuli.
14. **Verbigeration:** Incomprehensible speech.

Catatonic disorder may occur due to:

- Advanced mood or psychotic illnesses
- Neurological disorders (e.g., nonconvulsive status epilepticus, and head trauma)
- Infections (e.g., encephalitis)
- Metabolic disturbances (e.g., hepatic encephalopathy, hyponatremia, and hypercalcemia)
- Drugs induced- like corticosteroids, immunosuppressants, and antipsychotics (i.e., neuroleptic) agents.

Management: The primary treatment includes:

- Identifying and treating underlying medical or pharmacological causes. Any offending substances should be removed.
- **Benzodiazepines,** especially, lorazepam, are effective in improving the symptoms.
- **Electroconvulsive therapy** is the definitive treatment in cases of life-threatening catatonia (if the patient is unable to eat) or if the patient has developed malignant catatonia.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page: 671

Q34. Psychiatry AIIMS 2020 (May and Nov INI-CET) ID: MD6709

Which of the following features differentiate delirium from dementia in Alzheimer's disease?

A Acuity of onset and agitation

B ✓ Acuity of onset and loss of consciousness

C Visual hallucinations and impaired memory

D Agitation and irritation

Correct Answer: Acuity of onset and loss of consciousness

Delirium is characterized by **acute onset** of symptoms and is often accompanied by diminished or **loss of consciousness and cognition.** This feature is useful to clinically differentiate between delirium and dementia.

Dementia is a group of diseases marked by chronic, progressive cognitive impairment with clear consciousness.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 697, 701, 704

Q35. Psychiatry INI-CET 2022 ID: MF5047

Which of the following is false about bipolar disorder?

A ✓ It most commonly affects ages 35-45 years

B Type 1 is common in both sexes

C Prevalence in general population is 1-2%

D Attempt to commit suicide is seen around 15% of people

Correct Answer: It most commonly affects ages 35-45 years

The false statement among the following is option A. The age of onset for **bipolar type I** disorder ranges from, children of age 5 or 6 years to adults of age 50 years or older, with a mean age of **18 years**. The mean age of onset for **bipolar type II** is **20 years**.

Patients with bipolar disorder can present with **both manic** and **depressive** episodes or with **manic** episodes **alone**. The bipolar disorder presented with **mania** and **depression** for at least a week is known as **bipolar type 1 disorder**. Whereas, if it is presented with **hypomania** and **depression** it is known as **bipolar type 2 disorder**.

The lifetime prevalence of bipolar type 1 disorder is **0 to 2.4%**, whereas bipolar type 2 disorder is **0.3-4.8%**. Bipolar type 1 disorder has an **equal prevalence** among men and women. Suicide attempt is seen in **around 15% of people** with bipolar disorder.

A **manic episode** is defined as a persistent elevated, expansive, or irritable mood for at least 1 week which requires hospitalization. **A hypomanic episode** is also similar to a manic episode except it doesn't cause social and occupational functioning impairment. It lasts for at least 4 days and **doesn't show psychotic symptoms**.

A person is said to be depressed if he has at least **four** of the following symptoms for at least **2 weeks**:

- Change in appetite and weight
- Change in sleep and activity
- Lack of energy
- Guilty feeling
- recurring thoughts about death or suicide.

Lithium is used for the pharmacological management of bipolar disorder.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 347-349,377
The Maudsley Prescribing Guidelines in Psychiatry 13th Edition, Page no 243

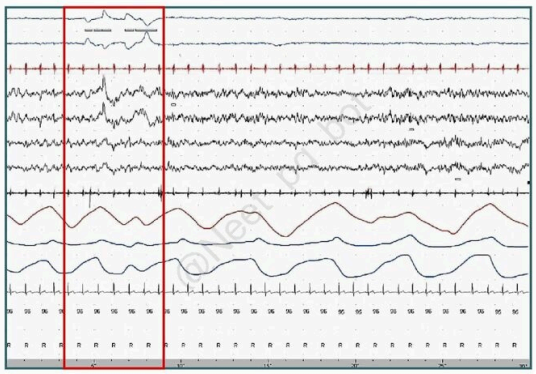
Q36. Psychiatry INI-CET 2022 ID: MF5078

On performing polysomnography in a patient, the waves of EOG, EEG, and EMG respectively from above downwards are seen below. Which stage of sleep does the marked area represent?

EOG- Electroocoulography

EEG- Electroencephalography

EMG- Electromyography



A ✓ REM sleep

B NREM I sleep

C NREM II sleep

D NREM III sleep

Correct Answer: REM sleep

REM sleep is represented by the marked area in the given polysomnography.

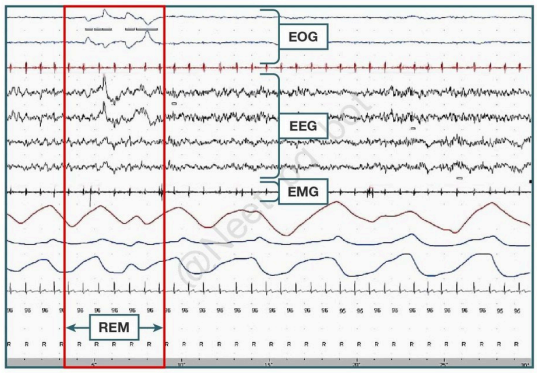
The three zones in the given image represent **EOG**, **EEG,** and **EMG**, respectively. In the marked area,

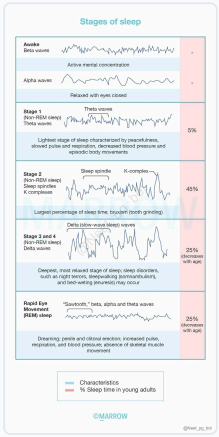
- EOG shows Positive activity
- EEG shows Mixed waves
- EMG shows Minimal activity

This is characteristic of **REM sleep**.

The following table helps in identifying the state of wakefulness and sleep from the polysomnograph :

	EEG	EOG	EMG
Active state	Fast	+	+
REM sleep	**Mixed**	**+**	**-**
NREM sleep	Slow	-	+





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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 534

Q37. Psychiatry INI-CET 2022 ID: MF5056

A patient diagnosed with schizophrenia was started on risperidone. After taking the medication last night, the patient rushes to OPD today with complaints of upwards rolling of his eyes. What is the management of the condition?

A Lorazepam

B ✓ Promethazine

C Olanzapine

D Stop antipsychotic

Correct Answer: Promethazine

The given clinical scenario is suggestive of **oculogyric crisis** and **promethazine** is used for the management.

Oculogyric crisis refers to the **spasm of the extraocular muscles**, usually causing **upward** and outward deviation of the eyes. It is a severe form of **acute dystonia**, an **extrapyramidal side effect** of antipsychotics like risperidone. It can be treated by drugs with antimuscarinic activities like promethazine.

Dystonia is **acute** in onset and occurs within **1-5 days** of starting treatment. It is more common in the **younger** population. The proposed mechanism of action is **D2 blockade** in the nigrostriatal pathway. There is typical involvement of **head and neck** muscles and presents in a variety of forms, such as **torticollis**, trismus, mouth opening dystonia, **grimacing**, dysarthria**, oculogyric crisis**, blepharospasm, and **swallowing difficulties**.

Risperidone is an atypical antipsychotic with **D2 blocking** but strong **5HT2 antagonistic** activity. Its main side effects include weight gain, hyperglycemia, **hyperprolactinemia**, dyslipidemia, fatigue, etc. At low doses, extrapyramidal side effects are less, but can be significant in higher doses. **Extrapyramidal side effects** include;

- **Parkinsonism**: Features are bradykinesia, rigidity, variable tremor, masked facies, and shuffling gait.
- **Acute dystonia**: Which causes spasms of muscles of the tongue, face, **eyes**, neck, back, etc.
- **Neuroleptic malignant syndrome:** Extreme rigidity, fever, unstable blood pressure,myoglobinemia, etc.
- **Rabbit syndrome:** Perioral tremors occur as a delayed variant of parkinsonism.

Other options:

Option A: **Lorazepam** is a benzodiazepine. Its adverse effects include **sedation, light-headedness, confusional state, weight gain,** etc.

Option C: **Olanzapine** is also an atypical antipsychotic. Its adverse effects are d**ry mouth, constipation, weight gain, impaired glucose tolerance** etc.

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References: Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 13th Edition, Page no 290, 291, 724
K.D Tripathi - Essentials of medical pharmacology 8th Edition, Page no 468, 469,471,494
Online Resource,
https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5623758/#:~:text=Oculogyric%20crises%20are%20defined%20as,are%20frequently%20accompanied%20by%20pain.

Q38. Psychiatry INI-CET 2022 ID: MF5068

Which of the following disease is exclusively seen in females?

A ✓ Rett syndrome

B Asperger syndrome

C Selective mutism

D Atypical autism

Correct Answer: Rett syndrome

Rett syndrome is exclusively seen in **females**. It is caused by **X-linked dominant** inheritance and hence affects only females, as most males **die in utero.**

Rett syndrome is a form of autism spectrum disorder. The child is normal at birth, but **by 1 year** of age **deterioration** in communication skills, motor behaviour, and social functioning are noticed. Symptoms are **ataxia**, loss of speech, teeth grinding, and **facial grimacing**. When the child is awake, intermittent hyperventilation and **disorganized breathing pattern** are observed. Stereotypical hand movements like **hand-wringing**, are characteristically seen. **Progressive gait disturbance** and **seizures** are seen. Decreased pigmentation of the substantia nigra, along with cerebral atrophy occurs, implying an abnormal dopaminergic nigrostriatal system.

Other options:

Option B: **Asperger syndrome** is a version of autism spectrum disorder. It is characterized by limited social interaction and restricted interest and behaviour. There is **no delay** in **language** development.

Option C: **Selective mutism** is a social anxiety disorder. The child hesitates to speak in one or more social settings, mostly the school.

Option D: **Atypical autism** is autism, atypical either in the age of onset or in symptomology or both.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 1124,1164,1167,1261

Q39. Psychiatry INI-CET 2022 ID: MC6215

Match the following:

1 - Kleptomania	A - Intense desire to drink alcoholic drinks
2 - Pyromania	B - Intense desire to mutilate
3 - Mutilomania	C - Intense desire to steal items of trivial value
4 - Dipsomania	D - Intense desire to burn things

A 1-D, 2- B, 3-A, 4-C

B 1-C, 2-B, 3-D, 4-A

C ✓ 1-C, 2-D, 3-B, 4-A

D 1-D, 2-C, 3-B, 4-A

Correct Answer: 1-C, 2-D, 3-B, 4-A

The correct option is 1-C, 2-D, 3-B, 4-A

Impulse control disorders are characterized by the **repeated failure to resist an impulse, drive, or urge** to perform an act that is rewarding to the person, at least in the short-term, despite consequences such as long-term harm either to the individual or to others; marked distress about the behavior pattern; or significant impairment in personal, family, social, educational, occupational, or other important areas of functioning.

It includes

1. **Pyromania**
2. **Kleptomania**
3. **Compulsive sexual behavior disorder**
4. **Intermittent explosive disorder**

Kleptomania is the **repeated failure** to **resist impulses** to **steal objects** that are of **no value** to the person. The individual can usually afford the object or does not need it, but it is the act of stealing itself that is the goal. The stolen objects are usually given away, **returned**, or kept hidden. The act is **not planned** and is carried out **alone**.

Pyromania is the **deliberate** setting of fires on **more than one occasion**.

Pyromania is significantly associated with the following:

- **Substance abuse disorder** (especially alcoholism).
- Affective disorders (**depression** or bipolar)
- Other impulse control disorders, such as kleptomania in female firesetters.
- Personality disturbances, such as **borderline personality disorders.**
- Attention-deficit/hyperactivity disorder and learning disabilities in childhood pyromania.

Dipsomania is an irresistible impulse to drink alcohol at a periodic intervals.

Mutilomania is an irresistible impulse to mutilate animals.

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References: Online Resource, https://icd.who.int/browse11/l-m/en#/http%3a%2f%2fid.who.int%2ficd%2fentity%2f826065555
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page No: 612, 613, 611
Textbook of Forensic Medicine & Toxicology: Principles & Practice 5 th edition, Page no: 413
The Essentials Of Forensic Medicine And Toxicology, 33rd edition, Page no : 482

Q40. Psychiatry INI-CET 2022 ID: MF7322

A 14-year-old girl presented with sudden-onset blindness for the past 4 hours. However, on history taking, it is noted that she is not concerned about it. However, she is concerned that her mother passed away recently and that she should have spent more time with her. Physical examination findings are normal. Which of the following is true about the condition?

A In children, equally among males and females.

B In adults, equally among males and females.

C In adults, occurs more in males than in females.

D ✓ In children, occurs more in females than in males.

Correct Answer: In children, occurs more in females than in males.

The above scenario is suggestive of **conversion disorder. Option D** is a true statement regarding the above condition as it is more commonly seen in **females**.

Conversion disorder is also called functional neurological symptom disorder, and there is no bodily cause involved in this condition. A conversion disorder is an **acute** and **temporary** loss or alteration in motor or sensory function triggered by psychosocial stress. Most of the patients have a history of traumatic events in childhood. The symptoms or deficits of conversion disorder are not intentionally produced but are a **symbolic representation** of an unconscious conflict (**primary gain**). It is more commonly seen in **females.**

Primary gain refers to keeping internal conflicts outside of awareness and the relief from stress. **Secondary gain** refers to the attention gained by the patient (from bystanders) as a result of being sick. If the secondary gain is intentional, then it would lead to the diagnosis of malingering. A related phenomenon is **la belle indifference**, in which the patient is **not concerned** about the serious symptoms.

Symptoms of conversion disorder:

- **Mutism, blindness, and paralysis** are the **most common** symptoms.
- Sensory system: Anesthesia, paresthesia, deafness, blindness, and tunnel vision
- Motor system: Abnormal movements, **gait disturbance** (astasia abasia), weakness, paralysis, tics, jerks, etc.
- CNS: Pseudo-seizures

Treatment: Hypnosis, anxiolytics, and behavioral relaxation exercises are effective. Psychoanalysis and insight-oriented psychotherapy help in the resolution of the symptoms.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 473-476
Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK551567/

Q41. Psychiatry INI-CET 2022 ID: MF7211

What is the dose range of lithium in mEq/L for acute attacks of mania and long-term maintenance?

A 0.5 to 1 mEq/L

B 0.1 to 0.5 mEq/L

C ✓ 0.6 to 1.5 mEq/L

D 2 to 3 mEq/L

Correct Answer: 0.6 to 1.5 mEq/L

Dose range of lithium in mEq/L for acute attacks of mania and long-term maintenance is **0.6 to 1.5 mEq/L**

Lithium has a **narrow therapeutic index (0.8 to 1.2 mEq/L)** and, therefore, requires monitoring to suspect toxicity. The toxic effects of lithium usually start manifesting when the serum level crosses 1.5 mEq/L.

The range of **1.0-1.5 mEq/L** is only favoured for **acute attacks** of mania with lower doses of **0.6-1.0 mEq/L** for **long term maintainance.**

Acute intoxication is characterized by

- Vomiting
- Profuse diarrhea
- Coarse tremors
- Ataxia
- Coma
- Convulsions.

A fine action tremor is common in patients taking lithium at therapeutic doses and can develop at any time during treatment. Therapeutic lithium tremor is generally symmetric, limited to the arms, and nonprogressive. The tremor from lithium toxicity is **coarser** and **more disabling**, may also affect the legs, and occurs with other symptoms of toxicity.

Serious effects of lithium toxicity will involve confusion, hyperreflexia, and seizures. Motor and cognitive adverse effects can become irreversible, with **persistent cerebellar tremor** being the most common.

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References: Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition, Page: 377
https://www.fda.gov/, https://www.accessdata.fda.gov/drugsatfda_docs/label/2011/017812s028,018421s027lbl.pdf
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5310104/

Q42. Psychiatry INI-CET 2022 ID: MF7212

A patient who is a chronic alcoholic presents with seizures and altered sensorium. He has been drinking for the past 10 years and takes 24 units per day. His last drink was 2 days back. He is found to have auditory hallucinations and secondary delusions. What will you use for the management of this patient?

1. Diazepam
2. Thiamine
3. Naltrexone
4. Flupenthixol depot injection

A 1,4

B 1,2,4

C 1,3,4

D ✓ 1,2

Correct Answer: 1,2

The given clinical scenario is suggestive of **delirium tremens**. Management of this patient involves the administration of **diazepam** and **thiamine.**

Delirium tremens is the most severe form of **alcohol withdrawal syndrome**. It occurs usually **within 3–4 days** of alcohol withdrawal.

Characteristic features of delirium tremens:

- **Clouding** of **consciousness**, with **disorientation** to time and place
- Poor attention and **distractibility**
- **Visual hallucinations** and **illusions**, which are often vivid and very frightening
- Autonomic disturbances like **tachycardia, fever,** hypertension, **diaphoresis,** and pupillary dilatation
- Psychomotor agitation and ataxia

Benzodiazepines such as **diazepam** (intravenously) or **chlordiazepoxide** (orally every 4 hours) are given. It is also recommended to administer **thiamine** in patients with alcohol-related disorders to prevent **Wernicke-Korsakoff syndrome**.

Acute alcohol intoxication can be treated with the evacuation of stomach and bowel and gastric lavage with an alkaline solution. Administration of 1 L of normal saline with 10% glucose, 100 mg thiamine, and 15 U insulin may be useful. If the coma progresses deeper, stimulants like strychnine and caffeine can be used. Patients should be started on artificial respiration if there is respiratory depression or difficulty breathing.

Chronic alcohol intoxication can be treated with disulfiram (daily dose of 250 mg OD), conditioned reflex treatment, hypnosis, and psychotherapy.

According to the **Motor Vehicle Amendment bill (2019)**, whoever while driving, or attempting to drive, a motor vehicle, has, in his/her blood, **alcohol exceeding 30 mg/100 mL of blood**, detected in a test by a breath analyzer, is **punishable by law.**

Naltrexone is an opioid receptor antagonist which is used along with cognitive behavioral therapy in cases of opioid withdrawal and alcohol withdrawal. It reduces craving and consumption, also reduces the severity of withdrawal symptoms.

Flupenthixol is an antipsychotic **antagonist** of both **D1 and D2 dopamine receptors.** The depot injections are used for the management of **schizophrenia** and **depression.**

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References: Harrison's Principles of Internal Medicine, 21st Edition, 3561- 3562
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, 631,632

Q43. Psychiatry INI-CET 2022 ID: MF7445

Match the following:

1. Eonism	A. Exposure of one's genitals to an unsuspecting stranger
2. Exhibitionism	B. Sexual gratification by intercourse with dead bodies
3. Frotteurism	C. Sexual pleasure by wearing clothes of the opposite sex
4. Necrophilia	D. Sexual gratification by rubbing private parts against another person

A 1-B;2-A;3-D;4-C

B ✓ 1-C;2-A;3-D;4-B

C 1-C;2-D;3-A;4-B

D 1-C;2-B;3-D;4-A

Correct Answer: 1-C;2-A;3-D;4-B

The correct answer is option B: 1-C;2-A;3-D;4-B

1. Eonism	C. Sexual pleasure by wearing clothes of the opposite sex
2. Exhibitionism	A. Exposure of one's genitals to an unsuspecting stranger
3. Frotteurism	D. Sexual gratification by rubbing private parts against another person
4. Necrophilia	B. Sexual gratification by intercourse with dead bodies

Paraphilias or perversions are sexual stimuli or acts that are deviations from normal sexual behaviors but are necessary for some people to experience arousal and orgasm.

Eonism (also known as **transvestism**) is a sexual perversion, where a person's whole personality is dominated by a desire to be identified as a member of the opposite sex. This is usually seen in males who obtain sexual pleasure by wearing female clothes. There is no hormonal/genital abnormality.

Exhibitionism is the urge to expose genitals to a stranger or unsuspecting person.

Frotteurism refers to the sexual gratification achieved by touching or rubbing against a non-consenting person. This behavior often occurs in busy and crowded places. It is predominantly described in men.

Necrophilia is an obsession with obtaining sexual gratification from dead bodies.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Pg 595 - 597
The Essentials Of Forensic Medicine And Toxicology, 33rd edition, Page no : 433

Q44. Psychiatry INI-CET 2022 ID: MF7539

Which of the following is the most appropriate definition of stress?

A External pressure

B Mental tension

C ✓ It is the body's response to anything that requires attention or action causing physical, emotional, or psychological strain.

D Internal strain

Correct Answer: It is the body's response to anything that requires attention or action causing physical, emotional, or psychological strain.

According to the WHO, **stress** can be defined as any type of change that causes **physical, emotional,** or **psychological strain.** Stress is your body's response to anything that requires attention or action. Everyone experiences stress to some degree.

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References: Online Resource, https://www.who.int/news-room/questions-and-answers/item/stress#:~:text=Stress%20can%20be%20defined%20as,to%20your%20overall%20well%2Dbeing.

Q45. Psychiatry INI-CET 2022 ID: MF7544

A pregnant mother was on SSRIs. The risk of which of the following is increased in the newborn?

A. Low APGAR score.
B. ADHD.
C. Persistent pulmonary hypertension.
D. Delayed developmental milestones.

ADHD- Attention deficit hyperactivity disorder.

A A, C
B ✓ A, B, C, D
C B, C, D
D A, B, C

Correct Answer: A, B, C, D

The newborn of a pregnant mother on SSRI has an increased risk for **A, B, C** and **D**. SSRIs like sertraline are the most commonly used antidepressants to treat **depression** in pregnant women.

SSRIs and **maternal depression** have both been found to cause a low APGAR score.

The development of **ADHD** in the newborn has been found to be commonly associated with **untreated maternal depression**. But studies regarding SSRIs having a causal association with the development of ADHD have been inconclusive. This can be contributed to maternal depression being a **confounder**.

When used in the second half of pregnancy, SSRIs have been associated with an increased risk of precipitating **persistent pulmonary hypertension** in the newborn.

There has been evidence of a **transient delay** in **developmental milestones** in patients who were exposed to SSRIs during pregnancy. Most studies have observed that the development returns to normal after some time. But if the mother is severely depressed, delayed developmental milestones have also been attributed to the mother's disinterest in the infant.

The other side effects of SSRIs include:

System	Side effects of SSRIs
Sexual	**Anorgasmia**, Decreased libido
Gastrointestinal (5-HT3 receptor)	**Nausea**, diarrhea, anorexia, vomiting, flatulence, and dyspepsia
Nervous system	Extremely **vivid dreams** or nightmares, Anxiety, Insomnia/sedation, seizures, emotional blunting
Anticholinergic	**Dry mouth**, constipation, sedation
Cardiovascular	Drug-induced long QT syndrome
Hematological	Easy bruising
Endocrine	Mammoplasia, galactorrhea

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 993-995, 1018-1019, 1042-1043
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page no 626, 627
Online Resource, https://www.uptodate.com/contents/infants-and-children-with-antenatal-exposure-to-selective-serotonin-reuptake-inhibitors-and-serotonin-norepinephrine-reuptake-inhibitors-risk-of-abnormalities-in-growth-motor-skills-and-cognition?search=ssri%20pregnancy&topicRef=5007&source=see_link
Online Resource, https://www.uptodate.com/contents/antenatal-exposure-to-selective-serotonin-reuptake-inhibitors-ssris-and-serotonin-norepinephrine-reuptake-inhibitors-snris-risk-of-psychopathology-in-the-offspring?search=adhd%20ssri&source=search_result&selectedTitle=3~150&usage_type=default&display_rank=3
Online Resource, https://pubmed.ncbi.nlm.nih.gov/28440103/
Perinatal Psychopharmacology, Page. 6, 57, 74-79,

Q46. Psychiatry NEET 2024 ID: MG6717

A patient comes with complaints of difficulty falling asleep. He consumes black coffee one hour before bed. Which of the following is the role of caffeine in the wakefulness?

A Caffeine has no effect, and he can continue taking black coffee

B Caffeine causes release of chemical from locus coeruleus that causes awakening

C Caffeine has histaminic action in brain that leads to awakening

D ✓ Caffeine inhibits adenosine receptors, thus leading to awakening

Correct Answer: Caffeine inhibits adenosine receptors, thus leading to awakening

The role of caffeine in the wakefulness in this patient is that **caffeine inhibits adenosine receptors**, thus leading to **awakening.**

Caffeine, a commonly used **stimulant**, plays a significant role in **promoting wakefulness** and **alertness**. It functions primarily as an **antagonist** at **adenosine A2A receptors** in the brain. **Adenosine**, a natural **neuromodulator** which accumulates in the basal forebrain area promotes **sleepiness** and **relaxation** by activating these receptors. By blocking adenosine A2A receptors, caffeine counteracts these sedative effects, thus helping to increase wakefulness and reduce fatigue.

In addition to its receptor antagonism, caffeine **inhibits cyclic nucleotide phosphodiesterases**, which leads to **increased** levels of **cyclic AMP** (cAMP) in the brain. This action enhances the release of neurotransmitters such as **norepinephrine (NE)** and **dopamine (DA)**, further contributing to increased neural activity and alertness. Caffeine is **rapidly absorbed** from the digestive tract and crosses the blood-brain barrier easily, affecting brain function soon after consumption.

Caffeine's stimulating effects can also be influenced by factors such as metabolism, which varies widely among individuals. For instance, **smoking** can **reduce caffeine's half-life**, making it clear from the body faster, while **oral contraceptives** can **prolong** its effects. Therefore, consuming caffeine close to bedtime, as in the case of drinking black coffee an hour before bed, can interfere with sleep by sustaining wakefulness and reducing the ability to fall asleep.

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References: Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition, Page No: 546, 685

Q47. Psychiatry NEET 2024 ID: MG6718

Which of the following SNRI is used in the treatment of ADHD?

SNRI - Selective Norepinephrine Reuptake Inhibitors

ADHD - Attention-Deficit/Hyperactivity Disorder

A Guanfacine

B Methylphenidate

C ✓ Reboxetine

D Modafinil

Correct Answer: Reboxetine

The **SNRI** that can be used for the treatment of ADHD is **Reboxetine.**

Reboxetine, a SNRI has shown **potential efficacy** in treating **ADHD**, particularly in patients who **do not respond** to **methylphenidate**. Common side effects of reboxetine include **drowsiness, sedation**, and **gastrointestinal issues**.

In the **treatment** of ADHD , **pharmacotherapy** is the **primary approach**, with **central nervous system (CNS) stimulants** being the most effective. Stimulants such as **methylphenidate** and **amphetamine salts** are commonly used due to their high efficacy in managing symptoms, although they come with risks of **dependence** and **addiction**. For patients who cannot tolerate stimulants or have a **family history** of **addiction**, **non-stimulant** options are preferred. These include **atomoxetine**, a **norepinephrine reuptake inhibitor (NRI)**, and **guanfacine**, an **alpha-2 adrenergic agonist**.

Modafinil is used primarily for **narcolepsy** and is not a standard treatment for ADHD.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page No: 1169-1178
Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition, Page No: 347
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page No: 137

Q48. Psychiatry NEET 2024 ID: MG6720

A 45-year-old woman presents with a feeling of nervousness and has abdominal pain and diarrhoea. She has had a history of heartburn, and indigestion for the past 25 years. She gives similar complaints in the family of being "tense and nervous". What other symptoms might you expect to see in this condition?

- **A** ✓ Tingling of extremities
- **B** Neologism
- **C** Idea of reference
- **D** Hallucination

Correct Answer: Tingling of extremities

In this scenario, given the patient's feelings of **nervousness**, **abdominal pain**, **diarrhoea**, and a history of long-term digestive issues along with **familial history**, you might also expect to see **tingling of extremities** as a symptom of **Generalized Anxiety Disorder (GAD)**.

According to **DSM-5** (and ICD-11) criteria, GAD is characterized by **excessive** and **uncontrollable anxiety** and worry about various aspects of life, persisting for **at least six months**. This anxiety is typically accompanied by at least three additional symptoms from a specific list, which includes:

- The presence of excessive anxiety and worry about a variety of topics, events, or activities.
- Worry occurs more often than not for at least six months and is clearly excessive.
- The worry is experienced as very challenging to control.
- The worry in both adults and children may easily shift from one topic to another.
- The anxiety and worry are accompanied by **at least three** of the following physical or cognitive symptoms (In children, only one of these symptoms is necessary for a diagnosis of GAD):
  - **Edginess** or **restlessness**
  - **Tiring easily**; more fatigued than usual impaired concentration or feeling as though the mind goes blank
  - **Irritability** (which may or may not be observable to others)
  - **Increased muscle aches** or soreness
  - **Difficulty sleeping** (due to trouble falling asleep or staying asleep, restlessness at night, or unsatisfying sleep)

Neologism, idea of reference, and **hallucinations** are not typical symptoms of GAD but are more associated with other psychiatric conditions such as **psychotic disorders** like **schizophrenia**.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page : 196
Online Resource, https://www.carepatron.com/templates/dsm-5-generalized-anxiety-disorder-checklist

Q49. Psychiatry NEET 2024 ID: MG7182

A girl was referred to the psychiatry department by the plastic surgeon. She had visited multiple plastic surgery departments requesting full facial reconstruction. She complains of dark blemishes and spots on her face, which are not present. She always says that she is ugly and looks constantly in the mirror and the back of the spoon and windows. Her mother assured her of no such issues, but still, she persists in getting the surgery. What should the psychiatrist do?

A Start on atypical antipsychotics

B Give fitness for surgery

C ✓ Start her on selective serotonin reuptake inhibitors

D Initiate insight-oriented psychotherapy

Correct Answer: Start her on selective serotonin reuptake inhibitors

The given history suggests the presence of **body dysmorphic disorder (BDD)** in the patient, as she is preoccupied with her appearance and self-perceived flaws, despite being told otherwise by her family. The psychiatrist should start her on **selective serotonin reuptake inhibitors(SSRI).**

BDD is characterized by **persistent preoccupations** about one or more **perceived defects** or **flaws** in **one's appearance,** but the defects or flaws are negligible or not even observable to others. In response to their appearance concerns, individuals with BDD demonstrate a range of mental acts:

- Comparing themselves with others
- **Checking** in the mirror
- Camouflaging their perceived flaws
- Avoidance- from minor social avoidance to being housebound.

The **specific focus** of BDD **on appearance** serves to differentiate it from other conditions characterized by repetitive behaviours.

The treatment for BDD typically involves a combination of **SSRIs** and **psychotherapy**. SSRIs, such as **fluoxetine** and **clomipramine**, have been shown to reduce symptoms in many patients, helping **alleviate** the **obsessive thoughts** and **anxiety** associated with BDD.

Psychotherapy, particularly **cognitive-behavioural therapy (CBT)**, is crucial as it addresses the distorted self-image and underlying emotional issues. CBT helps patients reframe their thoughts about appearance and develop healthier coping strategies.

It's important to note that while **surgical interventions** may be sought by patients, they are often **ineffective** and can lead to further dissatisfaction and distress. Therefore, focusing on mental health treatment is essential. Regular follow-ups and a supportive therapeutic relationship can significantly enhance the effectiveness of treatment.

Other options:

Option A: **Start on atypical antipsychotics:** Antipsychotics are generally not the first-line treatment for BDD. They may be considered in cases with comorbid psychotic symptoms, but this patient does not exhibit hallucinations or delusions.

Option B: **Give fitness for surgery:** Giving fitness for surgery would be inappropriate and potentially harmful. Surgery is not the solution for BDD, as patients typically remain dissatisfied with the results and continue seeking more surgeries.

Option D: **Initiate insight-oriented psychotherapy:** While psychotherapy, particularly cognitive-behavioral therapy (CBT), is effective for BDD, insight-oriented psychotherapy (focused on understanding unconscious conflicts) is less effective than targeted approaches like CBT in managing this condition.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 426-429
Kaplan and Sadock's Comprehensive Textbook of Psychiatry - Ebook - 10th Edition, Page no 4593, 4589

Q50. Psychiatry NEET 2024 ID: MG7181

A girl presented with multiple episodes of loss of consciousness. The episodes usually happen in the presence of the family members in the morning. She regains consciousness after 15 to 30 minutes and usually remembers the events during these episodes. There is no history of tongue bite or urinary incontinence. What should be done in this scenario?

- **A** ✓ Start behavioural therapy
- **B** Initiate aversion therapy
- **C** Give valproate
- **D** Start on ketogenic diet

Correct Answer: Start behavioural therapy

In this scenario, the girl is experiencing episodes of loss of consciousness that may suggest **conversion disorder**. The most appropriate **management** is to start **behavioural therapy** to help address her symptoms and underlying stressors.

Conversion disorder, also referred to as **functional neurological symptom disorder** in the DSM-5, involves symptoms or deficits that affect **voluntary motor** or **sensory functions**, **suggesting** another **medical condition**. However, these symptoms are believed to arise from **psychological factors**, often triggered by conflicts or **stressors**. The symptoms are **not intentionally** produced, are not the result of substance use, and are not limited to pain or sexual issues. The **primary gain** associated with conversion disorder is **psychological** rather than social, monetary, or legal.

Common **presenting symptoms** include:

- **Non-epileptic seizures**
- Weakness and/or **paralysis**
- Movement disorders
- Speech or visual impairment
- Swallowing difficulty
- Sensory disturbances
- Cognitive symptoms

Resolution of conversion disorder symptoms often occurs **spontaneously** but can be enhanced through supportive or behavioural therapy. For patients who are resistant to psychotherapy, it can be beneficial to focus on **stress** and **coping strategies** rather than directly addressing their symptoms as imaginary, as this may exacerbate their condition.

Additional techniques, such as **hypnosis, anxiolytics,** and **behavioural relaxation exercises**, may also be effective in managing symptoms. In certain cases, medications like **parenteral amobarbital** or **lorazepam** can help gather further information, especially if the patient has experienced a recent traumatic event.

Psychodynamic approaches, including **psychoanalysis** and **insight-oriented psychotherapy**, allow patients to explore their **internal conflicts** and the **symbolic meanings** of their symptoms.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 473

Q51. Psychiatry NEET 2024 ID: MG7180

If a child with ADHD goes untreated into adolescence, what is he most likely to develop?

Attention deficit hyperactivity disorder- ADHD

A Selective mutism

B Separation anxiety disorder

C ✓ Conduct disorder

D Binge eating disorder

Correct Answer: Conduct disorder

If a child with **ADHD** goes **untreated** into adolescence, among the following options he is most likely to develop **conduct disorder**.

The course of ADHD is variable. The majority of the cases **persist** into adult life. The hyperactivity may sometimes disappear, but the **decreased attention span** and **impulse-control problems** usually **persist**. Patients with ADHD are vulnerable to **conduct disorder**, **antisocial behaviour**, substance use disorders, and mood disorders.

ADHD is a **neuro psychiatric condition** characterized by a pattern of **diminished attention span and increased impulsivity or hyperactivity.** The **prefrontal cortex** of the brain has been implicated in its etiology and **dopamine** is the main neurotransmitter.

ADHD can begin during **infancy** but is usually recognized between **1-3 years** of age. ADHD was known as minimal brain dysfunction earlier. The characteristics of children with ADHD, in order of frequency, are

- **Hyperactivity**
- **Attention deficit** (short attention span, distractibility)
- **Impulsivity** (action before thought, abrupt shifts in activity, lack of organization)
- Memory and thinking deficits
- Specific learning disabilities
- Speech and hearing deficits.

According to **DSM-5** criteria, a diagnosis of ADHD requires symptoms to be present **before age 12**, persist for at least **six months**, **negatively impact** social, academic, or occupational activities, and occur in two or more settings. **Pharmacotherapy** is the **first-line treatment**, with CNS stimulants like **methylphenidate** and **amphetamine** salts being the most effective due to their mild side effects. Non-stimulant options include atomoxetine, clonidine, and guanfacine. Additionally, **psychosocial interventions**—such as academic organization skills, parent training, behaviour modification, cognitive-behavioural therapy (CBT), and social skills training—are used as adjuncts to medication.

Note: **Conduct disorder** is characterized by a **persistent pattern** of **behaviour** that violates the basic rights of others and **disregards societal rules**. The severity of the behaviours can lead to emergencies, especially if they are alarming to parents. Individuals with severe and chronic symptoms may later be diagnosed with **antisocial personality disorder** after age 18. Treatment typically involves **behaviour modification** strategies and, if necessary, placement in a corrective institution.

Other options:

Option A: **Selective mutism** refers to the **lack of speaking** (being mute) in one or **more specific social situations**, commonly in school, but speaking well in other situations, commonly at home. The child may be either **completely silent** at school or maybe just **whispering**. Rarely, a child may be silent at home but communicate normally at school.

Option B: **Separation anxiety disorder** is characterized by **developmentally inappropriate** and **excessive anxiety** when the patient is separated from their attachment figure. It is a normal developmental phenomenon in infants. It peaks between **9 months** and 18 months and usually **diminishes** by **two and a half years of age**.

Option D: **Binge-eating disorder** is the **most common eating disorder**. It is characterized by **rapid intake** of abnormally **large quantities** of food over a short period of time, even when not hungry. There are feelings of **guilt** about the eating episodes, but these patients do not compensate in any way following the episodes of binge.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no. 1169-1178, 787

Q52. Psychiatry NEET 2024 ID: MF7161

A chronic alcoholic met with a vehicular accident. His blood alcohol level is >300 mg/dl. He needs hospitalization and will be withheld from taking alcohol. The drug that will be used to manage his withdrawal symptoms is:

A Methyl alcohol

B Fomepizole

C ✓ Lorazepam

D Buspirone

Correct Answer: Lorazepam

The first-line drug used in the management of this patient's withdrawal symptoms is **intravenous(IV) lorazepam.**

Delirium tremens is the most severe form of alcohol withdrawal syndrome. It occurs usually within **3–4 days** of alcohol withdrawal. **Acute infection,** hepatitis and pancreatitis predispose delirium tremens.

Clinically, patients present with confusion, agitation, and fluctuating levels of consciousness. **Visual** or **tactile hallucinations** along with features of autonomic hyperactivity like **tachycardia,** diaphoresis, **fever**, insomnia, and **hypertension** are also seen.

Delirium tremens is treated using benzodiazepines, starting with a higher dosage, which is gradually tapered as withdrawal symptoms disappear. Benzodiazepines such as **IV lorazepam** or **oral chlordiazepoxide** (every 4 hours) are given.

In the case of **benzodiazepine-resistant** cases, **IV phenobarbital** is the first-line drug, and **IV propofol** is the second-line drug.

It is also recommended to administer **thiamine** in patients with alcohol-related disorders to **prevent Wernicke-Korsakoff** syndrome

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 629, 631, 632
Harrison's Principles of Internal Medicine, 21st Edition, Page no. 2273, 2274, 3561
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6286444/

Q53. Psychiatry NEET 2024 ID: MG7178

A 19-year-old girl child came agitated to the emergency department after consuming some drug with a previous history of suicidal tendencies. The parents do not know the mode and route of the drug administered. She presented with tachycardia, tachypnea, diaphoresis, and dilated pupils. What is the likely drug administered?

A Chlorpheniramine

B Morphine

C Heroin

D ✓ Cocaine

Correct Answer: Cocaine

In the given clinical scenario, the patient presenting with **tachycardia, tachypnea, diaphoresis**, and **dilated pupils** after consuming some drug with a **previous history** of **suicidal tendencies** points towards a diagnosis of **cocaine** toxicity leading to **sympathetic toxidrome**.

Cocaine is a **CNS stimulant** obtained from coca plants belonging to the family *Erythroxylaceae*. The route of intake is by **snorting**, IV route and smoking. Tattoos may be made to cover the injection marks.

It **inhibits** the uptake of **dopamine, norepinephrine**, and **serotonin**.

At lower doses, it leads to alertness but at higher doses, it produces stereotyped behavior, involuntary actions, and paranoia. There is a **transient euphoria**. Tactile hallucinations (**Magnan's symptom** or **cocaine bugs**) are characteristic.

There is a dose-dependent **increase in heart rate** and **blood pressure**. It causes **dilation of pupil**s and is associated with chills and perspiration. In severe cases, **intracranial bleeding** due to increased blood pressure, and **myocardial infarction** due to coronary vasospasm can occur. Ultimately, the patient goes into a stage of depression with loss of reflexes, paralysis of muscles, cardiorespiratory failure, and coma.

Management - Administration of activated charcoal and gastric lavage with warm water and potassium permanganate in case of ingestion. **Benzodiazepines** to control heart rate/seizures. **Amyl nitrite** is the **antidote** given via inhalation. **Thiamine 100 mg IV** and **Naloxone 2 mg IV** can also be given.

Other options:

Option A: **Chlorpheniramine** is an effective **H1 antagonist** that tends to be mildly sedative, making it suitable for daytime use. A notable number of patients experience side effects involving central nervous system stimulation.

Option B and C: **Morphine** and **Heroin** are isolated from **opium**. The clinical features include a **euphoric feeling** in the stage of excitement followed by **headache, giddiness,** and a tendency to sleep in the stage of **stupor**. Intoxication is characterized by **bradycardia, hypotension**, **respiratory depression, hypothermia,** and **pinpoint pupils**. Respiratory depression can even result in death.

The image below shows the ***Erythroxylum*** **plant:**



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References: The Essentials Of Forensic Medicine And Toxicology, 33rd edition, page no: 602-604
Online Resource, https://emedicine.medscape.com/article/813959-clinical#b4
Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition, page no: 865

Q54. Psychiatry NEET 2024 ID: MG7179

A 19-year-old presented to the psychiatry OPD. He has a charming persona and is well-composed and behaved in front of interviewers. His mother complains of him getting into fights and bunking classes, drinking alcohol and getting involved in police cases. What kind of personality disorder does he have?

A Paranoid

B ✓ Antisocial

C Narcissistic

D Schizotypal

Correct Answer: Antisocial

The patient most likely has **antisocial personality disorder**. Although the patient seems **charming** and **well-composed**, he is **aggressive** and clearly exhibits **disregard** for **social norms,** and shows **no remorse** for his actions. These findings are characteristic of antisocial personality disorder.

Antisocial Personality Disorder is characterized by a **long-term pattern** of **manipulation, exploitation**, or **violation** of others' rights. Individuals with antisocial personality disorder tend to show **no regard** for right or wrong and often disregard the feelings, rights, and safety of others.

Other options:

Option A: **Paranoid** personality disorder is characterized by **unwarranted suspicion,** hypersensitivity, jealousy, envy, rigidity, excessive self-importance, and a tendency to blame and ascribe **evil motives** to others.

Option C: **Narcissistic** personality disorder is marked by a **heightened** sense of **self-importance**. Individuals with this personality disorder display a **lack** of **empathy** and grandiose feelings. This is to mask their fragile self-esteem and vulnerability.

Option D: Individuals with **schizotypal** personality disorder appear **strange** to surrounding people. They have **ideas of reference, odd** or **magical thinking**, inappropriate or constricted affect, and suspiciousness. It is most **commonly** associated with **depression** in nearly half of the patients.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page no 926, 1767, 1804-1806
Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 609, 749

Q55. Psychiatry NEET 2020 ID: MD0862

A 25-year-old male presented to the clinic with a complaint of the feeling of insects crawling under the skin. Which of the following drug abuse can cause the symptoms of this patient?

A Cannabis

B ✓ Cocaine

C Amphetamine

D Alcohol

Correct Answer: Cocaine

The feeling of insects crawling under the skin, also known as **cocaine bugs** or **formication**, is a feature of **cocaine** abuse.

Other features of acute cocaine intoxication are delusions of persecution, paranoia, and suspiciousness. It is essential to differentiate acute cocaine intoxication from acute schizophrenia which it closely resembles.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no: 676

Q56. Psychiatry NEET 2020 ID: MA6682

A 60-year-old male who lost his wife 3 months back complains that his intestines have become rotten. He feels that he is responsible for his wife's death and should be sent to prison. He complains of feeling low all the time and has lost interest in daily activities since his wife's death. What is the most likely diagnosis?

A Normal grief reaction

B ✓ Psychotic depression

C Delusional disorder

D Schizophrenia

Correct Answer: Psychotic depression

The given clinical scenario of persistent low mood, anhedonia, guilt, and self-critical thought content along with **nihilistic delusion** (rotten intestine), points towards a diagnosis of **major depression with psychotic features**.

Differentiating features of grief and depression have been enlisted in the following table:

Grief	Depression
Feelings of emptyness and loss	Persistent low mood and inability to anticipate pleasure
Dysphoria comes in waves and tends to decrease in intentsity over time	Low mood tends to be persistent and consistent
Associated with thoughts and reminders of deceased/loss	Not tied to specific thoughts and pre-occupations
May be present along with positive emotions and humor	Pervassive unhappiness and misery
Thought content associated with thoughts and memories of deceased/loss	Thought content associated with self-critical and pessimistic ruminations
Self-esteem is generally preserved	Worthlessness and self-loathing is common
Ideas of death focused on joining the deceased	Ideas of death focused on ending life due to hopelessness

Note: Tricyclic antidepressants are considered to be the first-line drugs for psychotic depression.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 366
DSM-5 (R) Guidebook: The Essential Companion to the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Page no 161
The Maudsley Prescribing Guidelines in Psychiatry 13th Edition, Page no 279

Q57. Psychiatry NEET 2020 ID: MC8645

What is the treatment of choice for postpartum blues?

A Fluoxetine

B Cognitive behavioural therapy

C Lithium carbonate

D ✓ Support and education

Correct Answer: Support and education

Postpartum blues is common in mothers in the puerperal period and requires nothing more than **education and support**. It is characterised by a labile mood, sadness and subjective confusion. It is thought to be due to changes in hormone levels, the stress of childbirth and anxiety about the responsibilities of motherhood.

If symptoms last longer than 2 weeks, an evaluation for **postpartum depression** is needed.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 839

Q58. Psychiatry NEET 2020 ID: MD0754

A 25-year-old woman complains of a creepy crawling sensation in her legs. It is more at night and prevents her from sleeping. She is relieved of these symptoms by either walking or moving her legs. Which of the following drugs is used in treating the condition?

A ✓ Gabapentin

B Pramipexole

C Vitamin B12

D Iron tablets

Correct Answer: Gabapentin

The history is suggestive of **restless leg syndrome** and **gabapentin** is the first-line choice of treatment for this condition.

Gabapentinoids are considered superior to dopamine agonists due to the high frequency of intermediate and long-term augmentation with dopamine agonists, as well as their risk of impulse control disorders.

Restless legs syndrome (RLS) is an irresistible urge to move the legs when at rest or while trying to fall asleep or an uncomfortable sensation of creeping-crawling feeling. It is worse at night and moving legs or walking reduces discomfort. Uremia, neuropathies, as well as iron-deficiency and folic acid–deficiency anemias can produce secondary RLS.

In patients with restless legs syndrome, **iron stores** are initially repleted, and the aggravating factors are eliminated. If RLS still persists, initial therapy with a **gabapentinoid** (pregabalin, gabapentin, gabapentin enacarbil) is preferred, rather than a dopamine agonist (pramipexole, ropinirole, rotigotine).

FDA-approved drugs for the **treatment** of restless leg syndrome:

- Gabapentin
- Ropinirole
- Pramipexole
- Rotigotine

Note: A **dopamine agonist** is used as an alternative in patients with increased risk for side effects from gabapentinoids, such as those with obesity and its complications, a past or present history of moderate to severe depression, a disorder causing gait instability or respiratory failure, and a history of substance use disorder.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page no: 501, 502, 508, 598, 1638
Harrison's Principles of Internal Medicine, 21st Edition, Page no: 208
Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition, Page no 528
Online Resource, https://www.uptodate.com/contents/management-of-restless-legs-syndrome-and-periodic-limb-movement-disorder-in-adults?search=restless%20leg%20syndrome%20doc&source=search_result&selectedTitle=4~150&usage_type=default&display_rank=4#H1664003

Q59. Psychiatry NEET 2020 ID: MD0861

A first-year MBBS student came to OPD with intermittent episodes of a feeling of impending doom accompanied by intense perspiration. Usually, the episodes occur prior to her exams. What is the most likely diagnosis?

A ✓ Panic disorder

B Conversion disorder

C Social anxiety disorder

D Generalized anxiety disorder

Correct Answer: Panic disorder

The most likely diagnosis for this patient is **panic disorder**.

A **panic attack** is an abrupt surge of **intense fear** or intense discomfort. Recurrent panic attacks for more than one month is classified as **panic disorder.**

Panic attacks can occur in mental disorders other than panic disorder, particularly in specific phobia, social phobia, and PTSD.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no. 394

Q60. Psychiatry NEET 2020 ID: MD0679

What is the maximum period of voluntary self-admission under the Mental Healthcare Act 2017?

A 48 hours

B 7 days

C ✓ 30 days

D 90 days

Correct Answer: 30 days

The maximum period of **voluntary self-admission** is **30 days.**

When a person has a mental illness that requires admission, and if he/she is able to make his or her own decision and voluntarily requested admission, then it is called voluntary self-admission.

According to the Mental Healthcare Act, the **maximum duration** for which such admission can last is **30 days.**

If the admission **requires more** than 30 days, or if it requires **readmission** within 7 days after discharge, then such patients should be examined by two psychiatrists and can be admitted for a maximum period of **90 days.**

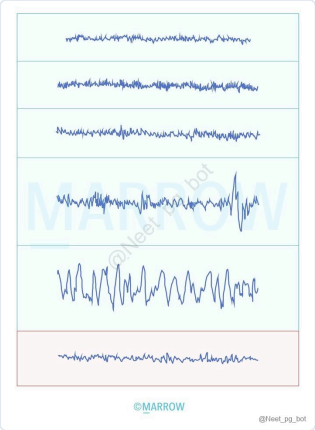
Note: The aforementioned question is as per the 2020 recall. This concept has been updated, and the admissions are classified as independent and supported. Please refer to the MCQ MG6497 for recent information.

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References: Online Resource, http://www.amhonline.org/text.asp?2018/19/1/9/235323

Q61. Psychiatry NEET 2020 ID: MD0756

Identify the stage of sleep based on EEG finding highlighted in red.



A Stage 1

B Stage 2

C Stage 3

D ✓ REM sleep

Correct Answer: REM sleep

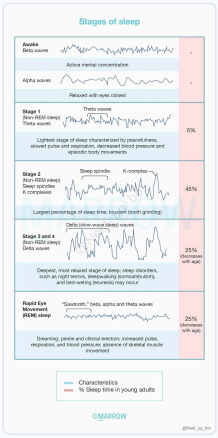
The marked area in the given EEG shows **sawtooth waves** that are characteristic of **REM sleep**.

Sleep is comprised of two distinct physiological states, **non-rapid eye movement** (NREM) sleep and REM sleep. **Stages I** to **IV** make up the NREM sleep with marked **lower physiological functions** than during wakefulness. Dreams which occur during this state are usually **lucid** and purposeful.

REM sleep is characterized by a **high level** of **brain activity**. The physiological activity is similar to that during wakefulness. In REM sleep,

- Pulse, respiration, and blood pressure are increased.
- Brain oxygen use increases during REM sleep.
- Thermoregulation is altered - Poikilothermia prevails during REM sleep.
- There is loss of muscle tone - body movements are absent during REM sleep.
- There is partial or full penile erection - This is used to evaluate the cause for **impotence**.
- **Dreams** are abstract and surreal.

In normal adults, a REM episode has a **latency** of about **90 minutes** during which the sleep is in the NREM stage. This latency is shortened in **narcolepsy** and **depressive disorders**.



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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no. 533, 534, 535

Q62. Psychiatry NEET 2020 ID: MD0680

A middle-aged female patient presents with a history of loss of interest in day to day activities, weight loss, insomnia, and sadness for six months. She also recalls hearing a voice once that ordered her to kill herself. What is the DSM-V diagnosis?

A ✓ Major depressive disorder with psychotic features

B Schizophrenia

C Schizoaffective disorder

D Delusional disorder

Correct Answer: Major depressive disorder with psychotic features

Since the patient fulfills the DSM-V criteria for major depressive disorder and has had an episode of auditory hallucination, a diagnosis of **major depressive disorder with psychotic features** is made.

Diagnostic criteria for depression:

1. Depressed mood most of the day
2. Anhedonia- diminished interest in day to day activities
3. Significant weight loss or weight gain. Change in appetite.
4. Insomnia or hypersomnia
5. Psychomotor agitation or retardation
6. Fatigue or loss of energy
7. Feelings of worthlessness or excessive, inappropriate guilt
8. Diminished ability to think or concentrate, indecisiveness
9. Recurrent thoughts of death, suicidal ideation

At least **five out of nine** symptoms must be present over a two week period and **one** symptom **must be depressed mood or anhedonia.**

In major depressive disorder with psychotic features, **delusions and/or hallucinations** are present along with the depression symptoms.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page nos 357 and 363

Q63. Psychiatry NEET 2025 ID: MH0160

1. A 21-year-old female presents with amnesia for an event that occurred 2 weeks ago (the death of her father in an RTA, which she witnessed). She reports having flashbacks of the event repeatedly. What is the likely diagnosis?

A Post traumatic stress disorder

B Adjustment disorder

C Dissociative disorder

D ✓ Acute stress disorder

Correct Answer: Acute stress disorder

The likely diagnosis is Acute stress disorder. Acute stress disorder is characterised by a relationship between an exceptional stressor (such as the death of a loved one, natural catastrophe, accident, or rape) and the onset of symptoms. It is characterised by the development of transient emotional, somatic, cognitive, or behavioural symptoms as a result of exposure to an extremely threatening or horrific nature. Post-traumatic stress disorder (PTSD) and acute stress disorder both present with the same set of symptoms: * Intrusive symptoms such as recurrent nightmares and flashbacks of the event, psychological distress due to stimuli that are reminders of prior trauma * Avoidance of external reminders of trauma * Negative mood or cognition related to the trauma * Altered level of arousal, such as sleep disturbances and hypervigilance A diagnosis of PTSD is made if these symptoms present ≥1 month following the traumatic event. If these symptoms are present for a duration of up to 1 month, then a diagnosis of acute stress disorder is made.

Pearl PM2336: Differential diagnosis of Trauma and Stress related disorder

	PTSD	Adjustment disorder	Bereavement
Stressor	Extreme traumatic events like rape or accident	Financial/ Illness/ Relationship issues	Death of a loved one
Time	Symptoms **develop within 6 months** and persist more than 1 month	Symptoms **develop within 3 months** and resolve in 6 months if stressor eliminated	Symptoms **develop within 2 months** and resolve within 2 months
Symptoms	Flashbacks and nightmares	Mostly depressive and anxiety	Mostly depressive

Pearl PM0378: Dissociative Disorders DSM-5 Classification and characteristics of dissociative disorders:

Dissociative disorder	Characteristic features
Dissociative amnesia: **Failure to remember** important information about oneself after a stressful event.	Amnesia usually resolves in minutes or days but may last years. Retrograde amnesia > antegrade amnesia
Dissociative fugue: Amnesia combined with **sudden wandering** from home after a stressful event.	Adoption of a different identity. The patient denies memory loss during the fugue state.
Dissociative identity disorder: Previously called **multiple personality disorder.** At least two distinct personalities (“alters”) in an individual.	More common in women (particularly those sexually abused in childhood) In a forensic setting (Eg: jail), malingering and alcohol abuse must be considered and excluded
Depersonalization disorder: Recurrent, persistent feelings of **detachment from one’s own body**, the social situation, or the environment (derealisation) when stressed.	Understanding that these perceptions are only feelings, i.e., normal reality testing.
Dissociative disorder not otherwise specified: Dissociate symptom (Eg: trance-like state, memory loss) in -	• Persons exposed to intense coercive persuasion (Eg: brainwashing) • Indigenous to particular locations or cultures (Eg: Amok in Indonesia)

ICD-11 classifies Dissociative Disorders as: * Dissociative Identity Disorder * Partial Dissociative Identity Disorder * Depersonalization-Derealization disorder * Dissociative Amnesia * Trance Disorder * Possession Trance Disorder * Dissociative Neurological symptom disorder * Secondary Dissociative syndrome

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References: Tasman's Psychiatry - 5th Edition • Page no 1480

Q64. Psychiatry NEET 2025 ID: MH0155

2. A schizophrenic patient responding well on haloperidol, for the past 2 years, presented with orofacial dyskinesia, choreiform, tic-like movements and possible dystonia. What is the probable diagnosis and treatment?

A ✓ Tardive dyskinesia - Valbenazine

B Acute dystonia - Ropinirole

C Akathisia - Propanolol

D Oral tremor - Amantadine

Correct Answer: Tardive dyskinesia - Valbenazine

The given clinical scenario of a schizophrenic patient responding well to haloperidol for the past 2 years, presenting with orofacial dyskinesia, choreiform, tic-like movements, and possible dystonia, suggests the probable diagnosis to be Tardive dyskinesia, which is treated with Valbenazine. Tardive dyskinesia occurs due to dopaminergic (D2) receptor supersensitivity. It presents with oro-facial dyskinesia after months to years of treatment with anti-psychotics. It can be treated with VMAT-2 inhibitors, such as Valbenazine and Deutetrabenazine. Other options: Option B: Acute dystonia is characterised by spasms of the muscles of the tongue, face, neck and back. It occurs within 1-5 days of the intake of antipsychotics Option C: Akathisia consists of motor restlessness and occurs within 5-60 days of the intake of antipsychotics. Option D: Oral tremor is seen in Rabbit syndrome. Rabbit syndrome is an antipsychotic-induced rhythmic motion of the mouth/lips resembling the chewing movements of a rabbit. It occurs months to years after the intake of antipsychotics and is treated with Antiparkinsonian drugs. Pearl PM0479: Extrapyramidal side effects of antipsychotic drugs

Adverse reactions	Time of onset	Features	Treatment
Acute dystonia	1 to 5 days	Spasm of muscles of tongue, face, neck, back	Diphenhydramine or benztropine
Akathisia	5 to 60 days	Restlessness	**Propranolol**, lorazepam, diphenhydramine, benztropine
Parkinsonism	5 to 30 days	Bradykinesia, rigidity, tremors	Dose reduction, change medication, antiparkinsonian agents
Tardive dyskinesia	Months to years	Orofacial dyskinesia (may be reversible with early recognition and drug discontinuation)	VMAT-2 inhibitors **valbenazine** and **deutetrabenazine**
Neuroleptic malignant syndrome	Weeks to months	Extreme rigidity, fever, unstable blood pressure	**Dantrolene**

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page no 178, 179
Online Resource • https://pubmed.ncbi.nlm.nih.gov/15015902/

Q65. Psychiatry NEET 2025 ID: MH0501

3. A 68-year-old female with breast cancer with metastasis presented with breathlessness and poor health. The patient is awake and anxious. The oncology team believes that aggressive chemotherapy and radiotherapy will not be helpful. However, her daughter insists on 'trying everything', including ICU stay and mechanical ventilation. How should a doctor handle such a scenario?

- **A ✓ Convene a family meeting to discuss care goals and clarify the patient's preferences**
- B Shift to ICU and start aggressive treatment
- C Explain no futher treatment is possible and discharge the patient to hospice
- D Start a placebo therapy to relieve anxiety

Correct Answer: Convene a family meeting to discuss care goals and clarify the patient's preferences

In this case, the best way to manage the situation is to convene a family meeting to discuss care goals and clarify the patient's preferences. Here, the patient is suffering from an advanced illness with a poor prognosis. The patient and her family are facing decisions regarding aggressive interventions such as chemotherapy, radiotherapy, and mechanical ventilation. The recommended approach is to organize an early and structured family meeting. During this meeting, the patient's preferences should be identified, and goals should be set in collaboration with the family. This helps to reduce unrealistic expectations and may lead to shorter ICU stays when the outcome is likely to be poor. A 7-step protocol can be followed to decide upon the patient's goals for care, ensuring high-quality palliative and end-of-life care: * Collect Comprehensive Information: Make sure all medical details are complete, accurate, and clearly communicated to the patient, family, and healthcare team. * Understand Patient and Family Hopes: Explore the hopes of the patient and family, and also identify the outcomes that are realistic and achievable. * Present All Care Options: Share every possible intervention, explaining the benefits, limitations, and potential outcomes of each. * Respond with Empathy: Support the patient and family emotionally as they process the situation and adjust expectations. * Formulate a Practical Plan: Create a care plan that focuses on attaining the realistic goals aligned with the patient's values. * Implement the Plan: Ensure the plan is carried out consistently, coordinating with all members of the care team. * Review and Reassess Regularly: Revisit the plan during each encounter to determine whether goals need adjustment based on the patient's evolving condition or preferences.

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References: Harrison's Principles of Internal Medicine, 22nd Edition • Page no. 77
Online Resource • Oxford Textbook of Communication in Oncology and Palliative Care - 2nd Edition - Page no. 72

Q66. Psychiatry NEET 2025 ID: MH0157

4. A friend of Mr.K, an MBA student, describes him as being focused on rules and being very rigid. He often delays submission of assignments due to wanting them to be perfect. He also reports that Mr K keeps items that are worn out even if they don't hold any sentimental value. Mr K thinks himself to have high moral values. The friend describes that Mr K was often called 'rigid' during adolescence. When confronted with these facts, Mr K denies them outright. What is the most likely personality disorder?

A ✓ Obsessive compulsive personality disorder

B Dependent personality disorder

C Narcissistic personality disorder

D Paranoid personality disorder

Correct Answer: Obsessive compulsive personality disorder

The most likely personality disorder is obsessive-compulsive personality disorder (OCPD), also known as Anankastic personality disorder. According to ICD-10, the diagnostic guidelines for OCPD require the presence of at least three of eight traits or behaviours given below. These include feelings of * Excessive doubt * Preoccupation with details * Perfectionism that interferes with task completion * Excessive conscientiousness * Excessive pedantry * Adherence to social conventions * Rigidity and stubbornness * Unreasonable insistence that others submit to exactly their way of doing things * Intrusion of insistent and unwelcome thoughts or impulses This disorder is more often seen in males, and major depressive episodes are frequent in such patients. Psychodynamically, this disorder is believed to result from fixation at the anal sadistic phase, with reaction formation as a defense mechanism. These patients are usually treated with psychotherapy. Other options: Option B: Dependent personality disorder requires the presence of at least three of six traits or behaviors, according to ICD-10, which include * Allowing others to make decisions for them * Subordination of one's own needs to those of others and undue compliance with their wishes * Unwillingness to make even reasonable demands on the people * Feeling uncomfortable or helpless when alone * Preoccupation with fears of being abandoned by a person with whom one has a close relationship * Difficulty in taking even the usual decisions without consultation or support from others Option C: Narcissistic personality trait is characterized by ideas of grandiosity and an inflated sense of self-importance. The person has an attention-seeking, dramatic behavior, needs constant praise, and is unable to face criticism. Option D: Paranoid personality disorder requires the presence of at least three out of seven traits or behaviors according to ICD-10. These traits include excessive sensitiveness, a tendency to persistently bear grudges, significant suspiciousness, a combative and tenacious sense of personal right, repeatedly doubting the partner, a tendency to experience excessive self-importance, and preoccupation with unsubstantiated conspiratorial explanations of events. Pearl PM0379: Personality Disorders

Key features of Personality disorders

Cluster A (Odd, Weird, Eccentric)

Paranoid	Suspicious, distrustful, hypervigilant
Schizoid	Prefers to be loner, detached, unemotional
Schizotypal	Eccentric & odd thoughts, perceptions & behavior, magical thinking and beliefs

Cluster B (Dramatic, Emotional and Erratic)

Antisocial	Disregard for and violation of rights of others, illegal activities
Borderline	Chaotic relationships, sensitivity to abandonment, labile mood, impulsivity, inner emptiness, self-harm
Histrionic	Dramatic, superficial, attention-seeking
Narcissistic	Grandiosity, lack of empathy

Cluster C (Anxious, Fearful, Introvert)

Avoidant	Avoidance due to fears of criticism and rejection
Dependent	Submissive, clingy, needs to be taken care of
Obsessive-compulsive/ Anankastic	Rigid, controlling, perfectionist

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page no 114, 116, 118

Q67. Psychiatry NEET 2025 ID: MH0161

5. A 30-year-old male reports that whenever he is outside, the birds communicate to him through flapping of wings and chirping. He believes that God sends him messages through the birds, guiding him to safety. These features are characteristic of :

- **A ✓ Delusional perception**
- B Sudden delusional idea
- C Visual hallucinations
- D Delusional memory

Correct Answer: Delusional perception

These features are characteristic of Delusional perception. Delusional perception means a normal perception that has been given a private and illogical meaning. It is a first-rank symptom of Schizophrenia. The first rank symptoms of schizophrenia described by Kurt Schneider include : * Hallucination: * * First-person hallucination * Second-person hallucination (voices directly addressing the patient) * Third-person hallucination (voices speaking about the patient) * Thought phenomenon: * * Thought withdrawal * Thought insertion * Thought broadcasting * Made phenomenon: * * Made affect * Made impulses or drives * Made volitional acts * Delusional perception * Somatic passivity Other options: Option B: A sudden delusional idea is a false, fixed belief about an idea. Option C: Visual hallucinations is a visual perception of something that is not actually there. Option D: Delusional memory is a false, fixed belief about a memory.

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page no 55

Q68. Psychiatry NEET 2025 ID: MH0156

6. A 35-year-old man is brought to the psychiatry emergency with bizarre symptoms. A social worker reports that the man lives in a nearby village that was recently hit by an earthquake. The man appears confused and does not know how he travelled the 110 km to this city. He is unable to recollect his personal details and has no history of any substance abuse or head trauma. The family members are worried about the patient. What is the most likely diagnosis?

A ✓ Dissociative fugue

B Dissociative amnesia

C Transient global amnesia

D Dissociative identity disorder

Correct Answer: Dissociative fugue

The most likely diagnosis from the given clinical scenario is dissociative fugue. Dissociative fugue is characterised by sudden episodes of travelling away (usually away from home). The person usually adopts a new identity during the episode, with complete amnesia for the previous life. It usually occurs due to severe stress. The termination is abrupt and is followed by amnesia for the episode, but the patient recollects the memories of earlier life. Other options: Option B: Dissociative amnesia is characterised by a sudden inability to recall important personal information. The patient has amnesia related to stressful or traumatic life events. There is no evidence of an underlying medical illness. Option C: Transient global amnesia is caused by temporary cerebral ischaemia in the distribution of the posterior cerebral circulation. It has an abrupt onset and resembles amnestic syndrome. Option D: Dissociative identity disorder is characterised by the presence of two or more distinct identities or personality states of which only one is being manifest at a time. Usually, one personality is not aware of the existence of the other personality. It was previously called multiple personality disorder.
Pearl PM0378: Dissociative Disorders DSM-5 Classification and characteristics of dissociative disorders:

Dissociative disorder	Characteristic features
Dissociative amnesia: **Failure to remember** important information about oneself after a stressful event.	Amnesia usually resolves in minutes or days but may last years. Retrograde amnesia > antegrade amnesia
Dissociative fugue: Amnesia combined with **sudden wandering** from home after a stressful event.	Adoption of a different identity. The patient denies memory loss during the fugue state.
Dissociative identity disorder: Previously called **multiple personality disorder.** At least two distinct personalities ("alters") in an individual.	More common in women (particularly those sexually abused in childhood) In a forensic setting (Eg: jail), malingering and alcohol abuse must be considered and excluded
Depersonalization disorder: Recurrent, persistent feelings of **detachment from one's own body**, the social situation, or the environment (derealisation) when stressed.	Understanding that these perceptions are only feelings, i.e., normal reality testing.
Dissociative disorder not otherwise specified: Dissociate symptom (Eg: trance-like state, memory loss) in -	• Persons exposed to intense coercive persuasion (Eg: brainwashing) • Indigenous to particular locations or cultures (Eg: Amok in Indonesia)

ICD-11 classifies Dissociative Disorders as: * Dissociative Identity Disorder * Partial Dissociative Identity Disorder * Depersonalization-Derealization disorder * Dissociative Amnesia * Trance Disorder * Possession Trance Disorder * Dissociative Neurological symptom disorder * Secondary Dissociative syndrome

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page no 27, 100-102

Q69. Psychiatry INI-CET 2025 ID: MG9269

1. Which of the following statements is false?

A ✓ Severe cases of alcohol withdrawal can be treated with I.V Flumazenil

B Opioid withdrawal is rarely fatal

C Chronic opioid withdrawal can be treated with long acting drugs like Buprenorphine

D Cannabis withdrawal is associated with minimal physical symptoms

Correct Answer: Severe cases of alcohol withdrawal can be treated with I.V Flumazenil

Severe cases of alcohol withdrawal can be treated with I.V Flumazenil is the false statement. Alcohol withdrawal symptoms include autonomic hyperactivity, such as anxiety, arousal, sweating, facial flushing, mydriasis, tachycardia, and mild hypertension. The classic sign of alcohol withdrawal is tremulousness, commonly called the "shakes" or the "jitters". It also causes psychotic and perceptual symptoms (e.g., delusions and hallucinations), seizures, and delirium tremens. These are commonly treated with benzodiazepines such as oxazepam, diazepam, chlordiazepoxide, and lorazepam. Flumazenil is a benzodiazepine antagonist and is not used to treat alcohol withdrawal. It is used to reverse benzodiazepine-induced sedation and in emergency care of benzodiazepine overdosage
Other options: Option B: Opioid withdrawal can cause symptoms like drug craving, feeling of temperature change, musculoskeletal pain, and GI distress. Signs of opioid withdrawal include confusion, drowsiness, vomiting, and diarrhea. It is not typically life-threatening, especially compared to alcohol or benzodiazepine withdrawal, which can lead to seizures or delirium tremens. Hence, opioid withdrawal is rarely fatal. Option C: Chronic opioid withdrawal can be effectively managed using long-acting opioids like buprenorphine or methadone. These medications help reduce withdrawal symptoms and cravings and are a cornerstone of opioid substitution therapy. Option D: Cannabis withdrawal is associated with minimal physical symptoms. The most common features include irritability, cannabis cravings, nervousness, anxiety, restlessness, insomnia, disturbed or vivid dreaming, decreased appetite, weight loss, depressed mood, and tremors. Pearl PM0416: Substances of abuse

Substance	Withdrawal symptoms	Intoxication features
Alcohol	Tremors, delirium, seizures	Blackouts, nystagmus and slurring of speech
BZD	Tremors, insomnia, anxiety	Lethargy and drowsiness.
Opioid/ Heroin	Diarrhea, yawning, mydriasis, lacrimation, piloerection, cold turkey phenomenon	Euphoria, depressed mental status, miosis, respiratory depression, constipation.
Nicotine/ Tobacco	Increased appetite	Shows a biphasic pattern. Early stimulatory symptoms of vomiting, hypertension and tachycardia. Later depressive symptoms of hypotension and bradycardia.
Cannabis	Irritability, sweating, tremors, decreased appetite	Increased appetite, euphoria, conjunctival injection, dry mouth, panic, synesthesia, impaired time perception, amotivational syndrome, run amok and flashbacks.
Amphetamine	Depressed mood, suicidal thoughts, unpleasant dreams	Acute: Sympathetic overactivity, paranoid hallucinatory syndrome, violent behavior, psychosis and diaphoresis. Chronic: Runs & crashes, choreiform movements, tooth decay. Can resemble schizophrenia.
Cocaine	Depressed mood, suicidal thoughts	Acute: Sympathetic overactivity, agitation, euphoria, chest pain, seizures and mydriasis Chronic: Cocaine psychosis (Magnan sign) and black pigmentation of the tongue.
LSD/ Haluccinogen	Nothing significant	Synesthesia, brisk tendon reflex, tachycardia, hypertension, panic, ataxia, visual hallucinations, euphoria and dysphoria.
Phencyclidine (PCP)	Nothing significant	Tachycardia, hypertension, ataxia, hallucinations, violent behavior, dissociation, amnesia and nystagmus.

Pearl PM0417: Alcohol Withdrawal Syndrome

Manifestation	Symptoms/signs	Onset since last drink (hours)
Tremulousness	Continuous tremor of > 8 Hz (or) Bursts	6–8

	of tremor activity < 8 Hz	
Alcoholic hallucinosis/ Psychotic and perceptual symptoms	Visual, auditory or tactile; **intact orientation**; stable vital signs.	8-12
Seizures **(rum fits)**	Single or multiple generalized tonic-clonic	12–24
Delirium tremens	Confusion, agitation, fever, tachycardia, hypertension, diaphoresis, **impaired orientation**	< 72

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page 35-45
Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition • Page 461
Online Resource • https://www.msdmanuals.com/professional/special-subjects/illicit-drugs-and-intoxicants/opioid-toxicity-and-withdrawal#Symptoms-and-Signs_v25243684
Online Resource • Cambridge Textbook of Effective Treatments in Psychiatry Page 381, 383

Q70. Psychiatry INI-CET 2025 ID: MG9267

2. "Lord hanuman is celibate. I am celibate. So, I am Lord Hanuman." What is this type of thinking seen in Schizophrenia?

A Loosening of association

B Neologism

C ✓ Autistic thinking

D Verbigeration

Correct Answer: Autistic thinking

This type of thinking as described is autistic thinking which is seen in Schizophrenia. Autistic thinking is defined as thinking in which the thoughts are largely narcissistic and egocentric, with emphasis on subjectivity rather than objectivity, and without regard for reality. It is seen in schizophrenia and autistic disorder. The reasoning "Lord hanuman is celibate. I am celibate. So, I am Lord Hanuman" is an illogical personal association not grounded in rational logic, characteristic of autistic thinking in schizophrenia. Other options: Option A: Loosening of association is defined as gradual or sudden deviation in train of thought without blocking, also known as derailment. Option B: Neologism refers to new word or phrase whose derivation cannot be understood and is often seen in schizophrenia. Option D: Verbigeration refers to meaningless and stereotyped repetition of words or phrases, also seen in schizophrenia. Pearl PM2383: Disorders of thought and perception DISORDERS OF THOUGHT

DISORDER OF STREAM OF THOUGHT

(A) Disorder of tempo

(i) **Flight of Ideas:** Continuous flow of rapid, pressured speech that jumps from topic to topic.

- "I like mangoes, the weather is nice, India is great"

(ii) **Retardation of thinking**: Slow speed of speech and a prolonged latent period before a response

- "I went to the market (pause)..... the shop was closed"

(iii) **Circumstantiality:** Inclusion of irrelevant details and going on in a roundabout manner, but their thought ultimately reaches the answer

- Doctor: "How did you get that bruise?"
- Patient: "I woke up and had a cup of coffee, read the newspaper and went for a walk. It was a wonderful day in the park. I returned home and watched the TV. I took a nap in the afternoon after lunch. I slipped and fell in the bathroom. The fall gave me this bruise."

(iv) **Clang associations:** An association of thoughts with the sound of words rather than by their meaning (e.g., through rhyming)

- I am Sam...I like jam...I travel in a tram"

(B) Disorder of continuity

(i) **Perseveration:** Repetition of out of context words, phrases, or ideas beyond the point of relevance.

- Q1. What's your name?: "Rahul"
- Q2. Where are you from?: "Rahul"
- Q3. How old are you?: "Rahul"

(ii) **Thought blocking:** A sudden disruption of thought or a break in the flow of ideas

- "I went to the market (extreme silence)....."

DISORDER OF FORM OF THOUGHT

(i) **Tangentiality:** In response to a question, the patient gives a reply that is appropriate to the general topic but does not answer the question exactly. The speech moves away from the actual answer.

- Doctor: "Have you had any trouble sleeping lately?"
- Patient: "I usually sleep in my bed, but now I'm sleeping on the sofa"

(ii) **Derailment:** A breakdown in both the logical connection between ideas and the overall sense of goal-directedness. The words make sentences, but the sentences do not make sense.

- "Some people just don't understand why I had breakfast in America during the first world war"

(iii) **Neologisms:** The invention of new words or phrases or the use of conventional words in idiosyncratic ways.

- The world is not 'gooder' for people like me"

(iv) **Desultory thoughts:** Speech is grammatically correct, however, sudden unrelated ideas force their way in from time to time.

- My actual problem is that the people never understand me and my stomach feels like an empty bucket"

(v) **Word salad:** Using a mixture of unintelligible meaningless words and sentences

- Tree bus rain food home car tiger"

DISORDER OF POSSESSION OF THOUGHT

A. Thought alienation

(i) **Thought insertion:** Thoughts are being inserted into the patient's mind and they recognize them as being foreign and coming from without any stimulus

- "I have repeated thoughts of killing my neighbors and I believe aliens are putting these ideas into my head at night"

(ii) **Thought withdrawal:** Delusion that someone or something is removing from one's mind and the patient has no power over this.

- "Lately, I am losing all my memories and I believe that the 'government agents' are stealing them when I go to sleep"

(iii) **Thought broadcasting:** Belief that one's thoughts are quietly escaping from one's mind and that other people might be able to access them and hear or see them.

- "I believe my exact thoughts appear on the TV and the internet and I think I can broadcast them globally"

(iv) **Thought echo:** Patient hears the 'echo' of his own thoughts in the form of a voice after he has made the thought.

- "I thought of making a sandwich in the morning and surprisingly a voice, from nowhere, told me to make a sandwich out loud"

B. Obsessions and Compulsions

1. **Obsessions:** Recurrent and persistent thoughts, impulses. or images that cause distress to oneself.

- Fear of dirt or contamination
- Fear of becoming ill
- Constant thoughts of a certain number
- Need for symmetry or exactness
- Worry about whether something has been done "right"
- Forbidden sexual or religious thoughts

2. **Compulsions:** Repetitive behaviors or actions that the person feels driven to perform in response to an obsession

- Washing hands
- Blinking eyes
- Counting items, such as currency
- Checking to see if doors are locked
- Arranging items in a certain way
- Requiring constant approval from people around them

DISORDER OF CONTENT OF THOUGHT

(i) **Phobia**: Persistent fear of an object or situation, typically disproportional to the actual danger posed, in which the sufferer goes to great lengths in avoiding it

- Arachnophobia - fear of spider
- Barophobia - fear of gravity
- Cynophobia - fear of dogs
- Hemophobia - fear of blood
- Nosocomephobia - fear of hospitals

(ii) **Religious preoccupations:** Thoughts involving religious themes or subject matter.

- "I get instructions from God on how to make my country great"

(iii) **Ideas of hopelessness:** Thoughts of complete despair where the patient strongly believes that the circumstances will never get better for him.

- "I am never going to pass my final exams, no matter how hard I study"

(iv) **Antisocial urges:** Thoughts of indulging in some behaviors or actions that harm or lack consideration for the well being of others.

- "I have these urges to go rob a bank or blackmail my neighbors for their money"

(v) **Delusions**: False beliefs that are strongly held and unchangeable in the face of refuting evidence and that are not consistent with the person's educational, social and cultural background.
The three main criteria for a belief to be considered a delusion are:

- **Certainty** (held with complete confidence)
- **Incorrigibility** (not changeable by proof to the contrary)
- **Impossibility or falsity** of content (implausible, bizarre or patently untrue)

DISORDERS OF PERCEPTIONS

- **Illusion**: Misinterpretation of real external stimuli.
- **Hallucination**: False sensory perception, externally.

- **Pseudohallucination**: False sensory perception from within and having an insight.
- **Depersonalization:** Feeling as if one is disconnected from reality, feeling detached from one's own thoughts and one's own physical body.

  "I sometimes drift into a dream-like state where I lose contact with my own self"
- **Derealization:** Alteration in the perception or experience of the external world so that it seems unreal.

  "The world and everything around is nothing but just a dream"

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References: A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • Page no 55

Q71. Psychiatry INI-CET 2025 ID: MG9260

3. Which is the new psychoactive drug which has a rapid onset of anti-depressant action?

A Bupropion

B Mephedrone

C Cannabidiol

D ✓ Ketamine

Correct Answer: Ketamine

Ketamine is the new psychoactive drug which has a rapid onset of anti-depressant action. Ketamine is a dissociative anesthetic agent which has been shown to produce a rapid, substantial, and persistent reduction in symptoms in patients with major depressive disorder. It inhibits the postsynaptic glutamate binding protein N-methyl-d-aspartate (NMDA) receptor. Because abnormalities in glutamatergic signaling have been implicated in major depressive disorder, this may account for its efficacy. Patients usually receive a single infusion of ketamine over a 30-minute period at a concentration of 0.5 mg/kg. A positive response is usually seen within 24 hours, and improved mood lasts for about 2 to 7 days. Side effects like dizziness, headache, and poor coordination are commonly seen. Dissociative symptoms, including hallucinations, may also occur Although ketamine appears effective, its use in depression treatment is off-label. Only esketamine, its S-enantiomer, is FDA-approved for major depressive disorder. Other options: Option A: Bupropion is an antidepressant that acts by dopamine and norepinephrine reuptake inhibition. The onset of its therapeutic effect usually occurs during the second week of therapy. and is not known for rapid action. It is mainly indicated for smoking cessation. Option B: Mephedrone is a recreational psychoactive drug which has no proven antidepressant effects. Option C: Cannabidiol is a non-psychoactive compound from cannabis with potential therapeutic uses. However, its benefits are still being studied, and it is not approved for depression.

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References: Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition • Page no. 348
Online Resource • https://www.fda.gov/drugs/news-events-human-drugs/understanding-current-use-ketamine-emerging-areas-therapeutic-interest-06272024
Online Resource • https://www.ncbi.nlm.nih.gov/books/NBK470212/
Online Resource • https://pubmed.ncbi.nlm.nih.gov/28012094/
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KD Tripathi Essentials of Medical Pharmacology 8th Edition • Page no.134,411

Q72. Psychiatry INI-CET 2025 ID: MG9259

4. Which of the following drugs can be used in Acute Mania? 1. Lithium 2. Valproate 3. Haloperidol 4. Amitryptilline

A 1 only

B ✓ 1, 2 and 3

C 1 and 2 only

D 1, 2, 3 and 4

Correct Answer: 1, 2 and 3

Lithium, valproate and haloperidol (1, 2 and 3) can be used in Acute Mania. The pharmacological treatment of bipolar disorders is divided into both acute and maintenance phases. Treatment of acute mania includes: * Lithium: It is considered the prototypical mood stabilizer. But due to the slow onset of antimanic action with lithium, it usually is supplemented in the early phases of treatment by atypical antipsychotics, mood-stabilizing anticonvulsants, or high-potency benzodiazepines. Major drawback is its narrow therapeutic index (recommended plasma level 0.6 to 1.2 mEq/L). * Valproate: It is only indicated for acute mania at dose levels of 750 to 2,500 mg per day. It is well tolerated and associated with a rapid onset of response. * Carbamazepine and Oxcarbazepine: They are one of the first-line treatment for acute mania. * Clonazepam and Lorazepam. They are high-potency benzodiazepine anticonvulsants which are widely used for adjunctive treatment of acute manic agitation, insomnia, aggression, and dysphoria, and panic. * Atypical and Typical Antipsychotics: Older agents like haloperidol and chlorpromazine can be used, but atypical antipsychotics like olanzapine, risperidone, quetiapine have a lesser liability for excitatory postsynaptic potential and tardive dyskinesia. Lithium, carbamazepine, and valproic acid, alone or in combination, are the most widely used agents in the long-term treatment of patients with bipolar disorder. Amitryptilline (Option 4)is a tricyclic antidepressant and acts by blocking the reuptake of both serotonin and norepinephrine neurotransmitters. It is used to treat major depressive disorder, and not acute mania.

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References: Tasman's Psychiatry - 5th Edition • Page no. 1795,1799
Goodman & Gillman's The Pharmacological Basis Of Therapeutics - 14th Edition • Page no. 347
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Q73. Psychiatry INI-CET 2025 ID: MG9254

5. All of the following are cardinal symptoms of depression as per ICD-10 except:

A Reduced interest in pleasurable activities

B ✓ Lack of self-esteem and confidence

C Low mood most of day and at all times

D Reduced energy

Correct Answer: Lack of self-esteem and confidence

Lack of self-esteem and confidence is not a cardinal symptom of depression as per ICD-10. According to the ICD-10 Classification of Mental and Behavioural Disorders, a depressive episode is typically diagnosed when at least two of the following three cardinal symptoms are present: * Depressed mood (persistent low mood) to a degree that is definitely abnormal for the individual, present for most of the day and almost every day, largely uninfluenced by circumstances, and sustained for at least 2 weeks (Option C) * Loss of interest or pleasure in activities that are normally pleasurable (Option A) * Decreased energy or increased fatiguability. (Option D) In addition, one or more of the following symptoms should also be present to make up a minimum total of four symptoms: * Loss of confidence or self-esteem * Unreasonable self-reproach or excessive and inappropriate guilt * Recurrent thoughts of death or suicide, or any suicidal behaviour * Diminished ability to think or concentrate (e.g. indecisiveness or vacillation) * Psychomotor changes (agitation or retardation, subjective or observed) * Sleep disturbances of any kind * Change in appetite (increased or decreased), often with weight change Loss of confidence and self-esteem is considered an additional symptom of depression in ICD-10, not one of the cardinal symptoms (Option B). Pearl PM0496: Signs and symptoms of depression Mnemonic "SIGECAPS" Sleep disorder (increased or decreased) Interest deficit (anhedonia): loss of interest in previously pleasurable activities Guilt (worthlessness, hopelessness, regret) Energy deficit Concentration deficit Appetite disorder (increased or decreased) Psychomotor retardation or agitation Suicidal tendency

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References: Online Resource • https://iris.who.int/bitstream/handle/10665/37108/9241544554.pdf?sequence=1
A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition • page no. 3

Q74. Psychiatry NEET 2021 ID: MF1465

During psychotherapy, the therapist had mixed conscious and unconscious feelings towards his patient. This is known as_____.

A ✓ Countertransference

B Transference

C Dissociation

D Preoccupation

Correct Answer: Countertransference

In the given clinical scenario, the therapist has mixed conscious and unconscious feelings towards his patient. This is known as **counter-transference.**

Counter-transference is the positive and **negative feelings/emotions** expressed by healthcare professionals **towards** their **patients**. It is the process of the **physician unconsciously displacing** onto the patient patterns of behaviors or emotional reactions as if he/she were a significant figure from earlier in the physician's life. Psychiatrists should be alert to signs of countertransference issues like missed appointments by the psychiatrist, boredom, or sleepiness during a session. Supervision and personal therapy can help the psychiatrist recognize and deal with these issues.

Transference is the positive and **negative feelings expressed** by the patients towards the **healthcare professional**. It is the process of the **patient unconsciously displacing** onto individuals patterns of behavior and emotional reactions that originated with significant figures from the past (generally in childhood). This displacement is onto the psychiatrist in a clinical scenario, who is a parent surrogate. The patient may be angry, hostile, demanding, or obedient to an extreme degree because of former relationships and patterns of behaviors. Failure to recognize this can make the psychiatrist react to the patient's behavior like a personal attack.

These are not defense mechanisms but are commonly encountered during clinical practice or while giving psychotherapy. In an interview, transference and counter-transference are very significant expressions of unconscious processes.

Option C: **Dissociation** is an immature defense mechanism. In this, involuntary splitting/suppression of mental functions occurs from the rest of the personality in a way that allows the expression of forbidden unconscious impulses without having a sense of responsibility for actions.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page No 866, 867
A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition, Page No 208

Q75. Psychiatry NEET 2021 ID: MF1475

A patient stopped alcohol consumption for 3 days and presented with irritability, disorientation, paranoid delusions, agitation, visual hallucinations, and altered sensorium. What is the likely diagnosis in this case?

A ✓ Delirium tremens

B Wernicke's encephalopathy

C Korsakoff psychosis

D Alcohol induced psychosis

Correct Answer: Delirium tremens

The given clinical scenario of a patient who has not consumed alcohol for the past 3 days with disorientation, paranoid delusions, visual hallucinations, and altered sensorium is suggestive of **delirium tremens.**

Delirium tremens is a medical **emergency** that occurs during **alcohol withdrawal**. It is an **acute condition** and usually develops within 72 hours after the last drink. It has symptoms of both delirium and alcohol withdrawal. The clinical features of delirium tremens are:

- **Altered sensorium** with **disorientation**
- Perceptual distortions, most frequently tactile or **visual hallucinations**
- Autonomic hyperactivity with tachycardia, fever, anxiety, insomnia, hypertension. And sweating
- Fluctuating levels of psychomotor activity, ranging from **agitation** to lethargy
- Altered sleep-wake cycle

Other options:

Option B - **Wernicke's encephalopathy (alcoholic encephalopathy)** is associated with **thiamine deficiency.** Alcohol abuse may lead to thiamine deficiency because of poor nutritional habits, or malabsorption. It is an **acute neurological disorder** characterized by:

- Global confusion
- Ocular abnormalities like horizontal nystagmus, gaze palsy, anisocoria, and sluggish pupillary reflex
- Ataxia - mainly affecting the gait
- Vestibular dysfunction

It may resolve with thiamine therapy or progress to Korsakoff's syndrome.

Option C - **Korsakoff's psychosis** can follow Wernicke's encephalopathy due to **thiamine deficiency.** It is a **chronic amnestic syndrome** that is characterized by the following clinical symptoms in an intact and responsive patient:

- Impaired mental syndrome, especially recent memory
- Anterograde amnesia
- **Confabulation**

Option D - **Alcohol-induced psychosis** is characterized by the development of auditory hallucinations or paranoid delusions in the context of a heavy drinking or alcohol withdrawal. These patients have a clear sensorium, unlike the patients having delirium tremens.

Note: Tremulousness is the first alcohol withdrawal symptom.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 630 - 633

Q76. Psychiatry NEET 2021 ID: MF1452

A teacher demonstrated the steps of handwashing to her students. Students also performed the same activity later at school and at home. What is the type of learning in this case?

A Cognitive

B ✓ Psychomotor

C Affective

D Affective and cognitive

Correct Answer: Psychomotor

The type of learning seen in this case is **psychomotor** learning.

The **psychomotor domain** includes **learning** by **observation**, physical movements, and the usage of motor skill areas of the brain. The development of these skills requires practice, depending upon the complexity of the task. It is measured based on speed, precision, distance, and procedures in execution.

The three domains in learning are the **cognitive, psychomotor**, and **affective** domains. They are described as follows.

Cognitive learning includes the domain of **knowledge**. Bloom's taxonomy described six levels of cognitive skills. This was revised to include types of knowledge and cognitive processes. The cognitive processes include remembering, understanding, applying, analyzing, evaluating, and creating. Eg. Solving multiple-choice questions in the NEET PG entrance exam involves the usage of the cognitive domain.

Psychomotor learning includes the domain of **skills**. It includes imitation, manipulation, precision, articulation, and naturalization. Eg. Learning how to insert an IV cannula by observing the residents and nurses during the internship involves the usage of the psychomotor domain.

Affective learning includes the domain of **attitude** and **values**. It includes receiving, responding, valuing, organizing, and characterization by value. Eg. Communicating with patients with empathy, responding to their concerns, and interacting with the relatives of patients involves the usage of the affective domain.

Integration of all these three methods in teaching and learning methodology leads to a better understanding of the subject.

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References: Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK559109/
Online Resource, https://www.educarepk.com/psychomotor-domain-daves-taxonomy.html
Online Resource, https://lynnleasephd.com/2018/08/23/krathwohl-and-blooms-affective-taxonomy/

Q77. Psychiatry NEET 2021 ID: MF1454

Which of the following is a formal thought disorder?

A Obsession

B ✓ Derailment

C Delusion

D Thought insertion

Correct Answer: Derailment

Derailment is a **formal thought** disorder.

It is a breakdown in both the logical connection between ideas and the overall sense of being goal-oriented. It is generally identified in **schizophrenic patients** as a **generalized loosening of associations.**

For example, a person with derailment thought disorder might jump from talking about rabbits to the hair on their head to your sweater. They talk in chains of semi-related ideas. Their communication style has a generalized loosening of associations. Their ideas often fall further and further from the topic.

Formal thought disorders are:

- **Circumstantiality** is the overinclusion of trivial or irrelevant details that impedes the sense of getting to the point.
- **Clang association** is an association of thoughts with the sound of words rather than by their meaning (e.g., through rhyming).
- **Derailment** is the breakdown in both the logical connection between ideas and the overall sense of goal-directedness. The words make sentences, but the sentences do not make sense.
- **Flight of ideas** is a succession of multiple associations so that thoughts seem to move abruptly from idea to idea, through rapid, pressured speech.
- **Neologism** is the invention of new words or phrases or the use of conventional words in idiosyncratic ways.
- **Perseveration** is the repetition of out-of-context words, phrases, or ideas.
- **Tangentiality** is responding to a question, that is appropriate to the general topic without actually answering the question.
- **Thought blocking** is a sudden disruption of thought or a break in the flow of ideas.

Other options:

Option A - An obsession is a recurrent and intrusive thought, feeling, idea, or sensation.

Option C - A delusion is defined as a false belief based on incorrect inference about external reality that firmly sustains despite evidence to the contrary.

Option D - Thought insertion is a type of delusion where the patient believes that thoughts are not his/her own and have been inserted into his/her mind.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page No 202, 228, 418

Q78. Psychiatry NEET 2021 ID: MF1455

A 16-year-old girl presented with overfamiliarity, flight of ideas, elevated mood, increased sexual desire, and pseudohallucination. What is the likely diagnosis?

A ✓ Mania

B Hypomania

C Cyclothymia

D Schizomania

Correct Answer: Mania

In the given clinical scenario, history of overfamiliarity, elevated mood, flight of ideas, and increased sexual desire are suggestive of **mania**.

The **diagnostic criteria** for **mania** are:

A. Distinct period of abnormally and **persistently elevated/irritable mood**, and abnormally and persistently increased activity/energy, lasting at **least 1 week**.

B. During the period of mood disturbance and increased energy/activity, three (or more) of the following symptoms are present to a significant degree:

1. Increased self-esteem or grandiosity
2. Decreased need for sleep
3. **More talkative** than usual or a pressure to keep talking
4. **Flight of ideas**
5. Distractibility
6. **Increase in goal-directed activity** or psychomotor agitation
7. Excessive involvement in activities that have a high potential for painful consequences (unrestrained buying sprees, sexual indiscretions)

C. The mood disturbance is severe enough to cause marked social or occupational impairment or to necessitate hospitalization.

D. The episode is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication, other treatment) or to another medical condition.

Other options:

Option B - **Hypomania** is a distinct period of abnormally and **persistently elevated/irritable mood**, abnormally and persistently increased activity/energy, lasting for **at least 4 days**. It is similar to a manic episode but it is not severe enough to cause impairment in social or occupational functioning. No psychotic features are present.

Option C - **Cyclothymia** is a mild, **chronic mood disorder** with numerous depressive and hypomanic episodes over the course of at **least 2 years**.

Option D - **Schizomania** is an old term in DSM III, used to describe schizophrenic and manic symptomatology occurring in one episode, classified under **schizoaffective disorder**. Schizoaffective disorder is characterized by **active phase symptoms** of **schizophrenia**, like delusions, hallucinations, disorganized speech, grossly disorganized behavior, or negative symptoms. Additionally, there is a **presence of manic/depressive syndrome** that is not brief relative to the duration of the psychosis. The delusions or hallucinations are present for **at least 2 weeks without coexisting mood symptoms**.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 292, 293, 348, 358
Online Resource, https://pubmed.ncbi.nlm.nih.gov/1791861/

Q79. Psychiatry INI-CET 2023 ID: MF9160

A boy is experiencing low mood, decreased energy, and decreased interest in his surroundings. He is not taking food for the past one week. The thought sample says "I lost my head. I am dead. How can I eat?". What is the delusion seen in this case?

A Cotard's syndrome

B Hypochondriacal delusion

C ✓ Nihilistic delusion

D Bizarre delusion

Correct Answer: Nihilistic delusion

The type of delusion seen in this patient is a **nihilistic delusion**.

Nihilism refers to the **delusion of the non-existence of self** or parts of self. It also includes an attitude of rejection of established values or extreme skepticism regarding moral and value judgments. The patient may believe that they are **going to die soon** or their **organs are rotting**. **Delusion of nihilism** is often seen as a **mood-congruent delusion** in **depression with psychotic symptoms**. This patient has three major symptoms of depression — **low energy, depressed mood, and lack of interest** in the surroundings. However, these symptoms have to be present for a period of **at least 14 days** to make the diagnosis of major depressive disorder.

Other options:

Option A: **Cotard's syndrome** was first described by Jules Cotard. He described a psychotic disorder characterized by **severe depression** and **nihilistic delusions** regarding one's own body or existence. In the DSM or ICD, Cotard's syndrome is **not classified separately**. Nihilistic delusions have been mentioned as examples of **mood-congruent delusions** in a **depressive episode with psychotic features.**

Option B: Patients with **hypochondriacal delusions** are **fixated on** one or more **somatic symptoms** as evidence of some **severe**, yet undetected **disease**. They undergo **repeated** and **unnecessary medical examinations** and tests. There are frequent deteriorations of relationships with their doctors when nothing suggesting a disease is ever found, and this leads to doctor shopping.

Option D: **Delusions** are **false** and **fixed ideas** that are not shared by other people. **Bizarre delusions** are the ones whose **content is out of the realm of possibility**, for example, a patient believing that aliens abduct him and take him to Mars every night. Typically, patients with **schizophrenia** tend to have bizarre delusions.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page 9, 379-381, 451-453
Online Resource, https://icd.who.int/browse11/l-m/en#/http%3a%2f%2fid.who.int%2ficd%2fentity%2f675329566
Online Resource, https://link.springer.com/article/10.1007/s11920-009-0031-z

Q80. Psychiatry INI-CET 2023 ID: MF9169

La belle indifference is seen in:

A Mania

B Depression

C Schizophrenia

D ✓ Conversion disorder

Correct Answer: Conversion disorder

La belle indifference is seen in **conversion disorder**.

La belle indifference refers to a **lack of concern towards the symptoms**, despite the apparent severity of the disability. Although not pathognomonic of conversion disorder, it is often seen in association with it.

Conversion disorder is also known as **functional neurological symptom disorder** or **dissociative disorder**. It includes **acute** and **temporary loss of motor or sensory function without** any underlying **neurological disease**. Because the onset of such symptoms often coincides with psychological issues such as internal conflict, it was speculated that these issues converted to neurological symptoms. Conversion disorder can present with symptoms such as **paresis**, **non-epileptic seizures (NES)**, **blindness**, and anesthesia of the extremities. One of the gait disturbances which can be seen is **astasia-abasia** — a wildly **ataxic**, **staggering gait** with gross and irregular **truncal movements**, and thrashing and waving **arm movements**. The patient often has a carefree attitude towards his illness as mentioned above.

Patients of conversion disorder achieve **primary gain** by keeping **internal conflicts outside of their awareness**. These conflicts **manifest as** physical **symptoms**. As a result of being sick, the patients are **excused from obligations** and **receive support** that might not otherwise have been available. This is referred to as **secondary gain**.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page 453-456

Q81. Psychiatry INI-CET 2023 ID: MF9172

SCOFF protocol is used for assessing:

A Mental health disorder

B Behavioural disorder

C Sexual health disorder

D ✓ Eating disorder

Correct Answer: Eating disorder

The SCOFF (**S**ick, **C**ontrol, **O**ne, **F**at, **F**ood) questionnaire is a **screening tool** for **eating disorders** (anorexia nervosa and bullimia nervosa).

It consists of 5 questions addressing the core features of these disorders. The **SCOFF** questions are:

1. Do you make yourself **S**ick because you feel uncomfortably full?
2. Do you worry that you have lost **C**ontrol over how much you eat?
3. Have you recently lost more than **O**ne stone (~6.35 kg) in a 3-month period?
4. Do you believe yourself to be **F**at when others say you are too thin?
5. Would you say that **F**ood dominates your life?

A 'Yes' answer to 2 or more questions out of the 5 indicates likely anorexia or bulimia.

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References: Online Resource, https://www.bmj.com/content/319/7223/1467

Q82. Psychiatry INI-CET 2023 ID: MF9152

The tactile hallucinations seen in cases of chronic cocaine intoxication is?

A Oneroid state

B ✓ Formication

C Dipsomania

D Carphologia

Correct Answer: Formication

Formication, which refers to the sensation of insects crawling beneath the skin, is a feature of **cocaine abuse**. It is also referred to as **Magnan's symptoms** or **cocaine bugs**.

The symptoms of **acute stimulant intoxication** include **agitation**, **irritability**, **aggression**, and an increase in psychomotor activity. On examination, **tachycardia**, **hypertension**, and **mydriasis** may be seen. When **psychotic symptoms** such as **delusions** and **hallucinations** are present due to stimulants such as cocaine, it is referred to as **stimulant-induced psychotic disorder**. **Paranoid delusions and hallucinations** are **hallmarks** of stimulant-induced psychosis. **Tactile hallucinations** in the form of **formication** are seen in cocaine use. The presence of these symptoms depends on the dose and duration of the substance abuse and thus may not be seen in all patients. Cocaine-induced psychotic disorders are more frequently seen in **intravenous users** and those who use crack (an extremely **potent** and **addictive** form of **cocaine** that can be **smoked**).

Other options:

Option A: **Oneroid state** is a **dream-like state** which can happen because of substance use or some organic disorder.

Option C: **Dipsomania** refers to a **compulsive urge to drink alcoholic beverages**. Patients suffering from alcohol-use disorders feel a need for daily use of large amounts of alcohol for adequate functioning throughout the day.

Option D: **Carphologia** (also known as **floccillation**) refers to the **action of trying to pick imaginary objects**. The patient will usually be trying to pick lint from his clothes or the bedsheet. This is often seen in **delirium**. **Datura intake** can lead to a delirious state and may show carphologia.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page 302-305
The Essentials Of Forensic Medicine And Toxicology, 35th edition, Page 454, 458, 459

Q83. Psychiatry INI-CET 2023 ID: MF9156

Which of the following is used to define dependence?

1. Strong desire to use the drug
2. Priority to use drugs other than any other activity
3. Increased tolerance
4. Persistent use of drugs despite knowing the harmful effects

A 1, 2

B 1 only

C ✓ 1, 2, 3, 4

D 1, 2, 3

Correct Answer: 1, 2, 3, 4

According to ICD-11, **dependence** on any substance is characterized by:

- **Strong internal drive to use** the substance, which the patient is **unable to control**.
- **Increased priority is given to the use** of the substance over other activities.
- Persistence of substance use **despite harmful consequences.**
- Presence of the subjective sensation of **urge** or **craving** to use the substance.
- **Tolerance** to the effect of the substance; the patient may require **higher amounts** of the substance to achieve the same effect.
- Appearance of **withdrawal symptoms** on cessation or reduction in the dose of the substance.

The diagnosis of dependence can be made if these symptoms are present for at least a period of 3 months.

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References: Online Resource, https://icd.who.int/browse11/l-m/en#/http%3a%2f%2fid.who.int%2ficd%2fentity%2f1580466198
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page 270

Q84. Psychiatry INI-CET 2023 ID: MG1138

A pregnant female with bipolar disorder came to the clinic in the third trimester. She has had 4 episodes in the past and is stable on lithium 750 mg/day since 1 year during which time she has not had any episodes. What is the next best step in management?

A Stop lithium and let her be drug free

B Increase lithium dosage

C ✓ Decrease lithium dosage

D Switch to valproate

Correct Answer: Decrease lithium dosage

In this scenario of a **pregnant woman in her third trimester** with **bipolar disorder** with **multiple past episodes** and **currently stable on lithium**, the **best next step in management** is to **decrease lithium dosage within therapeutic levels**.

Lithium is a **mood stabilizer** with a **very narrow therapeutic index** and is the **first line** of management in **bipolar disorder**. Lithium therapy should be **continued throughout the pregnancy** in women who have **severe, frequent episodes**, and these patients should be counseled about the **reproductive risks** associated with therapy. Ideally, the next best step in this scenario would be to **monitor lithium blood levels monthly until 34 weeks**, and **weekly monitoring thereafter until delivery**. However, **dose reduction can be considered closer to delivery** as there may be an increased risk of **lithium toxicity in the fetus**. According to **NICE guidelines**, it is recommended to **stop lithium during active labour**, measure **lithium levels 12 hours after the last dose** and reestablish prophylactic levels after delivery.

First and second trimester are characterised by a significant **decrease of lithium blood levels** with a risk of subtherapeutic levels. In **third trimester** and the **postpartum**, **lithium levels gradually return** to their preconception level which implicates that in this period clinicians need to be aware of the **risk of lithium intoxication (Option B)**.

Bipolar disorder has been associated with **adverse perinatal outcomes** such as **preterm birth**. **Medication discontinuation in pregnancy (Option A)** can pose an increased risk for **relapse**. Moreover, **symptomatic women during pregnancy** more likely have **postpartum relapses** that can be severe.

Typical therapy for bipolar disorder includes mood stabilizers such as **lithium, valproic acid,** and **carbamazepine** and **antipsychotic** medications. Bipolar disorder during pregnancy is a complicated condition that is best treated in tandem with psychiatric care. Risks and advantages of mood stabilizers, some of which are **teratogenic**, must be considered while making decisions.

For example, **lithium use** during the **first trimester** has been linked to **Ebstein anomaly**, although very rarely. **Fetal echocardiography** is recommended for **lithium-exposed fetuses**. Fetal lithium exposure is also associated with **neonatal goiter, CNS depression, hypotonia** (floppy baby syndrome), and cardiac murmur.

Maternal adverse effects of lithium can be **acute** such as **tremors, ataxia, weakness, polyuria, polydipsia** or **chronic** such as **nephrogenic diabetes insipidus, thyroid dysfunction**, and **hyperparathyroidism**.

Due to all these adverse effects, patients on lithium therapy should have **routine monitoring** of serum electrolytes, serum calcium, thyroid profile, renal function tests, ECG, and serum lithium levels.

Other options :

Option D : It is not necessary to switch to valproate in the third trimester as the patient is already stable on lithium. Use of valproate in the first trimester is associated with significant teratogenicity.

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References: Williams Obstetrics 26th Edition, Page No 153-154, 1146, 1148
Online Resource, https://journalbipolardisorders.springeropen.com/articles/10.1186/s40345-018-0135-7
Online Resource, https://www.aafp.org/pubs/afp/issues/2008/0915/p772.html
Online Resource, https://www.nice.org.uk/guidance/cg192/resources/mental-health-in-pregnancy-and-the-year-after-giving-birth-250640652229
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5494438/

Q85. Psychiatry INI-CET 2023 ID: MG1126

HEADSSS assessment will be used by an intern for which of the following patient groups?

A Children under 5 years

B Neonates

C Infants

D ✓ Adolescents

Correct Answer: Adolescents

HEADSSS assessment is done in **adolescents** to determine **early symptoms of mental disorders** and **functional impairment** in major **life domains** such as **home** and **family**, **school**, **peers**, and **community**.

These domains are included in the **HEADSSS (Home, Education, Activities, Drugs, Sexuality, Suicide/Depression, Safety) Interview Guide**. An adolescent's **risk and protective factors** are identified during each visit using this assessment. **Risk factors increase** the likelihood that an adolescent will engage in **risky behaviors**, whereas **protective factors build** an **adolescent's resiliency** and **ability to cope with stress**.

Examples of questions in **HEADSSS** assessment :

Home	• Who lives at home, and where? • Do you have your own room? • What are relationships like at home? • What do parents and relatives do for a living?
Education	• School/grade performance-any recent changes? Any dramatic past changes? • Any years repeated/classes failed • Suspension, termination, dropping out? • Any current or past employment?
Activities	• On own, with peers (what do you do for fun? where? when?) • Hobbies-other activities? • History of arrests-acting out-crime?
Drugs	• Use by peers? Use by young person? (include tobacco, alcohol) • Use by family members? • Amounts, source, frequency, patterns of use/abuse
Sexuality	• Degree and types of sexual experience and acts? Number of partners? • Sexually transmitted diseases-knowledge and prevention? Contraception? Frequency of use? • History of pregnancy/abortion?
Suicide / Depression / Self Image	• Sleep disorders, appetite/eating behavior changes, emotional outbursts and highly impulsive behavior, history of withdrawal/isolation, hopeless/helpless feelings, suicidal ideation • History of past suicide attempts, depression, psychological counseling
Safety	• Do you ever feel unsafe?

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References: Nelson textbook of pediatrics 21st edition, Page No 186
Online Resource, https://depts.washington.edu/dbpeds/Screening%20Tools/HEADSS.pdf
Online Resource, https://www.heardalliance.org/wp-content/uploads/2011/04/HEADSS.pdf

Q86. Psychiatry AIIMS 2017 ID: MA1543

A 42-year-old man had a fall followed by one episode of vomiting. When brought to the casualty, he was awake and conscious but had unrelieved anterograde amnesia. Which of the following is the most likely cause?

A Drunkenness

B Diffuse axonal injury

C ✓ Concussion

D Cerebral venous thrombosis

Correct Answer: Concussion

The given history of **fall** and **anterograde amnesia** points towards the diagnosis of **concussion**.

A concussion is the most common type of traumatic brain injury. It can result from vehicle accidents, falls, sports injuries, etc. Anterograde amnesia/post-traumatic amnesia is the hallmark of a concussion. In this type of amnesia, the **events following the injury** cannot be recalled.

Other clinical features of concussion include headache, dizziness, nausea, and vomiting.

Option A: **Drunkenness** - the patient will be **drowsy or sleepy** and not awake.

Option B: **Diffuse axonal injury** - seen in high-speed deceleration which causes **disruption** and **shearing** of **axons** leading to **microhemorrhages.** Usually results in a persistent vegetative stage.

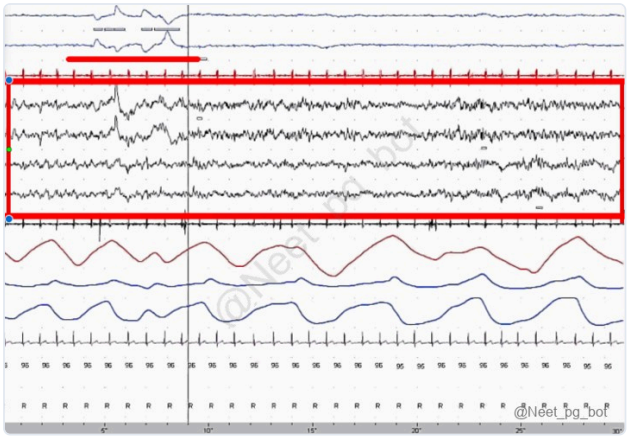
Option D: **Cerebral venous thrombosis** - acute thrombosis in dural venous sinuses; manifests as headache, visual abnormalities, or seizures.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Chapter 3.3
Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK459315/

Q87. Psychiatry AIIMS 2017 ID: MB9227

The marked area given in the sleep polysomnography is of _____



A ✓ REM sleep

B NREM stage 1

C NREM stage 2

D NREM stage 3

Correct Answer: REM sleep

The marked area in the given polysomnography represents **REM sleep**.

The three zones in the given image represent **EOG**, **EEG** and **EMG**, respectively.

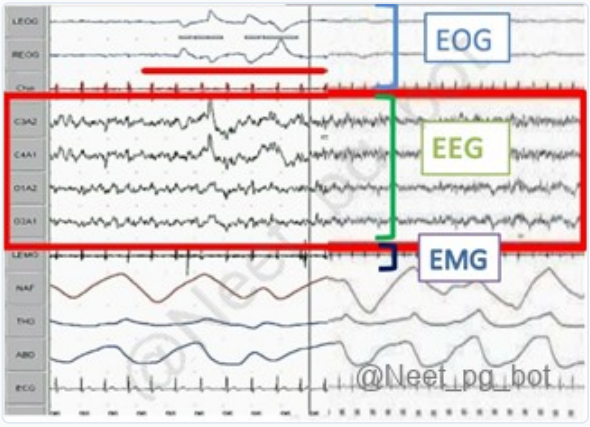
The following table helps identify the state of wakefulness from the graph :

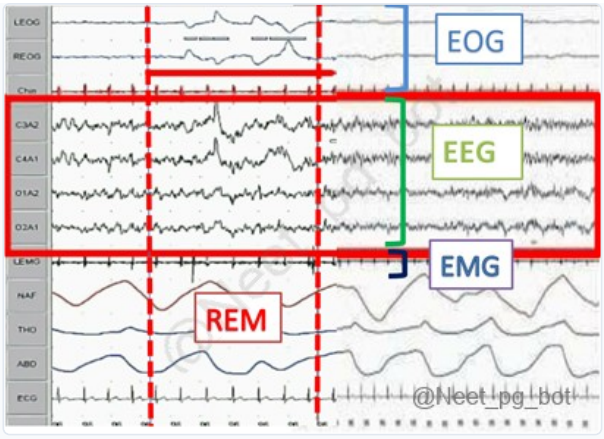
	EEG	EOG	EMG
Active state	Fast	+	+
REM sleep	**Mixed**	**+**	**-**
NREM sleep	Slow	-	+

In the marked area,

- EOG shows Positive activity
- EEG shows Mixed waves
- EMG shows Minimal activity

This is characteristic of **REM sleep**.





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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 5341

Q88. Psychiatry AIIMS 2017 ID: MB9204

Which of the following is the test for immediate memory?

A ✓ Digit span forward up to 7 digits with 2 skips allowed

B Serial (100-7) subtraction test up to 5 steps

C Digit span backwards up to 5 digits with 2 skips allowed

D Serial (20-1) subtraction test up to 5 steps

Correct Answer: Digit span forward up to 7 digits with 2 skips allowed

Forward digit span test is the **best test** among the given options.

Memory is of 3 types:

- Immediate
- Recent
- Remote

Immediate memory is **attention** dependent. Tests of attention and memory include

- Digit span test: **Most important** and **best test for immediate memory**
- Serial (100-7) subtraction test is the best for **concentration**
- Days of the week backwards
- Spell WORLD backwards
- (20–3) test

Digit span test

The examiner recites numbers randomly without any sequence and asks the patient to repeat the same. The examiner starts with a 2-number sequence and the digit span increases from 2 numbers to 3 numbers and so on. The patient is asked to repeat until he/she fails to repeat all numbers correctly.

Digit forward

- Normally a person can repeat up to **7 digits (+/- 2)**
- E.g., Examiner says 2 - 4 –8 –1 –5 –7 - 9. The person has to repeat in the same order 2 – 4 – 8 –1 –5 –7 –9.

Digit backward

- Reproduction of longer digits in backward direction.
- Normally a person can repeat up **to 5 digits** (+/- 1) in backward direction (making option C wrong)
- Eg : Examiner says 2 - 4 –8 –1 –5 –7 - 9. The person has to repeat it backwards. 9 -7 -5 -1 -8- 4- 2.

Serial (100-7) subtraction test

- 5 scores / steps are taken in to account
  - 1st score – When the patient tells 93 (100 - 7 = 93)
  - 2nd score - When the patient tells 86 (93 – 7 = 86)
  - And so on . . 100 , 93 , 86 , 79 , 72 , 65

Hence among the given options :

Digit span forward up to 7 digits with 2 skips allowed would be the answer of choice as it is the best test for immediate memory

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References: Strub and Black, Mental status examination in neurology , 4 edition, Page no 41
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page 203
Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page 205

Q89. Psychiatry AIIMS 2017 ID: MB9155

A patient believes he is the most important person in the world and so his neighbours and family members are trying to harm him as they are jealous of him. His wife says otherwise and says he behaves like this only recently and before that he was working peacefully as a school teacher. He is most likely suffering from?

A Delusion of grandiosity

B Delusion of persecution

C ✓ Delusion of grandiosity and persecution

D Delusion of grandiosity, persecution and reference

Correct Answer: Delusion of grandiosity and persecution

- Belief that he is the most important person in the world – **Delusion of grandeur**
- Belief that his neighbours and family members are trying to harm him – **Delusion of persecution**
- Belief that others feel jealous **– Part of delusion of persecution**

A scenario of this kind is most likely seen in **manic patients** with

- Primary delusion of Grandeur (and)
- Secondary delusion of Persecution

Delusion of reference:

- Delusion in which events, objects, or other persons in one's immediate environment have a **particular and unusual significance** in reference to the person experiencing the delusion.
- **Examples :**
- A feeling that people on television or radio are talking about or talking directly to them
- Believing that headlines or stories in newspapers are written especially for them

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 3637

Q90. Psychiatry AIIMS 2017 ID: MB9187

A young patient is admitted to hospital with acute psychosis. He wakes up and asks for his wife even while she is there in the same room right next to him. Upon pointing her out, he states that this is not his wife and someone else is masquerading as his wife. The most likely diagnosis is?

A ✓ Capgras syndrome

B Fregoli syndrome

C Delusion of subjective doubles

D Othello syndrome

Correct Answer: Capgras syndrome

According to the given scenario,

A stranger has taken the guise of **Wife** (**Family person**)

This is characteristic of **Capgras syndrome**.

Capgras syndrome

The delusion in Capgras syndrome is the belief that a **familiar person has been replaced by an imposter**

Fregoli's phenomenon

- **Fregoli** syndrome is a disorder in which a person holds a delusional belief that different people are in fact a single person who changes his or her appearance or is in disguise.

Intermetamorphosis

- A very rare delusion that familiar persons can change themselves into other persons at will.

Doppelganger

- A person feels he has his own impostor (duplicate) – **Delusion of subjective doubles**

Othello syndrome

- It is a rare psychiatric condition marked by **morbid**, pathological, or **delusional jealousy**.
- Eg : Suspecting a faithful partner of infidelity, with accompanying jealousy.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page nos 319 and 321

Q91. Psychiatry AIIMS 2017 ID: MB9127

A young male patient is on 5 mg haloperidol for many days. However, recently for the last 4 days, he has an inner restlessness and urges to move. The most likely diagnosis is:

A ✓ Akathisia

B Tardive dyskinesia

C Rabbit syndrome

D Acute dystonia

Correct Answer: Akathisia

The probable diagnosis is **Akathisia.**

Haloperidol is an **antipsychotic** drug and a side effect is **Extrapyramidal symptoms.**

Inner restlessness and urge to move are **EPS** referred to as **Akathisia.**

Akathisia

Akathisia consists of **motor restlessness** accompanied by **subjective feelings of inner tension** and **discomfort** referable **chiefly to the limbs.**

Restless leg syndrome

Closely resembles akathisia but occurs in **absence of neuroleptic medication**. **No neurological deficit** is seen.

Tardive Dyskinesia

Tardive dyskinesia is **involuntary movements** of the **tongue, lips, face, trunk, and extremities** that occur in patients treated with long-term dopaminergic antagonist medications.

Rabbit syndrome

Rabbit syndrome is an antipsychotic-**induced rhythmic motion of the mouth/lips**, resembling **the chewing movements of a rabbit**.

The movement consists of a vertical-only motion, at about 5Hz, with **no involvement of the tongue**.

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References: Lishman's organic Psychiatry fourth edition, Page no 746
Harrison's Principles Of Internal Medicine - 19th Edition, Page no 2624

Q92. Psychiatry AIIMS 2017 ID: MB9226

An IT employee who has recently taken up the job is feeling guilty, hopeless and not able to concentrate on work. His symptoms started 3 years back when he started college. The most likely diagnosis is:

A Depressive disorder

B ✓ Dysthymia

C Adjustment disorder

D Cyclothymia

Correct Answer: Dysthymia

The above scenario is consistent with the diagnosis of **dysthymia**.

Dysthymia / Persistent Depressive Disorder:

- **DSM V Criteria for diagnosis of dysthymia:** Depressed mood for most of the day as indicated by either subjective account or observation by others, for at **least 2 years**. In children and adolescents - at **least 1 year.**

  Presence of **two (or more)** of the following while being depressed.
  - Poor appetite or overeating
  - Insomnia or hypersomnia
  - Low energy or fatigue
  - Low self-esteem
  - Poor concentration and/or difficulty making decisions
  - Feelings of hopelessness

  Early-onset, beginning in childhood or adolescence and in most cases by the age of 20 years. More common in unmarried and young persons with low income.

Cyclothymia:

- A **milder form** of **bipolar disorder** which is chronic in nature.
- Characterized by episodic mood swings of **hypomania** and **mild depression**.

Adjustment disorder:

- It is an **abnormal and excessive reaction** to an identifiable life stressor.
- It is a **temporary** condition
- Eg: Feeling sad or hopeless in an exaggerated manner after a stressful life event. Inability to cope with stressful situations.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page nos 363 and 365

Q93. Psychiatry AIIMS 2017 ID: MC9609

Mutism and akinesis in a person who is aware of his surroundings and somewhat alert is best described as a feature of:

A Oneroid state

B Twilight state

C ✓ Stupor

D Delirium

Correct Answer: Stupor

Akinetic mutism is a feature of **Stupor.**

- In this condition , the patient is unable to initiate action and speech but appears awake and alert.
- There is severe reduction of relational function – speech and action.

Akinetic mutism is also seen in **Schizophrenia with affective psychosis and dissociative states.**

Stupor :

- Lesion at :
  - Diencephalon
  - Upper brain stem
  - Frontal lobes
  - Basal ganglia
- **Locked In syndrome**

Option A, Oneroid state

- Impaired consciousness.
- Marked emotional changes in response to visual hallucination.
- Patient lives as if in a **dream** state – **occupational delirium.**

Option B, Twilight state

- Abrupt onset of abnormal violent behavior
- Occurs in
  - Epilepsy
  - Alcoholism
  - Brain trauma

Option D, Delirium

- Global impairment of consciousness and cognition.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, pg341

Q94. Psychiatry AIIMS 2017 ID: MC9610

A 25 year old female has thoughts of cutting her fingers, she plans and imagines doing it but never actually does it. She says she is not having any guilt of having such thought. And also says the thoughts are distressing her and she is unable to control them. The thoughts vanish either by ending with a seizure or automatically subsides on its own. Which of the following is the cause?

A Thought insertion

B ✓ Forced thinking

C Crowding of thoughts

D Obsession

Correct Answer: Forced thinking

In the above scenario, the patient has **forced thinking.**

Forced thinking is usually seen in patients with frontal lobe epilepsy.The thoughts are formed are not in the control of the patient and are distressing thoughts.

Option A: thought insertion is seen in schizophrenia. The delusion of thoughts being placed into patient's mind by someone else.
Option C: crowding of thoughts is also seen in schizophrenia. The patient has thoughts that are too fast and excessive and unexplainable. The patient can even locate these thoughts crowding at the back of the head.
Option D: The obsessive-compulsive symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.The patient feels the need to carry out certain behaviours to get rid of these thoughts.

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References: Lishman's organic Psychiatry fourth edition, pg 557
Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, pg 236
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, pg 251
Bradley's Neurology in Clinical Practice, 7th Edition, pg 88

Q95. Psychiatry AIIMS 2019 (May & Nov) ID: MC7069

Under the Mental Health Care Act 2017, a patient has the right to choose his or her caretaker and future course of action in treatment. What is this provision known as?

A Treatment directive

B Mental will

C Future directive

D ✓ Advance directive

Correct Answer: Advance directive

The provision by which a patient has the right to choose his caretaker and future course of action in treatment because of **illness or incapacity** is known as an **advance directive**. It is also known as a **living will,** personal directive, or medical directive.

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References: Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5932926/
Online Resource, https://indiacode.nic.in/handle/123456789/2249?view_type=search&sam_handle=123456789/1362

Q96. Psychiatry AIIMS 2019 (May & Nov) ID: MC0966

Which of the following is not a feature of melancholia?

A Severe anhedonia

B Weight loss

C ✓ Sleep-onset insomnia

D Profound feelings of guilt over trivial events

Correct Answer: Sleep-onset insomnia

Sleep-onset insomnia is a characteristic feature of **anxiety disorder**, not melancholia.

Melancholia is one of the oldest terms used in psychiatry, to describe the **dark mood of depression**. It is still used to refer to a state of depression characterized by:

- Severe anhedonia
- Early morning awakening (**terminal insomnia**)
- Weight loss
- Profound feelings of guilt (often over trivial events)

Melancholia is sometimes referred to as "**endogenous depression**" or depression that arises in the absence of external life stressors or precipitants.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, 11th Edition, Page no 360

Q97. Psychiatry AIIMS 2019 (May & Nov) ID: MC7746

Which of the following best describes paradoxical suicide?

A ✓ Suicide occuring at the time when the patient starts to recover

B Suicide after taking low dose of drug

C Accidental completion of suicide

D None of the above

Correct Answer: Suicide occuring at the time when the patient starts to recover

Paradoxical suicide is defined as suicide which occurs **when the patient begins to recover** from the depression

- This is because, when the patient is in depression, though he/she has the suicidal thoughts cannot act on it due to lack of energy.
- As the patient begins to recover from depression, **patients gain energy** to put their suicidal thoughts into action.

It is called **paradoxical**, as one might expect that the risk of suicide reduces as the patient starts to recover, but on contrary to that, the risk of suicide is increased during the recovery phase.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 771

Q98. Psychiatry AIIMS 2019 (May & Nov) ID: MC7745

Which of the following is true regarding depersonalization disorder?

- A ✓ More common after life threatening trauma
- B Seen in seizure and migraine patients
- C More common in females
- D Mean age at onset is 25 years

Correct Answer: More common after life threatening trauma

Depersonalization is common after a life-threatening trauma.

The **DSM-V** defines depersonalisation as experiences of **unreality**, **detachment**, or being an **outside observer** with respect **to one's thoughts, feelings, sensations, body, or actions** (e.g., perceptual alterations, distorted sense of time, unreal or absent self, emotional and/ or physical numbing).

Another term called **derealization** refers to the experiences of unreality or detachment **with respect to the surroundings.**

The DSM-V diagnostic criteria for **depersonalization/derealization** disorder is as follows:

1. The presence of persistent or recurrent experiences of **depersonalization**, **derealization**, **or both.**
2. Intact reality testing, during the episodes.
3. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.
4. The disturbance is **not attributable** to the physiological effects of a **substance**(e.g., a drug of abuse, medication) or another medical condition(e.g., **seizures**). **(option b)**
5. The disturbance is not better explained by another mental disorder, such as schizophrenia, panic disorder, major depressive disorder, acute stress disorder, posttraumatic stress disorder, or another dissociative disorder.

Characteristic features of depersonalization/derealization disorder:

- The condition commonly occurs after a life-threatening trauma.
- There is no gender preponderance. **Male:Female ratio is 1:1 (option c).**
- Mean age at onset is **16 years** (**option d**).
- The condition is commonly associated with symptoms such as an altered sense of time, difficulty in remembering past memories, anxiety and depression.

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References: DSM-5 (R) Guidebook: The Essential Companion to the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Page no 302-303

Q99. Psychiatry AIIMS 2019 (May & Nov) ID: MC3374

A patient says that he feels that there is something wrong with him mentally and it is because of the summer season. How much would he score on the insight scale?

A 2

B ✓ 3

C 4

D 5

Correct Answer: 3

The patient would **score 3**, as he is aware of being sick but has attributed it to **external factors.**

Insight is rated on a 5 point scale:

1. **Complete denial** of illness.
2. Slight **awareness** of being sick and needing help but **denying** it at the same time.
3. Awareness of being sick, but is attributed to physical or **external factors.**
4. **Intellectual** insight - awareness of being ill, and that the symptoms are due to own particular irrational feelings and thoughts.
5. **Emotional** insight - the highest level of insight. Intellectual insight with making significant **changes** in future behavior.

Loss of insight is seen in schizophrenia, mania, delusional disorders, etc.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 214

Q100. Psychiatry AIIMS 2019 (May & Nov) ID: MC5211

A patient was prescribed escitalopram. Which of the following adverse effect is the least likely?

A Nausea

B Vivid dreams

C Anorgasmia

D ✓ Sialorrhea

Correct Answer: Sialorrhea

Sialorrhea is **not a side effect** of escitalopram.

Escitalopram is a selective serotonin reuptake inhibitor (SSRI). **SSRIs** are associated with **dry mouth, and not sialorrhea.**

System	Side effects of SSRIs
Sexual	**Anorgasmia**, Decreased libido
Gastrointestinal (5-HT3 receptor)	**Nausea**, Diarrhea, Anorexia, Vomiting, Flatulence, and Dyspepsia
Nervous system	Extremely **vivid dreams** or nightmares, Anxiety, Insomnia/sedation, Seizures, Emotional blunting
Anticholinergic	Dry mouth, Constipation, Sedation
Cardiovascular	Drug induced long QT syndrome
Hematological	Easy bruising
Endocrine	Mammoplasia, Galactorrhea

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 1019

Q101. Psychiatry AIIMS 2019 (May & Nov) ID: MD0205

A 48-year-old male presents to the ER with confusion and shaking of the entire body. He has a history of excessive alcohol consumption. What is the cause of delirium tremens in an alcoholic?

A ✓ Acute infection

B Fatty liver

C Gradual withdrawal of alcohol

D Small doses of regular consumption of alcohol

Correct Answer: Acute infection

Among the given options, **acute infection** can be a cause of **delirium tremens** in an alcoholic patient.

Physical illness e.g., **hepatitis** or **pancreatitis** predisposes to the syndrome; a person in good physical health rarely has delirium tremens during alcohol withdrawal.

The syndrome usually develops on the third day of admission. A patient admitted for an unrelated condition may unexpectedly have an episode of delirium, which is the first sign of a previously undiagnosed alcohol-related disorder.

Episodes of delirium tremens usually begin at the age of 30-40 years, after 5 to 15 years of heavy drinking, and are typical after a **binge.** Gradual withdrawal of alcohol does not cause delirium tremens. However, **sudden withdrawal** can cause it.

Treatment of alcohol withdrawal:

- For patients showing **withdrawal** symptoms, **chlordiazepoxide** 25-50 mg every 2-4 hours can be administered orally.
- For patients with **delirium tremens**, **50-100 mg of chlordiazepoxide** should be administered every 4 hours.
- If oral administration of drugs is not possible, **intravenous lorazepam** can be tried.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page No. 631

Q102. Psychiatry AIIMS 2019 (May & Nov) ID: MD0206

A patient of anorexia nervosa was eating all her meals according to the prescribed diet chart but there is no change in her weight after 1 week. Which of the following needs to be done?

A ✓ Observe the patient for 2 hours post meal

B Ask the psychiatrist to prescribe anti-anxiety medication

C Increase the calories from 1500 to 2000 in her diet

D Increase water intake

Correct Answer: Observe the patient for 2 hours post meal

Observe the patient for **2 hours post-meal** because the patient might vomit the food.

Treatment of anorexia nervosa:

- **Hospitalization:**
  - Patients with a weight 20% less than expected weight for height should be considered for hospitalization.
  - During a hospitalized treatment program, patients should be weighed daily, early in the morning after emptying the bladder.
  - The daily fluid intake and urine output should be recorded.
  - If vomiting is occurring, hospital staff members must monitor serum electrolyte levels regularly and watch for the development of hypokalemia.
  - Because food is often regurgitated after meals, the staff may be able to control vomiting by making the bathroom inaccessible for at least 2 hours after meals or by having an attendant in the bathroom to prevent the opportunity for vomiting.
- **Psychotherapy:**
  - Cognitive-behavioral therapy (CBT)
  - Dynamic psychotherapy
  - Family therapy
- **Pharmacotherapy:**
  - Pharmacological agents are not the first-line treatment but **cyproheptadine**, **amitriptyline**, **clomipramine**, **pimozide**, and **chlorpromazine** show some improvement in these patients.
  - Trials of fluoxetine have resulted in some reports of weight gain.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page No. 514-515

Q103. Psychiatry INI-CET 2024 ID: MG5852

Which of the following is not a priority condition under the WHO Mental health gap action plan?

A Schizophrenia and other psychotic disorders

B ✓ Obsessive compulsive disorder

C Depression

D Mental health disorders in children

Correct Answer: Obsessive compulsive disorder

Obseesive compulsive disorder is not a priority condition under WHO Mental health gap action plan.

WHO aims to offer health planners, policymakers, and donors a comprehensive and cohesive set of initiatives and projects to enhance the provision of care for mental, neurological, and substance use disorders through the **Mental Health Gap Action Programme (mhGAP).**

The priority conditions identified by the above criteria for mhGAP are **depression, schizophrenia and other psychotic disorders, suicide, epilepsy, dementia,** disorders due to use of **alcohol,** disorders due to use of **illicit drugs,** and **mental disorders in children**.

The intervention package includes evidence-based strategies for prevention and management, considering cost-effectiveness, severity of disorders, poverty reduction, and human rights protection.

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References: Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK310854/
Online Resource, https://www.who.int/publications/i/item/9789240084278

Q104. Psychiatry INI-CET 2024 ID: MG5851

Which of the following is not true about 'rapid cyclers'?

A ✓ Occurs commonly in men

B Commonly seen with concomitant hypothyroidism

C At least 4 distinct episodes per year

D Can be triggered by antidepressants

Correct Answer: Occurs commonly in men

Option A is the false statement. Rapid cycling disorder is more **common** in f**emales**.

Rapid cycling disorder is diagnosed when the patient has a combination of **4 or more bipolar episodes within the last 12 months**. Two separate episodes can be demarcated from one another by either switch in polarity or full/partial remission for at least 2 months. **Manic** and **hypomanic** episodes are considered to be on the same pole, hence a switch in polarity includes an episode of mania/hypomania with an episode of depression.

Possible risk factors for rapid cycling disorder include **female patients**, **diabetes**, childhood abuse or family trauma, and alcoholism or substance abuse. Rapid-cycling bipolar patients are significantly more likely than non-rapid-cycling patients to have a lifetime history of comorbidities like anxiety disorders, ADHD, OCD, bulimia nervosa, and substance use disorders.

The drug of choice for prophylaxis of rapid cycling disorder is **valproate**

Other options

Option B: Conditions like hypothyroidism and diabetes are possibly involved in some cases of rapid cycling .

Option D: Usage of antidepressants could worsen rapid cycling in bipolar depression.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page 360
Online Resource, https://pubmed.ncbi.nlm.nih.gov/6404231/
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4519402/

Q105. Psychiatry INI-CET 2024 ID: MG5850

Which among the following is not part of the Beck's cognitive theory on depression?

A Cognitive decline

B Automatic thoughts

C Dysfunctional beliefs

D ✓ Introjection

Correct Answer: Introjection

Introjection is not a part of the Beck's cognitive theory of depression.

Aaron Beck's **cognitive triad** consists of:

- Beliefs about oneself - a pessimistic self-image
- Perceptions of the surroundings - a tendency to see the world as unfriendly and challenging,
- Thoughts about the future - anticipating pain and defeat.

The **psychodynamic** understanding of depression, initially defined by Sigmund Freud and later expanded by Karl Abraham, is considered the classic view of depression. This theory includes four main points: (1) **disturbances** in the **infant-mother relationship** during the oral phase (the first 10 to 18 months of life) create a predisposition to depression later in life; (2) depression can arise from the real or imagined **loss of an important object**; (3) **introjection of the lost object** serves as a defense mechanism to cope with the distress of the loss; and (4) because the lost object is associated with both love and hate, **feelings of anger** are turned inward **toward the self**.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page 354-55

Q106. Psychiatry INI-CET 2024 ID: MG5845

Which of these is not an essential feature for the diagnosis of autism spectrum disorder?

A Persistent deficits in social communication

B Abnormalities in socializing

C Restrictive patterns of behaviour and interests

D ✓ Cognitive impairment

Correct Answer: Cognitive impairment

Cognitive impairment does not form part of the diagnostic criteria for autism spectrum disorder.

The DSM-5 diagnostic criteria for **autism spectrum disorder** are as follows:

- Persistent **deficits in social communication** and **social interaction** across multiple contexts, currently or by history
- Restricted, **repetitive patterns of behavio**r, interests, or activities, currently or by history
- Symptoms must be present in the **early developmental period** (but may not become fully manifest until social demands exceed limited capacities, or may be masked by learned strategies in later life).
- Symptoms cause clinically significant **impairment in social, occupational,** or other important areas of current functioning.
- These disturbances are **not better explained by intellectual disability** or global developmental delay. Intellectual disability and autism spectrum disorder frequently co-occur; to make comorbid diagnoses of autism spectrum disorder and intellectual disability, social communication should be below that expected for the general developmental level.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no : 1156

Q107. Psychiatry INI-CET 2024 ID: MG5846

All the following can be treated with supportive psychotherapy except

A Person with cognitive decline and physical symptoms

B ✓ Person who has good motivation and control over frustration

C Patient with acute psychiatric illness along with crisis

D Patient with severe chronic disease and ego dysfunctioning

Correct Answer: Person who has good motivation and control over frustration

Supportive psychotherapy is not indicated in a person who has **good motivation and control over frustration**. Such individuals are better treated by **expressive psychotherapy**.

Supportive psychotherapy places major etiological emphasis on **external** rather than intrapsychic events, particularly on **stressful environmental and interpersonal influences** on a **severely damaged self**. The general aim of supportive treatment is the amelioration or relief of symptoms through behavioral or environmental restructuring within the existing psychic framework. This often means helping the patient to **adapt better to problems** and to live more comfortably with his or her psychopathology.

Supportive therapy uses several methods, either singly or in combination, including warm, friendly, strong leadership; help in developing pleasurable activities (e.g., hobbies); removal of excessive strain, when possible; medication to alleviate symptoms; and guidance and advice in dealing with current issues. This therapy uses techniques to help patients feel **secure**, **accepted**, **encouraged,** and not anxious.

Supportive psychotherapy is generally indicated for those patients for whom classic psychoanalysis or insight-oriented psychoanalytic psychotherapy is typically contraindicated- those who have **poor ego strength** and whose potential for **decompensation is high**.

Amenable patients fall into the following major areas:

- Individuals in **acute crisis** or a temporary state of disorganization and inability to cope, whose intolerable life circumstances have produced **extreme anxiety or sudden turmoil** (e.g., individuals going through grief reactions, illness, divorce, job loss, or who were victims of crime, abuse, natural disaster, or accident);
- Patients with **chronic severe pathology** with fragile or **deficient ego functioning** (e.g., those with latent psychosis, impulse disorder, or severe character disturbance);
- Patients whose **cognitive deficits and physical symptoms** make them particularly **vulnerable** and, thus, unsuitable for an insight-oriented approach (e.g., certain psychosomatic or medically ill persons);
- Individuals who are **psychologically unmotivated** (e.g., patients who come to treatment in response to family or agency pressure and are interested only in immediate relief or those who need assistance in very specific problem areas of social adjustment as a possible prelude to more exploratory work).

Because support forms a tacit part of every therapeutic modality, it is rarely contraindicated as such. The typical attitude regards better-functioning patients as unsuitable not because they will be harmed by a supportive approach, but because they will not be sufficiently benefited by it.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no : 850-852
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page no : 2358-2367

Q108. Psychiatry NEET 2023 ID: MF8184

A schizophrenic patient was prescribed drug A after he did not respond to haloperidol and thioridazine. He now presents with excessive salivation, an increase in blood glucose, and hyperlipidemia. What is drug A?

A Ziprasidone

B Risperidone

C ✓ Clozapine

D Aripiprazole

Correct Answer: Clozapine

Drug A is **clozapine**, an **atypical antipsychotic** used in the treatment of **refractory schizophrenia**. **Excessive salivation**, an **increase in blood glucose**, and **hyperlipidemia** are **side effects** of clozapine.

The side effects of clozapine are

- Agranulocytosis
- Weight gain
- Seizures
- Drooling
- Hyperthermia
- Hyperglycemia
- Hypertriglyceridemia

Clozapine causes the **highest weight gain** among all antipsychotics. Agranulocytosis is a dose-independent side effect and seizures are a dose-dependent side effect.

Other options:

Option A: Ziprasidone - Although it is an atypical antipsychotic, it does not typically cause excessive salivation, significant increases in blood glucose, or hyperlipidemia. The side effects of ziprasidone include orthostatic hypotension, QT prolongation, and minimal weight gain.

Option B: Risperidone - Risperidone is another atypical antipsychotic, but its side effect profile differs from the symptoms presented in the question. Risperidone is more likely to cause hyperprolactinemia, weight gain, sedation, dystonia, and parkinsonism features than the symptoms mentioned.

Option D: Aripiprazole - It is an atypical antipsychotic used to treat schizophrenia and bipolar disorder, but its side effects are typically less severe than those of clozapine. Aripiprazole is more likely to cause nausea, anxiety, and insomnia than the symptoms described.

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References: A Short Textbook Of Psychiatry 7th edition, Page no. 52
Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK459313/
Harrison's Principles of Internal Medicine, 21st Edition, Page no 3554, 3555(t), 3556

Q109. Psychiatry NEET 2023 ID: MF5261

A woman, who is 4 days postpartum, presented with tearfulness, mood swings, and occasional insomnia. What is the likely diagnosis?

A Postpartum depression

B ✓ Postpartum blues

C Postpartum psychosis

D Postpartum anxiety

Correct Answer: Postpartum blues

Postpartum blues is the most likely diagnosis in a postpartum woman with **tearfulness**, **mood swings**, and **insomnia** from **day 4.**

Postpartum blues, also known as baby blues, is a transient mood disorder characterized by sadness, tearfulness, confusion, and dysphoria. There are no suicidal thoughts or thoughts of harming the baby. These symptoms typically develop **3-5 days after delivery.** They are thought to be due to hormonal changes, the stress of giving birth, and the responsibilities of motherhood. They **resolve spontaneously within 10-14 days.**

Management involves **reassurance, education,** and **support.** The patient must be followed up and assessed if symptoms persist for >2 weeks to evaluate postpartum depression.

Other options:

Option A: When the symptoms of depressed mood, anxiety, insomnia, and weight changes **start within 4 weeks** as per **DSM 5** or **6 weeks** as per **ICD 11,** and the symptoms **persist for ≥ 2 weeks**, the diagnosis will be **postpartum depression.** The patient may have **occasional thoughts** of **harming** the **baby** and **suicidal thoughts**. It can persist for months to years when untreated.

Option C: **Postpartum psychosis** is a psychiatric **emergency** that usually occurs within **4 weeks** after delivery (DSM 5) or **6 weeks** after delivery (ICD 11). The predominant symptoms are **hallucinations**, **delusions**, **suicidal thoughts**, and thoughts of **harming the baby**. Psychotic symptoms such as insomnia, restlessness, labile mood, and agitation are seen. Lithium**,** antipsychotics, and an antidepressant is the treatment of choice.

Option D: **Postpartum anxiety** is not defined by DSM-5. However, some platforms might describe postpartum anxiety as excessive or severe worry after childbirth.

Note: Zuranolone is the first-ever oral drug to be approved specifically for the treatment of postpartum depression. It is a neuroactive steroid that enhances the action of GABA in the brain and regulates mood.

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References: DSM-5 (R) Guidebook: The Essential Companion to the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Page no: 68, 112
Online Resource, https://faculty.ksu.edu.sa/sites/default/files/icd-_11_2018.pdf

Q110. Psychiatry NEET 2023 ID: MF8179

A 25-year-old female reported symptoms of anxiety, palpitations, sweating, breathlessness, chest pain, and a feeling of impending doom. She recalls having experienced 5-6 similar episodes in the past 6 months, each episode lasting for about 20-30 minutes. What is the likely diagnosis?

A Depression

B ✓ Panic disorder

C Generalized anxiety disorder

D Phobia

Correct Answer: Panic disorder

The given clinical scenario is suggestive of a **panic disorder**. Panic disorder refers to an **acute intense attack** of **anxiety** accompanied by feelings of **impending doom.** Such attacks may not only occur in response to a fearful situation, or phobic objects, or in response to obsession or separation as in other anxiety disorders.

Panic disorder is characterized by the presence of an abrupt surge of **intense fear** or **intense discomfort** that reaches a peak within minutes. During the episode, the person may experience symptoms like **palpitations, sweating,** trembling or shaking, shortness of breath, nausea, **dizziness,** chest pain or discomfort or fear of dying. The patient also might feel like losing control, having a heart attack, or **going crazy.**

There is a dysfunction in major neurotransmitter systems like norepinephrine, **serotonin,** and GABA. Panic-inducing substances or **panicogens** may induce such attacks and include substances like 5-35% carbon dioxide, sodium lactate, bicarbonate, yohimbine (α blocker), **flumazenil** (GABA antagonist), cholecystokinin and caffeine.

It is **not attributable** to any **drug abuse**, medication, **medical condition** or mental disorder.

Treatment of this condition can be done by **cognitive behavioral therapy** and drugs like benzodiazepines (**alprazolam**, lorazepam), selective serotonin reuptake inhibitors (**paroxetine**), tricyclic antidepressants and monoamine oxidase inhibitors.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, page 392- 397

Q111. Psychiatry NEET 2023 ID: MF8186

A chronic alcoholic is brought to the emergency department with confusion, ataxia, and painful eye movements. The 6th cranial nerve is also involved. What is the likely diagnosis?

A ✓ Wernicke's encephalopathy

B Korsakoff psychosis

C Delirium tremens

D De Clerambault syndrome

Correct Answer: Wernicke's encephalopathy

The above clinical history is suggestive of **Wernicke's encephalopathy** caused by **thiamine** (vitamin B1) deficiency.

Wernicke's encephalopathy is an **acute**, **neuropsychiatric syndrome** characterized by **nystagmus** and **ophthalmoplegia**, mental status changes (**confusion**), and unsteadiness of stance and gait (**ataxia**). It is a medical emergency, and thiamine should be initiated immediately, either intravenously or intramuscularly. The current standard of treatment for patients with symptoms of Wernicke's encephalopathy is to give **thiamine 100 mg intravenously before** administering **glucose-containing** IV **fluids.**

Other options:

Option B: **Korsakoff psychosis** - While Korsakoff psychosis is also associated with thiamine deficiency and can occur along with Wernicke's encephalopathy, it is primarily characterized by **memory loss and confabulation.** The presence of confusion, ataxia, and painful eye movements with 6th cranial nerve involvement is more indicative of Wernicke's encephalopathy.

Option C: **Delirium tremens** - It is a severe form of **alcohol withdrawal syndrome** and usually occurs **within 3-4 days** of alcohol withdrawal. The features of delirium tremens are **clouding** of **consciousness**, **disorientation** to time and place, poor attention and **distractibility, visual hallucinations and illusions**, **tachycardia, fever,** hypertension, **diaphoresis,** and pupillary dilatation, and psychomotor agitation and ataxia.

Option D: **De Clerembault syndrome** (erotomania) - It is characterized by the delusional belief that someone is deeply in love with them. It is more common in women than in men, commonly a young woman who believes that a man whom she considers to be of higher social and/or professional class, is in love with her. This condition is not related to alcohol abuse and does not cause the neurological symptoms described in the question.

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References: Online Resource, https://www.ebmconsult.com/articles/thiamine-administration-before-iv-glucose-alcoholics
Harrison's Principles of Internal Medicine, 21st Edition, Page no 2525, 3561
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC2552541/#:~:text=A%20syndrome%20which%20was%20first,is%20in%20love%20with%20her.
Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Page No: 631, 632

Q112. Psychiatry NEET 2023 ID: MF8199

A middle-aged man complains of very early ejaculation during intercourse. What is the non-pharmacological management that can be advised to the patient?

A Cognitive behavioral therapy

B Exposure and response prevention therapy

C ✓ Squeeze technique

D Sensate focus therapy

Correct Answer: Squeeze technique

The clinical scenario of ejaculation very early on vaginal penetration is suggestive of **premature ejaculation (PE),** and the **squeeze technique** is a non-pharmacological management (behavioral method) used to prevent it.

PE is a **disorder of the orgasm phase** of the normal sexual cycle.

In premature ejaculation, there is a **persistent** or **recurrent achievement** of orgasm and ejaculation **before** the patient actually wishes to. According to DSM-5, it is classified as follows:

- **Mild** if ejaculation occurs within approximately **30 seconds** to **1 minute** of vaginal penetration
- **Moderate** if ejaculation occurs within approximately **15 to 30 seconds** of vaginal penetration
- **Severe** if ejaculation occurs at the **start** of sexual activity or within approximately **15 seconds** of vaginal penetration.

Pharmacologic therapy when combined with behavioral treatment appears to be more effective in treating **premature ejaculation.**

The **squeeze technique** is a behavioral method used on the penis to prevent premature ejaculation. Here, the penis is stimulated until just before orgasm after which it is gently squeezed.

Other options:

Option A: Although **Cognitive behavioral therapy** (CBT) can be beneficial for some patients with sexual dysfunction. It is not a specific technique for managing PE. Instead, CBT is more useful in addressing underlying psychological issues or relationship problems that may contribute to the condition. CBT is used in the treatment of depression, phobia, anxiety and panic disorders, OCD, personality disorders, and somatoform disorders.

Option B: **Exposure and response prevention therapy** is a strategy within CBT that is effective in the management of OCD. It is also useful in other conditions like tics but is not a suitable method for managing premature ejaculation.

Option D: **Sensate focus therapy** is a couple-oriented sex therapy that involves the exchange of sensual touch between partners to alleviate anxiety and negativity associated with sex and to build better sexual intimacy and communication. It does not specifically address the issue of premature ejaculation like the squeeze technique does and is hence not useful.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 575-577, 579, 580
Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK547551/#:~:text=In%20the%20squeeze%20technique%2C%20the,decrease%20the%20level%20of%20arousal.
Online Resource, https://www.ncbi.nlm.nih.gov/books/NBK470241/
Online Resource, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC1114344/

Q113. Psychiatry NEET 2019 ID: MA5962

Which of the following drugs is used as an anti-smoking agent?

A Busulfan

B Acamprosate

C ✓ Varenicline

D Gabapentin

Correct Answer: Varenicline

Varenicline is an **α4β2 nicotinic acetylcholine receptor partial agonist** used in the treatment of nicotine addiction.

The FDA approved medication to aid in smoking cessation include:

- NRT-**Nicotine Replacement Therapy**
- **Bupropion**
- Varenicline

Drugs are only adjunctive to **behavioral therapy**, which is the **first line of management.**

Behavior therapy is the most widely accepted and well-proven psychological therapy for treating Tobacco use disorder. It consists of several techniques:

- Skill training and relapse prevention.
- Stimulus control.
- Mindfulness approaches.

Bupropion:

- It enhances both noradrenergic and dopaminergic neurotransmission by inhibiting reuptake.
- Used for aiding **smoking cessation** and for the treatment of depression in combination with SSRIs.
- Side effects: seizures, sedation

Varenicline:

- **α4β2 Nicotinic ACh receptor partial agonist.**
- Used for **smoking cessation**.
- Side effects: sleep disturbance, may depress mood and increase suicidal tendencies.

Mirtazapine:

- **Serotonin receptor antagonist.**
- Used for the treatment of **depression** in combination with SSRIs and for **insomnia** in depressive patients.
- Side effect: sedation

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References: Kaplan and Sadock's Synopsis of Psychiatry - 12th edition, Table 4-32, page no 310, 311

Q114. Psychiatry NEET 2019 ID: MC7527

What is the false statement regarding somnambulism?

A Disorder of sleep arousal

B ✓ Person will have full consciousness

C Usually, terminates in awakening followed by confusion

D There may be neurological condition associated

Correct Answer: Person will have full consciousness

Somnambulism is characterized by sequence of behaviors **without full consciousness.**

Note: About 80% of sleepwalkers have a family history of sleepwalking, and an identical twin is much more likely to sleepwalk if their other twin sleepwalks.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 555

Q115. Psychiatry NEET 2019 ID: MC7586

Which of the following is not a risk factor for delusional disorder?

A Recent immigration

B Social isolation

C Family history

D ✓ Young age

Correct Answer: Young age

Advanced age is a **risk factor** for delusional disorders, not young age.

Risk factors for delusional disorders:

- Advanced age
- Sensory impairment or isolation
- Personality features (e.g.unusual interpersonal sensitivity)
- Family history
- Social isolation
- Recent immigration

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 331

Q116. Psychiatry NEET 2019 ID: MC7500

Premature ejaculation is a disorder of which phase of the normal sexual cycle?

A Desire

B Arousal

C ✓ Orgasm

D Pain

Correct Answer: Orgasm

Premature ejaculation is a disorder of the **orgasm phase** of the normal sexual cycle.

Premature ejaculation is when ejaculation occurs before the completion of satisfactory sexual activity. In severe cases, it is characterized by ejaculation either before penile entry into the vagina or soon after penetration. The causes can be biological or psychological, like performance anxiety.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page number 579
A Short Textbook of Psychiatry by Niraj Ahuja, 7th Edition, Page number 127-130

Q117. Psychiatry NEET 2019 ID: MC7504

Confusion assessment test is used in:

A ✓ Delirium

B Dementia

C Schizophrenia

D Depression

Correct Answer: Delirium

Confusion assessment test is used in detection of **delirium.**

Delirium is reported to occur in a wide range of mechanically ventilated ICU patients and can be detected by the Confusion Assessment Method (CAM)-ICU or the Intensive Care Delirium Screening Checklist. These tools are used to ask patients to answer simple questions and perform simple tasks and can be used readily at the bedside.

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References: Harrison's Principles of Internal Medicine 20th Edition, Page no 2029

Q118. Psychiatry NEET 2019 ID: MC7587

Which of the following has a poor prognosis with exposure and response prevention in OCD?

A Pathological doubt

B Magical thinking

C ✓ Hoarding

D Contamination obsession

Correct Answer: Hoarding

Hoarding behavior shows **poor prognosis** with exposure and response prevention.

Challenges posed by hoarding patients to typical cognitive behavior therapy (CBT) include poor insight into the behavior, low motivation, and resistance to treatment.

The most effective treatment for hoarding is a cognitive-behavioral model that includes:

- Training in decision making and categorizing
- Exposure and habituation to discarding
- Cognitive restructuring.

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References: Kaplan and Sadock's Synopsis of Psychiatry - 11th Edition, Page no 430, 431

Q119. Psychiatry NEET 2019 ID: MC7494

What is the fear of heights known as?

A Acarophobia

B Algophobia

C Agoraphobia

D ✓ Acrophobia

Correct Answer: Acrophobia

Acrophobia is the extreme or irrational fear of **heights**.

Phobia	Fear of
Acarophobia	Small bugs
Acrophobia	High places
Agoraphobia	Open or crowded places, leaving familiar setting of home
Aichmophobia	Pointed objects
Ailurophobia	Cats
Algophobia	Pain
Androphobia	Men
Astraphobia or astrapophobia	Thunder and lightning
Autophobia	Being alone, or of self
Bathophobia	Depths
Claustrophobia	Closed or confined places
Coprophobia	Excreta
Cynophobia	Dogs
Demophobia	Crowds
Erythrophobia	Blushing
Gynophobia	Women
Haphephobia	Being touched
Hematophobia	Sight of blood
Hydrophobia	Water
Hypnophobia	Sleep
Lalophobia or glossophobia	Speaking
Mysophobia	Dirt, contamination
Necrophobia	Dead bodies
Nyctophobia	Darkness, night
Panphobia	Everything
Pathophobia or nosophobia	Disease, suffering
Peccatophobia	Sinning
Pedophobia	Children
Phonophobia	Speaking aloud
Photophobia	Strong light
Sitophobia	Eating
Taphophobia	Being buried alive
Thanatophobia	Death
Toxophobia	Being poisoned
Xenophobia	Strangers
Zoophobia	Animals

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References: Kaplan and Sadock's Concise Textbook of Child and Adolescent Psychiatry - 10th Edition, Page no 11583

Q120. Psychiatry INI-CET 2021 ID: MF0794

A person often feels that his hands are contaminated and he is forced to wash his hands. Recently, he feels this repetitive, distressing thought of repetitive hand washing has begun affecting his performance. Which of the following is the best treatment option for this patient?

A DBT

B ✓ Exposure and response prevention

C Systematic desensitization

D CBT

Correct Answer: Exposure and response prevention

A patient presenting with an obsession with contamination and repetitive handwashing is suggestive of classical **obsessive-compulsive disorder (OCD)** and the best treatment option for this patient is **exposure and response prevention (ERP)**.

First line management of OCD in adults can include **pharmacotherapy** or **psychotherapy.** Psychotherapy is primarily based on **exposure and response prevention.**

Exposure and response prevention is a form of exposure therapy that is included under behavioral therapy. It is a more **specific** treatment modality for **OCD** than CBT. It refers to exposing the patient to OCD-inducing situations and preventing them from exhibiting the usual compulsive responses. The level of exposure and control of response to it, is gradual and is decided by the therapist and the patient. The dose and duration of exposure and response prevention therapy should be sufficient for it to be effective.

Cognitive therapies aim to address distortions in thought like increased responsibility to clean hands by replacing them with adaptive thinking patterns.

Pharmacotherapy consists of **SSRIs** and **clomipramine**. Pharmacotherapy is preferred in patients with severe symptoms or with **co-morbid depression**.

Combined therapy with Pharmacotherapy+ Psychotherapy are effective in patients not responding to either of them alone.

Somatic therapies like targeted neurosurgical lesions to cortico-striatal-thalamic cortical tracts (CSPC) and **deep brain stimulation** have proven efficacious in **treatment-refractory patients**.

Cognitive behavioral therapy (Option D) **with ERP** for OCD is preferred in children and adolescents**. Cognitive-Behavioral therapy** comprises of a number of treatment patterns that include both cognitive and behavioral interventions. Cognitive therapy aims to correct maladaptive beliefs and behavioral therapy facilitates better functioning by reducing symptoms. Broadly, it is used to treat depression, generalized anxiety disorder, post traumatic stress disorder, panic disorder, eating disorders, substance use disorder, obsessive compulsive disorder, insomnia, smoking, low back pain.

Other options:

Option A: **Dialectical behavioral therapy (DBT)** is a long-term behavioral intervention that is used in **borderline personality disorder** and to reduce suicidal behavior in adults.

Option C: **Systemic desensitization** is counterconditioning the patient by approaching the feared situation gradually, in a relaxed state. It is useful in **phobias, obsessions, compulsions**, and some **sexual disorders**. It includes relaxation training, hierarchy construction and desensitization of the stimulus.

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Q121. Psychiatry INI-CET 2021 ID: MF0868

Match the following

A) Depression	1) The feeling goes away as soon as you leave the hospital
B) Suicide	2) State of emotional and physical exhaustion caused by prolonged stress, relieved after relaxation
C) Burnout	3) Difficult to diagnose due to the shell of physician's pride
D) Overworking	4) Women physicians, substance abuse, relationship troubles

A A-3,B-4,C-1,D-2

B A-4,B-3,C-1,D-2

C ✓ A-3,B-4,C-2,D-1

D A-2,B-1,C-3,D-4

Correct Answer: A-3,B-4,C-2,D-1

The correct option is A-3,B-4,C-2,D-1.

- Depression - Difficult to diagnose due to the shell of physician's pride
- Suicide - Women physicians, substance abuse, relationship troubles
- Burnout - State of emotional and physical exhaustion caused by prolonged stress, relieved after relaxation
- Overworking - The feeling goes away as soon as you leave the hospital

Depression in doctors:

- Rates of depression are high in doctors.
- **It is difficult to diagnose depression in doctors** as they won't seek medical help because of the fear of being judged personally and professionally.
- Depression if untreated is a significant risk factor for suicide.

Suicide in doctors:

The risk of suicide is more among **female physicians**. Risk factors for suicide can be:

- Personal factors: Underlying psychiatric disorders, relationship troubles, stress, trauma, etc.
- Structural factors: It refers to the condition in which the doctors work. These include increased managerialism, paperwork, rushed patient interaction, inability to time off or rest, work pressure, etc.
- Endemic factors: Exposure to trauma and deaths.

Burnout:

It is classified in ICD 11 as an **occupational phenomenon**. It is defined as a syndrome resulting from chronic workspace stress that has not been managed successfully. It has three dimensions:

- having a feel of **energy depletion or exhaustion**
- Feeling of negativism or cynicism related to the job, or having increased mental distance from the job
- decreased professional efficacy.

It is a phenomenon related to occupational context and is not classified as a medical condition.

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Q122. Psychiatry INI-CET 2021 ID: MF0776

Which of the following is true regarding nicotine substitution therapy?

A Preferably given by gastrointestinal route

B Varenicline comes with a blackbox warning of cardiovascular death

C ✓ There should be a 15 minute gap between nicotine gum and coffee/soda/acidic food as they decreases its absorption

D Nicotine chewing gum is better for constant use as it gives 25% higher blood level than lozenges

Correct Answer: There should be a 15 minute gap between nicotine gum and coffee/soda/acidic food as they decreases its absorption

When a patient is using nicotine gum for nicotine substitution therapy, there should be a gap between nicotine gum and coffee/soda/acidic food as it decreases its absorption.

Treatment of nicotine addiction

1. **Behavioral therapy-** Elimination of the factors that favor smoking in the patient's environment, skill training, etc.
2. **Pharmacotherapy** used in the treatment of nicotine addiction are:

- **Nicotine replacement therapy**- It is used to reduce the symptoms of nicotine withdrawal without the use of tobacco. It is given for the initial maintenance period followed by a reduction period both lasting for about 6-12 weeks. It is available in various forms like
  - **Nicotine gum**- It releases nicotine by chewing and buccal absorption. Smokers can use 1-2 gums per hour and a maximum of 24 gums per day. **Acidic beverages** like coffee, tea, soda, and juice **should not be used during gum use** because they decrease absorption. Poor compliance is a drawback.
  - **Nicotine lozenges**- This is especially useful for people who smoke immediately after awakening. Users must suck the lozenges until it is completely dissolved and must not swallow it. It provides the **highest level of nicotine** compared to other nicotine replacement therapies.
  - **Nicotine patches**- It provides continuous delivery of nicotine and is available as 16 or 24-hour tapering preparations. It has good compliance.
  - **Nicotine nasal spray**- Provides nicotine concentration in the **blood similar to smoking**. Major adverse effects are rhinitis, watering eyes, and coughing
  - **Nicotine inhaler** - Even though it was designed to deliver the nicotine to the lungs, the actual absorption occurs in the upper throat.
- Non-nicotine medications
  - **Bupropion** is an **antidepressant** medication. It has dopaminergic and adrenergic actions.
  - **Varenicline** is a **partial agonist** of alpha-4 beta-2 **nicotinic receptor**. It blocks nicotine from binding to the receptor and decreases the rewarding aspects of cigarette smoking. At the same time, it also stimulates the receptor and reduces withdrawal and cravings. There were concerns about its neuropsychiatric and cardiovascular side effects, but according to recent studies **varenicline is considered a safe drug.**
  - **Nortriptyline -** Second-line drug
  - **Clonidine**- It decreases the sympathetic activity from locus ceruleus and is useful in controlling the withdrawal symptoms.

Nicotine addiction:

It acts on the **nicotinic** subtype of **acetylcholine receptors**. Its addictive properties are thought to be due to the activation of the **dopaminergic pathway** in the **ventral tegmental area**. It causes improved attention, problem-solving ability, learning, and lifts the mood of the users.

Withdrawal symptoms include intense craving for tobacco, irritability, difficulty concentrating, drowsiness, altered sleep, decreased heart rate, and blood pressure, decreased motor performance, and increased muscle tension.

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Q123. Psychiatry INI-CET 2021 ID: MF0834

Match the following

A) Twilight state	1) Subnormal intelligence
B) Oneiroid state	2) Impaired level of consciousness
C) Oligophrenia	3) Persecution
D) Paranoid ideation	4) Oneirophrenia

A A-4, B-2, C-3, D-1

B A-3, B-4, C-1, D-2

C A-1, B-4, C-2, D-3

D ✓ A-2, B-4, C-1, D-3

Correct Answer: A-2, B-4, C-1, D-3

The correct option is A-2,B-4,C-1,D-3.

- Twilight state - Impaired level of consciousness
- Oneiroid state - Oneirophrenia
- Oligophrenia - Subnormal intelligence
- Paranoid ideation - Persecution

Twilight state - It refers to a state of **disturbed consciousness** along with hallucinations.

Oneiroid state or oneirophrenia - It refers to **dream-like delusional derangement of consciousness** in which the patient is not fully oriented in time and place. The characteristic feature of this state is the presence of **kaleidoscopi**c quality of the dream-like experiences where reality, illusions, and hallucinations are merged into one. It is associated with **catatonic disturbances**. The term oneiroid schizophrenia refers to patients engaged in their hallucinations to an extent where there is the exclusion of involvement in the real world.

Oligophrenia - It refers to a group of mental states characterized by **subnormal levels of intellectual activity.** It is either congenital or acquired during early infancy. It is associated with other cognitive disturbances, affective disorders, and volitional disorders.

Paranoid ideation: It refers to thinking that is dominated by **persecutory, suspicious, or grandiose** contents. The thought content is less than that for delusions. Paranoid delusions refer to persecutory delusions and delusions of grandeur, control, and reference.

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Q124. Psychiatry INI-CET 2021 ID: MF0795

Which of the following is not classified as OCD as per ICD 11?

A Trichotillomania

B Body dysmorphic disorder

C Hypochondriac disorder

D ✓ PTSD

Correct Answer: PTSD

Post-traumatic stress disorder(**PTSD**) is **not classified as OCD** under **ICD 11**.

Obsessive-compulsive or related disorders as per ICD 11

It refers to a group of disorders that are characterized by repetitive thoughts and behaviors that are similar in etiology and important diagnostic validators. Important features of these disorders are **obsessions, intrusive thoughts, and preoccupations**. It includes:

- Obsessive-compulsive disorder
- **Body dysmorphic disorder**
- **Hypochondriasis**
- Olfactory reference disorder
- Hoarding disorder
- Body focused repetitive behavior disorders (Skin pricking, **hair pulling - Trichotillomania**)

Important points regarding ICD-11

The ICD 11 was released by the WHO in June 2018 and **will come into effect on 1 January 2022.**

Its designations for antibiotic resistance align with the **Global Antimicrobial Resistance System (GLASS).**

New chapters on

- **Traditional medicine**
- **Sexual health**: Gender incongruence has been moved from mental health to sexual health.
- **Immune system** disorders
- Diseases of the Blood and Blood forming Organs
- Sleep-Wake Disorders
- Extension codes

Gaming disorder has been added under Addictive disorders.

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Q125. Psychiatry INI-CET 2021 ID: MF2115

Which of the following is a true statement about obsessive compulsive disorder (OCD)?

A Atypical antipsychotics is the first line treatment

B Prevalence is 7 -10 percent in general population

C ✓ Depression is a common comorbidity

D Contamination is uncommon obsession

Correct Answer: Depression is a common comorbidity

Obsessive-compulsive disorder (OCD) is **commonly associated** with **depression**. The lifetime **prevalence** of the major depressive disorder in patients with OCD is **67 percent**. Other common psychiatric comorbidities in patients with OCD are social phobia, alcohol use disorders, panic, and personality disorders.

Obsessive-compulsive disorder (OCD) comprises symptoms that include intrusive thoughts, preoccupations, and compulsions. These recurrent obsessions or compulsions cause severe distress to the patient. The patient might have an obsession, a compulsion, or both.

An **obsession** is a **thought**, feeling, or sensation that is repetitive and intrusive. A **compulsion** is a conscious, recurrent **behavior**, eg: counting, checking, and are done to reduce the anxiety associated with the obsession. However, this act may not reduce the anxiety and might increase the anxiety even more.

Symptom patterns:

- **Contamination** - This is the **most common** symptom pattern of OCD. The patient feels that the objects are contaminated. They repeatedly wash their hands or avoid touching objects which are thought to be contaminated. In extreme cases, patients may rub the skin off their hands due to excessive hand washing or may refuse to leave the house in fear of germs.
- **Pathological doubt** - This is the **second most common** symptom pattern. The patient presents with an obsession with doubt and a compulsion to check. This obsession arises due to possible danger or violence (e.g., forgetting to turn off the stove or not locking a door) which is followed by multiple trips to check, for example, if the stove is turned off, if the door is locked, etc.
- **Intrusive thoughts** -This is the **third most common** symptom pattern. These patients have intrusive obsessional thoughts without compulsion. These thoughts could be sexual or an aggressive act that the patient doesn't agree. The patients might report themselves to the police or confess to a spiritual leader.
- **Symmetry** - This is the fourth most common pattern which can lead to a compulsion of slowness. Patients can take hours to eat a meal or shave their faces.

Diagnosis:

1. Presence of obsessions, compulsions or both
2. Symptoms are time-consuming
3. Symptoms not related to substance abuse
4. Not better explained by other mental disorders

Treatment of OCD includes:

- Pharmacotherapy: Selective serotonin reuptake inhibitors (SSRI) or clomipramine
- Behavioral therapy: Approaches include:
  - exposure and response prevention
  - desensitization
  - thought stopping
  - flooding
  - implosion therapy
  - aversive conditioning

A combination of behavioral and pharmacotherapy is most effective.

Other options

Option A - **Atypical antipsychotics** are **not** the first-line in the treatment of OCD. They can be given only when SSRIs and clomipramine (tricyclic antidepressant) are not effective.

Option B - The prevalence of OCD among the general population is **2-3 percent** and not 7-10 percent.

Option D - **Contamination** is the **most common obsession**.

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References: Kaplan & Sadock's Concise Textbook of Clinical Psychiatry 11th Edition, Page no 418-425

Q126. Psychiatry INI-CET 2021 ID: MF2122

Which of the following is true statement about Korsakoff syndrome?

A Triad of confusion, ophthalmoplegia, and ataxia

B Loss of long term memory but intact personality

C ✓ Anterograde amnesia with loss of recent memory

D Loss of implicit memory and immediate recall

Correct Answer: Anterograde amnesia with loss of recent memory

In **Korsakoff's psychosis**, there is **anterograde amnesia** with loss of recent memory.

Korsakoff's syndrome is a chronic amnestic syndrome that can follow Wernicke's syndrome. It is caused by thiamine deficiency that is associated with chronic alcohol abuse. The typical feature of this disease is **anterograde amnesia** with **loss** of **recent memory** in an alert and responsive patient. Patients with Korsakoff's syndrome also typically have a **change in personality**, with a lack of initiative, reduced spontaneity, and a lack of interest.

The treatment is with **100 mg of thiamine** orally for 3-12 months. Despite adequate treatment and nutritional support, very few of the patients fully recover. With treatment, patients may remain amnestic for up to 3 months, and then improve gradually during the next year.

Prolonged use of alcohol causes **alcohol-induced persisting amnestic disorder** with **loss of short-term memory**. When the symptoms are **acute**, it is called **Wernicke's encephalopathy** and when **chronic**, it leads to **Korsakoff's syndrome**. These conditions are caused due to thiamine deficiency and when they occur together, they are called Wernicke-Korsakoff syndrome.

Other options

Option A -Triad of **confusion, ataxia**, and **ophthalmoplegia** is seen in **Wernicke's encephalopathy.** There may also be vestibular dysfunction. Wernicke's encephalopathy is completely reversible. The treatment is parenteral administration of **100 mg thiamine** for 1-2 weeks.

Option B - **Personality** is **affected** in Korsakoff's syndrome. Recent memory is affected more compared to long-term memory.

Option D - **Implicit memory** refers to memory that does not require conscious recollection. Implicit aspects of memory are **preserved**, however, some studies conclude that implicit memory may be intact or impaired depending on the specific task parameters.

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